

GOVERNMENT OF ALBERTA

Annual Report

Health

2019-2020

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Preface

The Public Accounts of Alberta are prepared in accordance with the *Financial Administration Act* and the *Fiscal Planning and Transparency Act*. The Public Accounts consist of the annual report of the Government of Alberta and the annual reports of each of the 21 ministries.

The annual report of the Government of Alberta contains ministers' accountability statements, the consolidated financial statements of the province and the *Measuring Up* report, which compares actual performance results to desired results set out in the government's strategic plan.

This annual report of the Ministry of Health contains the minister's accountability statement, the financial information of the ministry and a comparison of actual performance results to desired results set out in the ministry business plan. This ministry annual report also includes:

- **the financial statements of entities making up the ministry including Alberta Health Services and the Health Quality Council of Alberta, for which the minister is responsible;**
- **other financial information as required by the *Financial Administration Act* and *Fiscal Planning and Transparency Act*, as separate reports, to the extent that the ministry has anything to report.**

Minister's Accountability Statement

The ministry's annual report for the year ended March 31, 2020, was prepared under my direction in accordance with the *Fiscal Planning and Transparency Act* and the government's accounting policies. All of the government's policy decisions as at **June 26, 2020** with material economic or fiscal implications of which I am aware have been considered in the preparation of this report.

[Original signed by]

Honourable Tyler Shandro

Minister of Health

Message from the Minister



It has been an honour to serve as Minister of Health this past year. Looking back, I feel very proud of the work that has been accomplished, not least of which was Alberta's response to the COVID-19 pandemic.

Resources were mobilized across the health system and across the province to respond to the pandemic. Alberta Health Services established a leading testing program and procured Personal Protective Equipment (PPE) early on in the pandemic and set aside acute care capacity to respond to a range of COVID-19 scenarios.

With the benefit of expert advice from Alberta's Chief Medical Officer of Health, public health measures were put in place that minimized the spread of COVID-19, while balancing the economic impact. As a result, our province is weathering the pandemic better than most, while remaining more open. This speaks to the strength of our health system, the people that make it work and Albertans who did their part.

In addition to responding to COVID-19, this past fiscal year included a number of accomplishments in line with our commitment to build a health system centered on patients, improving access to services while maintaining sound fiscal stewardship.

I ordered the first comprehensive review of Alberta Health Services since its formation in 2009. The review gathered input from staff, stakeholders and Albertans. The subsequent report includes 57 recommendations and 72 savings opportunities across all areas of Alberta Health Services operations including clinical, non-clinical and back office. The savings will be reinvested to improve access and patient care. Alberta Health Services is currently developing a plan to implement the recommendations.

In December, we launched the Alberta Surgical Wait-times Initiative. Albertans can expect to see wait-times reduced over the next four years to meet wait-time targets and ensure people receive their care within clinically recommended times. As part of the initiative, chartered surgical facilities will double their current volumes of procedures and complete 30 per cent of medically necessary scheduled surgeries by 2023. In addition, we committed \$100 million to upgrade existing surgical facilities in hospitals across the province. Our partners at chartered surgical facilities and Alberta Health Services are also committed to helping us perform additional surgeries that were delayed due to COVID-19.

In line with our commitment to address unsustainable increases in health spending, we implemented a new physician funding framework. The goal of the framework is to ensure physician spending stays within the \$5.4 billion physician compensation and development budget – which is now a record high in Alberta. We continue listening to physicians to ensure fair compensation and to respond to physicians' concerns, and adjustments have been made to the physician funding framework in response to feedback. We have also enhanced rural practice by: increasing funding to the Rural, Remote and Northern Program; increasing training spots for medical education for medical students who wish to pursue a career in rural Alberta; and increasing the on-call rate for rural physicians. Red tape and barriers have been removed for physicians interested in pursuing different compensation and models of practice including by simplifying the application process for clinical Alternative Relationship Plan and streamlining the approval process for chartered surgical facilities.

Supporting health professionals to work to their full scopes of practice is another government priority that was delivered this year. In September, we announced a \$3 million investment to support primary care networks to hire more nurse practitioners to work in community settings. We also amended the *Licensed Practical Nurses Profession Regulation* to allow licensed practical nurses to offer more services, including dispensing certain drugs.

Government began work on updating Alberta's continuing care legislation to remove unnecessary barriers and reduce red tape while ensuring high quality care and services. Given the challenges, this sector has faced resulting from COVID-19 pandemic, this work will now be expanded to include a broader review of facility-based continuing care. As well, we opened 611 new continuing care spaces, including 68 long-term care spaces, 536 designated supportive living spaces (including 179 for dementia care), and seven palliative care spaces.

This past year marked the launch of phase one of Connect Care, which allows Alberta Health Services to create a single electronic health record for each patient.

The other major focus this year, lead by Jason Luan, Associate Minister of Mental Health and Addictions, was expanding access to mental health and addiction treatment services and supports to enable a full continuum of care across the province.

Our health system demonstrated incredible resilience in the face of the COVID-19 pandemic. To the many Albertans that work across our health system, whether it be working in the frontlines providing direct care, or in the many labs or research facilities across the province: thank you for your dedication and hard work in the face of an unprecedented world-wide pandemic. I look forward to continued collaboration as we work together to make Alberta's health system the very best that it can be.

[Original signed by]

Honourable Tyler Shandro
Minister of Health

Message from the Associate Minister



As Associate Minister of Mental Health and Addictions, I have had the privilege this past year of working with Minister Shandro and the dedicated staff at Alberta Health to improve access to recovery-oriented mental health and addiction supports.

With a \$140 million investment, our government is working hard to respond to a range of urgent needs for mental health and addiction services in our communities and to bring attention and much-needed resources to these important health issues. This investment includes \$4.3 million per year over three years to create an additional 2,172 spaces for treatment at Fresh Start Recovery Centre and Sunrise Healing Lodge, both in Calgary, and Thorpe Recovery Centre near Lloydminster. We also announced \$8 million for additional psychosocial supports for people recovering from

opioid addiction, along with expanding the Virtual Opioid Dependency Program and the Opioid Agonist Therapy Gap Coverage Program.

The COVID-19 pandemic has only increased the urgency of this work. In response, we created the \$53 million COVID-19 Mental Health and Addiction Action Plan, which is comprised of a comprehensive suite of COVID-19-related mental health and addiction supports that will be implemented in 2020-21.

In August of last year, we commissioned a review of the province's supervised consumption services and the socio-economic impact these sites are having on Albertans' homes, businesses and neighborhoods. Released this past March, the report is helping inform government's next steps in regards to supervised consumption services across the province.

In addition, last November, we appointed a 23-member Mental Health and Addictions Advisory Council to develop strategic recommendations on a new recovery-oriented addiction and mental health strategy.

Our government also took the important step of implementing legislation that mandates the licensing of all residential addiction treatment providers in Alberta in accordance with the *Mental Health Services Protection Act*. This will help ensure Albertans receive safe, quality addiction and mental health care when seeking treatment at residential addiction treatment centres.

It has been a busy year, and while I am proud of all that we have accomplished, I recognize there is much still to do. I look forward to continuing to work with health and community partners to strengthen the entire continuum of mental health and addiction care to support Albertans on their paths of recovery.

[Original signed by]

Honourable Jason Luan
Associate Minister of Mental Health and Addictions

Management's Responsibility for Reporting

The Ministry of Health includes the Department of Health, Alberta Health Services, and Health Quality Council of Alberta. The executives of the individual entities within the Ministry have the primary responsibility and accountability for the respective entities. Collectively, the executives ensure the Ministry complies with all relevant legislation, regulations and policies.

Ministry business plans, annual reports, performance results and the supporting management information are integral to the government's fiscal and strategic plan, annual report, quarterly reports and other financial and performance reporting.

Responsibility for the integrity and objectivity of the accompanying Ministry financial information and performance results for the Ministry rests with the Minister of Health. Under the direction of the Minister, I oversee the preparation of the Ministry's annual report, including the financial information and performance results. The financial information and the performance results, of necessity, include amounts that are based on estimates and judgments. The financial information is prepared using the government's stated accounting policies, which are based on Canadian public sector accounting standards. The performance measures are prepared in accordance with the following criteria:

- ▶ **Reliability** – information used in applying performance measure methodologies agrees with the underlying source data for the current and prior years' results.
- ▶ **Understandability** – the performance measure methodologies and results are presented clearly.
- ▶ **Comparability** – the methodologies for performance measure preparation are applied consistently for the current and prior years' results.
- ▶ **Completeness** – outcomes, performance measures and related targets match those included in the Ministry's Budget 2019.

As Deputy Minister, in addition to program responsibilities, I am responsible for the Ministry's financial administration and reporting functions. The Ministry maintains systems of financial management and internal control which give consideration to costs, benefits, and risks that are designed to:

- ▶ provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded so as to maintain accountability of public money;
- ▶ provide information to manage and report on performance;
- ▶ safeguard the assets and properties of the province under Ministry administration;
- ▶ provide Executive Council, the President of Treasury Board, Minister of Finance, and the Minister of Health the information needed to fulfill their responsibilities; and
- ▶ facilitate preparation of Ministry business plans and annual reports required under the *Fiscal Planning and Transparency Act*.

In fulfilling my responsibilities for the Ministry, I have relied, as necessary, on the executives of the individual entities within the Ministry.

[Original signed by]

Lorna Rosen
Deputy Minister of Health
June 26, 2020

Results Analysis

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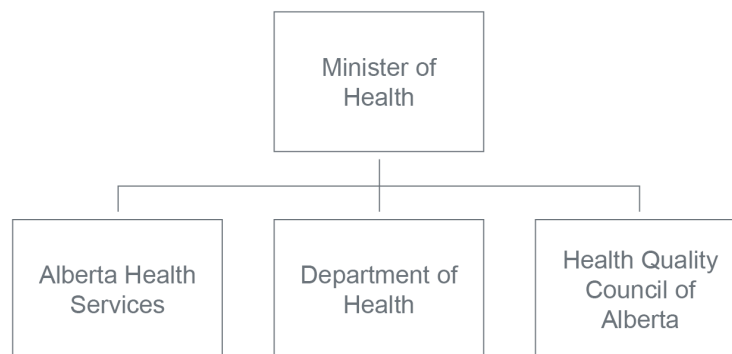
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Ministry Overview

Organizational Structure

The Minister of Health is responsible for setting strategic policy, directing legislation and overseeing the health system to ensure the health of Albertans is protected, resources are aligned with the goals of the government, and public dollars are used appropriately. The ministry consists of:

- **Department of Health;**
- **Alberta Health Services (AHS)** – Alberta’s health authority responsible for delivery of the majority of health care services across the province; and
- **Health Quality Council of Alberta (HQCA)** – the legislated council responsible for monitoring health service quality and patient safety.



Operational Overview

Department of Health

The Department of Health implements the Government of Alberta’s strategic direction for health and is responsible for the overall policy, legislation and monitoring of the health system’s performance. Key functions include: advising government on health policy, legislation and standards; protecting public health and promoting wellness; assuring quality; planning capital infrastructure; overseeing the delivery of health care; supporting information management and technology; enabling health care professionals; funding of the health system; and general oversight.

With direction from the Minister and Associate Minister, the Deputy Minister is responsible for the daily operations of the Department of Health, which is structured as follows:

Deputy Minister’s Office – provides leadership to the health system to ensure quality health services, drive innovation, and continue to build and maintain collaborative relationships across government ministries, AHS, and partner organizations. The office provides policy coordination and issues management for the Minister and Associate Minister, as well as leadership in priority-setting, decision-making, and operations of the ministry. The office is also responsible for ministry correspondence services.

Financial and Corporate Services Division – forecasts and manages the ministry’s budget, funds and monitors the financial activities of the department. It also provides financial advice and prepares annual financial statements, ensuring compliance with Government of Alberta financial legislation. The division oversees corporate planning and reporting; Alberta Health Services accountability; governance of health sector public bodies; and access to information requests under *Alberta’s Freedom of Information and Protection of Privacy Act* and *Health Information Act*. The division also oversees capital planning, including health facilities planning and coordinates infrastructure projects with Ministry of Infrastructure and AHS. The division manages registration, designation and bed survey processes and reporting. The division coordinates the grant approval process and provides general administrative and contracting-based corporate services to enable the department to fulfil its mandate. The division is responsible for the recovery of the cost of health services from liable third parties where appropriate.

Health Information Systems Division – manages the administration of Alberta’s *Health Information Act*, including health information policy and advice, as well as the strategic planning and delivery of information management and technology systems. The division also provides provincial governance of health information management; develops and implements legislative requirements, policy and best practice information related to the secure exchange of health information; delivers information technology solutions to support ministry operations and the provincial e-health environment; and stewards the provincial programs of Alberta Netcare, the province’s electronic health record, and MyHealth Records, the online personal health record for Albertans.

Health Service Delivery Division – is responsible for providing leadership and strategic direction, including the development and implementation of policy, legislation and standards, as well as ongoing monitoring for continuing care, addiction and mental health, and emergency health services. The division is also responsible for working with government, service providers and community partners to promote a coordinated, integrated community-based health care system for Albertans. The division works closely with other social ministries, external organizations, partners, other Canadian provinces and the federal government to plan health service delivery.

Health Standards, Quality and Performance Division – provides leadership for monitoring, assessing, and improving health system performance; health care research and innovation; and Indigenous health policy. The division is the primary source for Alberta’s overall health system data standards and analytics, evaluating the health system’s performance to support evidence-based policy decisions for all health sectors: primary health care, acute and emergency care, continuing care, addiction and mental health, public health, and pharmaceuticals. It is the ministry’s core strategic economic team developing and producing economic evaluations, predictive models, financial forecasts and value for money analyses.

The division is also responsible for ensuring acute and ambulatory care service planning, including transitions in models of care. Decisions are based on clinical evidence, integrate planning with other health care sectors, and ensure health delivery and capital investment are aligned with government policy direction. To improve existing acute and ambulatory care services, the division drives improvements through clinical appropriateness initiatives in collaboration with health care delivery partners, the Health Quality Council of Alberta and the College of Physicians and Surgeons of Alberta, to ensure a safe and high-quality health system.

Health System Integration Division – leads strategic policy development, health system integration and establishment of a vision for the health system of the future. The division leads and coordinates the department’s health system legislation reviews and input to the government’s decision-making and legislation processes. The division is also responsible for coordination of department priorities such as the AHS Performance Review, intergovernmental relations, and monitoring and reporting on the department’s red tape reduction initiatives.

Health Workforce Planning and Accountability Division – develops and implements health workforce policies, regulations, compensation strategies and governance models to enable a health workforce that meets Albertans' needs. The division collaborates with stakeholders, including physicians, professional colleges and associations, and other internal and external partners, to design and administer evidence-informed, value-oriented policies and health benefits that serve the needs of all Albertans.

Pharmaceutical and Supplementary Benefits Division – provides governance to, and oversees the administration of, the Alberta Health Care Insurance Plan, remuneration systems and claims processing for physicians and allied health professionals, and interprovincial reciprocal financial arrangements. The division designs and delivers community-based supplementary health benefit programs on behalf of Albertans requiring pharmaceutical, chiropractic, optical, dental, and other medical supports (wheelchairs, prosthetics, oxygen, medical/surgical supplies, etc.). These supplementary health benefit programs include premium-free prescribed drugs and other benefits to seniors as well as pharmacist-administered vaccinations.

The division also provides leadership to national and provincial organizations to ensure accountable and appropriate delivery of blood, organ and tissue donation, dialysis and other provincial clinical services. Through leading engagement with cross-Canada working groups under the direction of the Interprovincial Health Insurance Agreements Coordinating Committee, the division examines the state of interprovincial health coverage to increase access and reduce barriers.

Public Health and Compliance Division – provides strategic direction and leadership through the assessment, development and implementation of provincial policies, regulations, strategies, and standards. This work is focused on emerging public health crises; communicable diseases; immunization; compliance monitoring; environmental public health; health promotion; and, emergency preparedness, response and recovery. The division carries out these functions to support innovation and engagement with Albertans in wellness, health promotion, and injury and disease prevention. To support health system quality, the division collaborates with partners to perform compliance and monitoring activities and enforcement of the acts, regulations and standards administered by the division in the areas of physician billing; continuing care accommodations; residential addiction treatment facilities; health care services; infection prevention and control oversight; and protection for persons in care.

Office of the Chief Medical Officer of Health – provides leadership and public health expertise on all issues of public health importance such as health surveillance, population health, injury or disease control initiatives and opioid emergency response. This includes taking necessary measures to respond to health crises and to new and emerging pathogens, control and intervention programs to limit the spread of communicable diseases, infection prevention and control measures, and health risk assessments. The Chief Medical Officer of Health has overarching legislated responsibilities for monitoring and reporting on the health of Albertans and intervening to protect and promote the health of the public under authority of the *Public Health Act*. This is accomplished by supporting and sometimes leading the development of healthy public policy and fulfilling the obligations of the Act.

Office of the Alberta Health Advocates – includes the Health Advocate and the Mental Health Patient Advocate. The office supports Albertans in resolving their health-related concerns by helping them navigate the health care system; referring individuals to the appropriate complaints resolution services; providing information about the Alberta Health Charter; requesting the inspection of provincial health care facilities; and, addressing patients' issues and concerns in relation to the *Mental Health Act*.

Communications (CPE-Health) – provides Albertans and health system partners with information about ministry policies, programs, and initiatives. The CPE-Health team works with department staff

to develop and implement communications plans and offers communications support, such as media relations, issues management, writing and editing services, product development, and online communications services. The branch also works closely with AHS and other reporting entities to coordinate ministry communications.

Corporate Counsel – through Alberta Justice and Solicitor General, Corporate Counsel leads a team of lawyers that supports all aspects of the department activities ranging from contracting and procurement, to developing and interpreting legislation and general legal advice to the ministry.

Human Resources – is dedicated to supporting initiatives, delivering programs, and providing human resource expertise and services that attract, retain, and engage the department's workforce. The branch works in partnership with managers and employees to build and sustain workforce capacity to achieve business goals and create an environment where employees are respected, valued, engaged and resilient.

Key Highlights in the Past Year

The ministry's 2019-23 business plan, released as part of Budget 2019 on October 24, 2019, outlines government's vision for Alberta's health system with five outcomes:

- An integrated health care system that puts individuals at the centre and leads to improved health outcomes for Albertans;
- A high-quality health system based on sound fiscal stewardship, clear accountabilities, efficient service delivery and value for investment;
- Albertans have increased access to health professionals and the mix of health professionals that best meets their needs;
- Albertans are supported by accessible and coordinated mental health and addiction services; and,
- The well-being of Albertans is supported through population health initiatives.

In 2019-20, the Ministry of Health led government's response to the COVID-19 pandemic. In early 2020, the ministry's priorities shifted to preparedness, prevention, response, and recovery to enable Alberta's health system to manage the unfolding public health emergency – to reduce the spread of virus and ensure AHS was well prepared with staff and resources to respond to a range of scenarios.

This report provides an overview of important actions taken by the ministry in response to the pandemic and describes the ministry's progress on the government's commitment to provide sustainable, high-quality, patient-centred health care for all Albertans. Key highlights of the ministry's work to strengthen the health system, and close the gap in health outcomes and spending between Alberta and comparable provinces include:

Completed an independent performance review of AHS

- Initiated a performance review of AHS as part of government's commitment to the people of Alberta, in order to strengthen the health system and deliver better access and better results overall. This was the first comprehensive review of AHS since its formation in 2009.
- Released the AHS Performance Review report from contractor Ernst & Young LLP in February 2020. The report, available at <https://open.alberta.ca/publications/alberta-health-services-performance-review-summary-report>, included 57 recommendations and 72 savings opportunities to improve the quality and long-term sustainability of health service delivery. Opportunities were identified in four areas:
 - People – including management, physicians and the broader workforce and ensuring the right mix of staff is used in AHS operations, with staff working to their full scope of practice.
 - Clinical Services – reducing clinical waste and variation and ensuring Albertans are receiving the right care at the right time in the right place.
 - Non-Clinical Services – ensuring the best value for money for support services (e.g., housekeeping, food services, supply chain and back office).
 - Governance – strengthening role clarity between the Department of Health and AHS.
- Directed AHS to develop a comprehensive implementation plan, to be delivered in August 2020, to address the recommendations over the coming years. Implementing the proposed savings opportunities identified in the review will support AHS and the Ministry of Health in meeting budget targets. Savings will be reallocated to frontline services and government priorities like reducing surgical wait times.

Launched the Alberta Surgical Initiative

- Implementing the Alberta Surgical Initiative to improve and standardize the entire surgical system, with the goal of reduced wait times for medically necessary, publicly funded surgical procedures.
- Collaborating with partners to standardize clinical pathways for the patient's journey to surgery and recovery, and centralize surgery intake, building on the intake models currently in place for cancer surgery and hip/knee replacements.
- Reviewing opportunities to expand capacity in chartered surgical facilities, which provided approximately 40,000 publicly funded surgeries in 2019.
- Committed \$100 million to upgrade surgical suites in hospitals, which currently provide 85 per cent of all publicly funded surgeries.

Physician Compensation Management

- The government implemented a new physician funding framework on March 31, 2020 in order to avoid projected cost overruns while maintaining the budget for physicians at \$5.4 billion, ensuring physicians in Alberta are among the highest paid in Canada. The new physician funding framework is being carefully monitored for impacts to physicians and health care delivery, and necessary adjustments are made in consultation with the Alberta Medical Association. Of the framework's 11 proposals, eight have been fully implemented, one rescinded (complex modifiers), and two delayed (overhead and clinical stipends to allow further review by AHS). As part of the new physician funding framework, a Physician Compensation Advisory Committee has been formed, with physician and public members, to examine all aspects of the physician services funding model and provide recommendations to the Minister supporting the delivery of high-quality, patient-focused health services.
- Streamlined the process for creating a clinical Alternative Relationship Plan (ARP), by simplifying the application process, to encourage more physicians to move to this compensation arrangement versus fee-for-service compensation. Alberta Health has been promoting ARPs to physicians because they incent more comprehensive, patient-centred care, while providing physicians with predictable funding and government with cost certainty.
- Completed an independent review of the Academic Medicine Health Services Program (AMHSP) to identify improvements to governance of the program and in preparation for a new AMHSP master agreement.
- Launched a plan to bring radiologist compensation more in line with other provinces.

Ensured health professionals are able to work to their full scope of practice

- Implemented the Primary Care Network (PCN) Nurse Practitioner Support Program to increase the presence of nurse practitioners (NPs) in primary and community care. As of September 2019, there were 50 NPs working in PCNs and a commitment of \$3 million will support PCNs in hiring and funding more NPs.
- Amended the *Licensed Practical Nurses Profession Regulation* to allow LPNs to offer more services including administering blood or blood products and dispensing certain drugs.
- Began work to increase the number of midwifery courses of care available each year and enable midwives to work to their full capacity.

Implemented drug cost saving measures, thereby freeing up funds for re-investment

- Implemented the Alberta Biosimilars Initiative whereby adult patients on a government-sponsored drug plan (except pregnant women) must switch to the lower cost biosimilar version of select higher-cost biologic medications by January 15, 2021 unless there is a clinical reason not to.
- Expanded the Maximum Allowable Cost pricing policy to three additional drug categories where government pays for the lowest cost drug in the category, saving approximately \$15 million annually, to encourage cost-effective prescribing without compromising patient care.
- Changed the eligibility criteria for the seniors drug program, with expected savings of approximately \$36 million annually. Effective March 1, 2020, only Albertans aged 65 years or older are eligible to receive coverage, and seniors' family members under the age of 65 are no longer covered under the plan.

Invested in more long-term care and supportive living spaces, more home care services and more palliative and end-of-life care options

- Began a review of the legislative framework for Alberta's continuing care system that considers people-centred care, authority and responsibility for ensuring standards of care are met, and alignment of programs and services within and across continuing care. Public engagement was put on hold due to pandemic response priorities.
- Developed a plan to add and create new spaces and upgrade existing continuing care infrastructure, including both long-term care and designated supportive living spaces; implementation was put on hold due to pandemic response priorities.
- Opened 611 net new continuing care spaces, comprised of 68 long-term care, 536 designated supportive living (including 179 for dementia care), and seven palliative care spaces.
- Continued expansion of home care to enable more Albertans to remain in their homes and communities for as long as possible, and help ensure timely transition of inpatients out of hospital to an appropriate community setting.
- Committed \$3 million over four years to support caregivers across Alberta, including those caring for continuing care clients and residents.
- Continued collaboration with community organizations to expand supports for those living with dementia and their caregivers.
- Committed \$20 million over four years to support enhancement of palliative and end-of-life care across the province.

Expanded access to recovery-oriented mental health and addictions services and support

- Committed to improving the mental health and well-being of Albertans through the appointment of an Associate Minister of Mental Health and Addictions.
- Appointed a Mental Health and Addictions Advisory Council whose work and recommendations will form the basis of a new comprehensive provincial mental health and addiction strategy and coordinated action plan. The new strategy will identify key actions to improve access to recovery-oriented mental health and addiction services and will include a response plan specific to the COVID-19 pandemic.
- Budget 2019 committed \$140 million over four years to expand access to a continuum of recovery-oriented mental health and addiction services including awareness, prevention, harm reduction and treatment. This includes \$71 million for publicly funded residential addiction treatment and mental health spaces and \$28 million to expand medically assisted detoxification spaces to support 4,000 more Albertans to receive treatment.

- Increased psychosocial supports for people recovering from opioid addiction via opioid dependency programs including addiction counsellors and therapists and peer support programs, such as recovery colleges.
- Mandated all residential addiction treatment providers in Alberta to be licensed in accordance with the *Mental Health Services Protection Act*, ensuring the provision of safe, quality addiction and mental health care.
- Commissioned a review of supervised consumption services (SCS) to evaluate the social and economic impacts of current and proposed SCS sites. In addition to other evidence and information, results of the review will inform the development of a comprehensive, long-term approach to protect communities and improve recovery-oriented supports for Albertans dealing with addiction. The ministry also extended funding for existing SCS, while determining the best way to move forward on a city-by-city basis and improve consistency across all sites.
- Expanded programs to support Albertans in crisis, including police and crisis teams, provincial family violence treatment programs, diversion programs, and by developing mental health court models. Supporting Albertans in crisis will ensure that more people have access to early intervention to reduce the mental health impact of trauma and support their recovery journeys.
- Initiated a review of Alberta's *Mental Health Act* in response to a decision of the Alberta Court of Queen's Bench, to more effectively protect the rights of patients, improve the care provided to them, and modernize the Act. The review also sought to respond to some long-standing concerns of key partners regarding the Act.

Collaborated with Indigenous communities to improve access to health care

- Continued work in partnership with Indigenous communities to improve access to high-quality, culturally appropriate health services and programs that support improved health outcomes, including improved access to primary care, support for maternal, infant and child health, expanded access to a continuum of mental health and addiction services, and funding to support Indigenous communities in assessing their continuing care needs.

Invested in health infrastructure and health information management systems and technology

- Launched the first phase of Connect Care, which is a transformational large-scale IT project, allowing AHS to create a single electronic health record for each patient of AHS for services received in AHS, and ensuring all AHS health providers have access to and contribute to the same information. This project is supported by \$400 million in capital funding over five years.
- Commissioned an independent third-party review of three of the ministry's key IT platforms: Connect Care, Alberta Netcare and MyHealth Records to identify potential duplication of services and ensure maximum effectiveness, efficiency, and value for investment. The review is complete and the final report is available at <https://open.alberta.ca/publications/review-of-connect-care-alberta-netcare-and-myhealth-records-final-report>.
- Began the next phase of the Enabling New Models of Care (ENMOC) initiative to focus on the replacement and redesign of obsolete legacy IT systems to modernize administration of Albertans' health care coverage and health care cards, which ultimately supports key pieces of legislation including the *Alberta Health Care Insurance Act*. The ENMOC implementation project is expected to start in the first quarter of 2020-21.
- Committed \$3.55 billion in capital funding over the next four years for health facilities and equipment, the Capital Maintenance and Renewal program, and province-wide health IT systems.

Discussion and Analysis of Results

The World Health Organization declared COVID-19 a pandemic on March 11, 2020. In response to the public health crisis, contingency funding was added to Health's 2019-20 budget and an additional \$500 million was added to the 2020-21 budget to ensure Alberta's public health officials had the resources they needed to deal with the pandemic. The government declared a public health emergency on March 17, 2020, following the detection of the virus in Alberta, and based on advice of Alberta's Chief Medical Officer of Health.

A series of aggressive public health measures followed to limit the spread of the virus, including mandatory 14-day quarantine for anyone entering Alberta from outside of the country; cancelling all large gatherings; closing schools and daycares; restricting visitors to hospitals, long-term care and other continuing care facilities; and restricting access to certain businesses.

In May 2020, with public health guidelines in place, Alberta began re-opening parts of the economy, striking a careful balance to support the province's social and economic well-being while keeping the most vulnerable Albertans safe from COVID-19. As a result, and thanks to the efforts and sacrifices of all Albertans, we were able to keep 85 per cent of the economy functioning and we were able to start lifting more restrictions sooner than other provinces.

Albertans were kept informed through a provincial COVID-19 information website www.alberta.ca/coronavirus-info-for-albertans.aspx including live-streamed provincial updates. The Alberta government also worked with the federal government, municipal governments, and Indigenous governments to ensure all Albertans benefitted from coordinated pandemic planning and response.

The pandemic response was focused on protecting all Albertans from transmission, particularly those most vulnerable to poor health outcomes, and keeping the health system functioning. Government actions to reduce the impact of the virus on those in long-term care and licensed designated supportive living residences included limiting and restricting visitors to facilities, providing additional personal protective equipment (PPE) for staff, enhanced operational standards and requiring staff that work in multiple designated supportive living or long-term care facilities to choose one facility as their primary workplace.

Health partners had begun collaborating early in 2020 to ensure proven processes and well-trained teams were in place to respond to a pandemic. AHS' supply chain team was well prepared, and coordinated the procurement of additional ventilators and millions of pieces of PPE from across the globe to add to their stockpile of masks, gloves, gowns and face shields. The PPE was distributed to AHS and AHS-contracted facilities, and to non-AHS facilities including disability service providers, social service and civil society groups, and First Nations health organizations. Alberta also provided PPE to Ontario and Quebec, where early demand for PPE was highest and the supply chain was constrained. In the early stages of the pandemic, to ensure Alberta's health care system would have sufficient equipment, staff and hospital beds to meet the demands of pandemic-related health care, all non-urgent surgeries were postponed.

As part of helping Albertans manage their health during the pandemic, those not providing essential services were encouraged to stay at home as much as possible to help limit the spread of the virus. Government supported a variety of programs and services focused on helping Albertans maintain their physical, mental and emotional health, including telephone, online and virtual health care services and support. For example, Albertans were able to visit with their regular physician by phone or online, and government approved the use of a software application to enable those without

a regular doctor to connect to a doctor for routine medical advice and services through their computer, tablet or phone.

Laboratory technicians processing COVID-19 tests, other allied health professionals, and contact tracers were also at the forefront of Alberta's pandemic response. Although routine visits were paused, chiropractors, dentists and physiotherapists were able to continue to offer emergency services, as well as provide advice via phone and videoconference, as appropriate. Alberta also limited the supply of prescriptions to 30 days to protect the drug supply, and at the same time introduced a program to reduce the copayment for all Albertans covered by a government-sponsored prescription drug program.

Alberta was also the only province to supply its residents with non-medical masks, distributing more than 40 million masks in June and July. The partnership with drive-thru restaurants provided more than 700 locations where Albertans could receive the masks over the counter or while in their car, at the same time saving Alberta taxpayers millions of dollars in distribution and packaging costs.

Key success factors in Alberta's provincial pandemic response were the expansion of world-class testing capacity in Alberta Precision Labs and aggressive contact tracing. This was achieved by pausing routine lab tests and increasing the number of staff trained to trace the contacts of infected individuals. An online self-assessment tool helped Albertans determine whether they needed to be tested for COVID-19. Data collected from across the health system informed comprehensive predictive modelling of the virus outbreak to help experts anticipate and prepare for the expected demands on Alberta's health system and guide government's response and recovery efforts.

Review of Programs and Services – Red Tape Reduction

The Ministry of Health is committed to the ongoing review of programs and services to ensure that the best possible outcomes are being achieved for Albertans. As part of this ongoing review, the ministry is committed to reducing red tape to make life easier for hardworking Albertans and businesses. This includes reducing regulatory burden and unnecessary processes to encourage economic growth and job creation; and make Alberta one of the freest and fastest moving economies in the world.

Removing needless red tape – such as duplicate processes and rules – saves time, money and resources, while still ensuring the health and safety of Albertans. This includes streamlining processes and reducing regulatory requirements and unnecessary processes, where appropriate, within the health system.

The department completed an inventory of all regulatory requirements applicable to the ministry and its agencies (excluding AHS) governed by the *Alberta Public Agencies Governance Act*. The inventory included all the requirements in the Ministry of Health's legislation, regulations, policies and forms. The department has already reduced the overall requirements in acts and regulations by 3 per cent and will continue work to identify unnecessary requirements with the goal of reducing regulatory requirements by one-third.

These red tape reductions were achieved through amendments to four acts and three regulations administered by the Ministry of Health. Other changes provide potential cost-savings and revenue generating opportunities for businesses. For example, revisions to the *Food Regulation* allow operators of Bed and Breakfast businesses to serve meals at any time of the day, rather than just breakfast, creating the potential for additional revenue for these small business owners.

In addition, AHS recognizes the importance of reducing red tape to improve processes for patients and their families, as well as health sector businesses. Significant reductions in AHS requirements

have been achieved through centralizing and standardizing policies and forms. Going forward, the continued implementation of AHS' clinical information system, Connect Care, will play a key role in reducing duplication of information gathered from both health practitioners and patients in AHS.

Outcome One: An integrated health care system that puts individuals at the centre and leads to improved health outcomes for Albertans

Outcome statement |

Albertans' health and well-being is improved through an integrated and aligned health care system that is person-centred and structured around individuals, families, caregivers and communities. Albertans have appropriate access to health services throughout their lives, and are active partners in managing their health. Services and transitions will be timely and coordinated across primary care, public health, continuing care and acute care – including timely surgical procedures.

Key Objectives

1.1 Collaborate with health system stakeholders to improve timely access to surgical procedures and timely transition of all inpatients out of hospital to an appropriate community setting.

The Alberta Surgical Initiative (ASI) aims to reduce wait times for medically necessary, publicly funded surgical procedures to within clinically recommended timelines. This will be achieved through improvement and standardization of the entire surgical system – from the time patients seek advice from their family doctor, through to when they are referred to a specialist, and to their surgery and rehabilitation. Wait time information is available on the Alberta Wait Times Reporting website at waittimes.alberta.ca.

AHS is working with partners to standardize clinical pathways, and create a centralized intake model to manage wait list volume and demand across the province. There are currently central intake models for cancer surgery and hip/knee replacements. In 2019, AHS continued an assessment of current surgical intake processes to inform improvements and coordination. AHS is also working to bring surgical specialties into the Alberta Netcare eReferral system in alignment with the ASI.

In Alberta's surgical system, surgeries are performed in hospitals or independent clinics. The AHS Performance Review final report identified opportunities for AHS to better utilize hospital operating room time and resources, including consideration of alternative clinical pathways to enable a wider range of less complex procedures in chartered surgical facilities (previously referred to as "non-hospital surgical facilities") rather than using hospital operating rooms. The report also recommended creation of a dedicated independent provider's secretariat to provide expert advice on innovative approaches to procurement and funding models for health services delivered by independent providers, such as chartered surgical facilities.

Alberta's chartered surgical facilities offer safe, low-risk and cost-effective surgeries, must follow safety and quality standards, and must be accredited by the College of Physicians and Surgeons of Alberta. These facilities currently have capacity for 15 per cent of all publicly funded surgeries provided in Alberta each year. Last year, chartered surgical facilities provided about 40,000 publicly funded surgeries under contract with AHS at no cost to patients. Increasing the annual number of surgical procedures provided in these facilities will free up capacity in hospitals for emergency surgeries and more complex procedures.

At the end of January 2020, AHS started the process of gauging the interest and ability of chartered surgical facilities to provide more publicly funded surgeries as part of the ASI. In March 2020, the initiative was paused due to the pandemic. Despite the delay, AHS is expected to initiate a formal Request for Proposals process in fall 2020 as part of the ambitious goal to provide more surgeries with shorter wait times for Albertans.

In early March 2020, the government announced a capital investment of \$100 million to upgrade surgical suites, which currently provide 85 per cent of the province's publicly funded surgeries. Upgrades include large-scale renovations and some new operating rooms in Edmonton, Calgary, Grande Prairie and Lethbridge. These capital investments and dedicated operational funding will ensure existing operating rooms in the province's hospitals are used more efficiently. These upgrades will allow facilities across the province to perform additional cardiac, orthopedic, cancer, urological, and gynecological procedures, reducing the waitlist for these types of surgeries by 2023.

The AHS Performance Review identified several initiatives that AHS had previously undertaken to improve timely patient flow throughout the health care system. These initiatives, such as Enhanced Recovery After Surgery, brings physicians and clinical care teams together to standardize care before, during and after surgery to support quality care and patient outcomes, while also shortening time spent in hospital.

In 2019, AHS also worked on transition guidelines to better coordinate care from family doctor to the hospital and back home, and reported that as of the end of 2019, the Collaborative Care program (CoACT) is now active on 234 units at 53 sites across the province, with 33 units having reached full implementation. In the CoACT program care teams work together, in partnership with patients and families, to achieve optimal health outcomes. CoACT frameworks have been developed for Continuing Care, Addictions and Mental Health, and Women's Health. A framework for Emergency Departments is in development.

Coordination of care is also supported by electronically notifying a primary care doctor when their patient is admitted or discharged from a hospital in Alberta, receives day surgery, or visits an emergency department. The first clinics started receiving hospital eNotifications in August 2019.

1.2 Support Albertans in accessing appropriate and timely palliative and end-of-life care by increasing awareness of how and when to access palliative care options, shift from hospital to community-based home and hospice care, and expand effective caregiver supports.

Across Alberta, palliative and end-of-life care (PEOLC) is offered in a range of settings including, but not limited to, acute care hospitals, outpatient clinics, primary care provider settings, community hospice spaces, continuing care facilities (designated supportive living and long-term care), emergency medical services, and home care. A variety of programs and services aim to improve the quality of life for those nearing the end of life, including those living with a life-limiting illness. Consideration of options for timely, appropriate, and quality care could include advanced care planning discussions that enable reflection on personal values and preferences, establishing goals for care that span the disease process from early diagnosis to end-of-life, and providing bereavement support.

Alberta's ongoing work in PEOLC aligns with Health Canada's Action Plan on Palliative Care (2019), as well as PEOLC related initiatives and priorities occurring in other Canadian jurisdictions. Budget 2019 committed \$20 million over four years to support enhancement of PEOLC across the province. PEOLC enhancement initiatives were paused due to pandemic response priorities.

In addition to this investment, as part of Enhancing Care in the Community, AHS has implemented a number of PEOLC-related initiatives to improve community-based care and reduce reliance on acute care services. As of December 2019, these include expansion of home and palliative care services; medical treatment for palliative home care clients by emergency medical services personnel, which reduces hospitalizations; and, the Rural Palliative In-Home Funding Initiative, which increases equitable access to palliative home care services in rural areas. Covenant Health, a faith-based organization operating 17 hospitals and care centres across Alberta, works in partnership with AHS in the delivery of PEOLC. Covenant Health's Palliative Institute focuses on supporting excellence in palliative care through clinical support, education, research, policy, and community engagement.

In addition to these actions, AHS has implemented a number of provincial initiatives that support the overall delivery of PEOLC in any health care setting. These include developing and implementing a provincial Advance Care Planning and Goals of Care Designation policy, as well as corresponding resources and provincial 24/7 on-call palliative physician support for primary care providers caring for pediatric and adult PEOLC patients.

AHS has also successfully established a provincial PEOLC website through [MyHealth.Alberta.ca myhealth.alberta.ca/palliative-care](https://myhealth.alberta.ca/palliative-care) with information for patients, families/caregivers and health care providers. This work included developing and implementing guidelines and resources to support health care providers and volunteers in providing PEOLC and information about palliative care provided in hospices in Alberta.

1.3 Enhance home care options and access to self-managed care to continue the shift towards home and community care in order to support Albertans with disabilities and chronic conditions (including people living with dementia) and their caregivers.

The ministry is committed to ensuring the province's continuing care system provides Albertans with the health, personal care and accommodation services they need to support their quality of life and ability to age with autonomy and dignity in their own homes. Government committed \$3 million over four years to support caregivers across Alberta, including those caring for continuing care clients and residents.

Federal and provincial funding allowed continued expansion of home care to enable more Albertans to remain in their homes and communities for as long as possible and help ensure timely transition of inpatients out of hospital to an appropriate community setting.

The AHS Performance Review identified opportunities for AHS to strengthen its integration with primary care through expansion of community-based and home care programs. AHS' Intensive Home Care (IHC) program is an example of an enhanced program that is being implemented across the province. IHC provides case management and coordination of services, including health services, personal care and caregiver support for those clients recently discharged from hospital, to safely enable them to remain at home until an appropriate designated living option becomes available.

The government remains committed to improving the lives of people living with dementia and their caregivers and is supporting several initiatives to encourage innovation in both community and facility-based settings. The province invested \$881,000 in 2019-20 to address the occurrence of pain and depressive moods in designated supportive living and long-term care settings. In addition, 24 grant-funded initiatives were launched which will support people living with dementia, and their caregivers, to get connected at a local level (both community and facility based) in creative and supportive ways.

Community-based projects included dementia-friendly programming at the Camrose Public Library and dementia-friendly transportation by Drive Happiness. Facility-based projects included the

implementation of life stations and dementia wayfinding at Fairview and Sunderland Place Long-term Care Centres and cognition carts at Lamont Health Care Centre. Going forward, the impact of targeted grants will continue to be seen throughout Alberta, including increased supports for rural and remote parts of the province through expansion of the Alzheimer Society of Alberta and Northwest Territories' First Link® program and the community Dementia Ambassador volunteer program.

The government is committed to expanding access to self-directed funding options such as Self-Managed Care (SMC), a program that provides funding directly to a client or their legal representative to hire a care provider of their choice. SMC can improve the quality of life and experience of home care clients by giving them more control over who comes into their home to provide care, as well as the scheduling of their care. The SMC option is designed for individuals with complex care needs who require more significant amounts of personal support (such as assistance with bathing, eating etc.) and home support services (such as homemaking and meal preparation). The ministry is currently examining options for improving availability and access to self-directed funding for home and community care clients.

On February 7, 2020, the government announced a review of the legislative framework for Alberta's continuing care system to ensure Albertans have access to high-quality continuing care. The review will consider topics such as people-centred care; clear authority and responsibility for ensuring standards of care are met; and, improved alignment of programs and services within and across continuing care. Public engagement was put on hold due to the pandemic response priorities. When engagement resumes it will be vital to gather perspectives from Albertans including clients/residents and their families, and key system partners – including home care providers, facility operators, seniors' organizations and others.

1.4 Evolve the use of information technology to improve person-centred care, including modernizing administration of the Alberta Health Care Insurance Plan and Alberta's personal health care cards.

Modern-day demands on Alberta's publicly funded healthcare system require modern-day solutions, including evolving the use of information technology (IT) within the health system. IT supports integrated patient care, allows patients to be an active participant in their health and care, and gives Albertans many ways to access information about their health concerns by phone, via online chat or virtual visits, or through access to health information through the MyHealth Alberta website myhealth.alberta.ca.

These connections enabled by IT remain critical during the pandemic, including the ability to access the online COVID-19 assessment tool and the advice available through Health Link 811. Online resources were made available through the Government of Alberta's pandemic response website alberta.ca/covid-19-information.aspx. These resources include information on how Albertans can protect themselves and their families during the pandemic, as well as psychosocial and community supports for caregivers and factsheets with COVID-19 information for students, for example.

Supports for Albertans' mental and emotional health are important, even more so during the pandemic. Albertans can address mental health issues as early as possible to avoid escalation and the need for higher levels of service. Care and support can be accessed through a range of help lines, such as the Mental Health Help Line at 1-877-303-2642, the Addiction Help Line at 1-866-332-2322, and the Family Violence Information Line, available at 310-1818 for anonymous help in more than 170 languages.

The ministry is also developing protocols and processes for a 24-hour phone line staffed by a sexual assault nurse examiner to provide real-time guidance and support to health professionals, who may

be outside major urban centres, as they conduct sexual assault examinations, including evidence collection. The Ministry of Health is also collaborating with the Ministry of Justice and Solicitor General to gather data related to sexual assault evidence kit collection to identify opportunities for improvements.

In 2019-20, work progressed on a number of key IT systems and initiatives including Connect Care, Alberta Netcare, Community Information Integration, the Central Patient Attachment Registry, MyHealth Alberta, MyHealth Records, and Enabling Models of Care (ENMOC). The ENMOC initiative aims to redesign and replace nine core business applications supporting Alberta Health Care Insurance Plan (AHCIP) registration, enrollment, provider payments, new models of care, and data provisioning.

Connect Care (which launched in Edmonton in November 2019) provides a platform that allows AHS to establish an organization-wide system for patient medical charts, workflow and decision-making, and secure messaging between clinicians, ensuring all AHS health providers have access to and contribute to the same pool of information. Having a shared system supports team-based, integrated care with a focus on the patient, and efficient and effective service provision, which helps reduce disjointed care, gaps, overlaps, errors and delays. Government has committed \$400 million in capital funding over five years to support this largest clinical IT transformation project in AHS' history.

Alberta Netcare is the province's electronic health record. This system holds a consolidated copy of key patient-centred health information from multiple care delivery organizations, including hospitals, diagnostic facilities, pharmacies and clinics. AHS is a major contributor of information, which will become more standardized and robust with the implementation of AHS' Connect Care system. As of March 31, 2020 over 49,000 registered health care providers have access to the system.

Alberta Netcare also hosts an eReferral system, which connects health care providers and specialists to assist communication about non-urgent clinical questions and patient access to timely appointments with specialists. The number of specialties and the number of health zones that are a part of the eReferral program continues to grow. AHS reported that in 2019-20, two specialties were added to eReferral, namely Family Medicine - Transgender Care, and Psychiatry - Child and Adolescent.

In 2019-20, \$4.6 million was invested in the Community Information Integration/Central Patient Attachment Registry Project to support continuity of care across care settings. The Community Information Integration program brings patient health information from community clinics into Alberta Netcare for sharing between community-based physicians and other providers in the patient's circle of care. This information includes selected clinical data on physician office visits, and specialist consultation reports. It also connects to the Central Patient Attachment Registry, which links a patient to their family physician so that the physician can automatically receive notifications from AHS regarding patient admissions and discharges. These programs improve continuity of care, encourage better disease management and care planning, and enable smoother transitions between care settings, all while increasing efficiency in the system by avoiding duplicating care.

The MyHealth Alberta website provides Albertans with trusted health information, services, and tools that empower them to manage and participate in their health care journey. As of December 31, 2019, the number of visits to the site had increased by 78 per cent (over 22.4 million visits) compared to the same period last year (12.6 million visits).

MyHealth Records (myhealth.alberta.ca/myhealthrecords) is a secure online platform available to all Albertans 14 years of age or older, through a computer or mobile device. The platform can assist Albertans in taking an active role in managing their health, such as recording and tracking blood

pressure and fitness activities, and viewing their results from commonly ordered lab tests, immunization records, and medication records. As at March 31, 2020, MyHealth Records had over 140,000 registered users.

The adoption of new technology through the ENMOC initiative is key to improving administration of health care coverage including modernization of Alberta's personal health care cards. Health is collaborating with Service Alberta to determine the most effective method for modernizing personal health care cards to ensure the cards are safe, secure and durable. Health is also working with a third-party vendor on one-time revalidation and subsequent ongoing renewal processes for health care cards, ensuring only eligible Alberta residents receive services covered by the AHCIP.

Other IT system improvements in 2019-20 included an automated report submission process for PCNs; enhancement of Alberta's organ and tissue donation registry; enhancements to the Alberta Provincial Surveillance system; and, new reporting functionality for immunization and adverse events following immunization, as per the new requirements outlined in Alberta's *Immunization Regulation*.

The ministry paused some key IT change initiatives including progress on rolling out the Connect Care initiative, to concentrate IT efforts on supporting the COVID-19 response.

1.5 Engage with Indigenous communities and the federal government to improve access for Indigenous People to high-quality health services that support improved health outcomes.

The Alberta government is responsible for publicly funded health care and services for all Albertans, including First Nations, Métis, and Inuit peoples. In addition to this, the federal government provides specific programs and services for on-reserve First Nations Albertans. Despite investments in Indigenous health programs, Indigenous people experience disproportionately poorer health outcomes, for reasons including lower physician attachment rates, less access to primary care services, and experiences of racism in the health system.

The ministry is committed to working with Indigenous communities to improve access for Indigenous people to high-quality health services that support improved health outcomes. A series of Health Sub-Tables have been established under protocol agreements with the Treaty 8 First Nations of Alberta, the Blackfoot Confederacy, the Métis Nation of Alberta, and the Long-Term Government and Funding Arrangement with the Metis Settlements General Council. Formal working relationships have been established through the Regional Indigenous Organization process. Both types of working arrangements provide a means of engaging communities on matters that impact Indigenous peoples, and to identify activities with tangible outcomes to address Indigenous health inequities. The federal government is also engaged in supporting and coordinating the implementation of Jordan's Principle in Alberta to ensure that First Nations children are not denied access to health and social services due to jurisdictional disputes or their First Nations status.

In 2019-20, disparities in primary care continued to be addressed through the Clinical Alternative Relationship Plan (ARP) program. The Indigenous Wellness Program ARP and Siksika Health and Wellness ARP, with an annual funding envelope of \$8.2 million, funds primary care physicians for services in Indigenous clinics across the province, including the Indigenous Wellness Clinic in Edmonton, and services in Fort Chipewyan First Nation, Bigstone Cree Nation, Enoch Cree Nation, Maskwacis Cree First Nations, and Siksika Nation. The Indigenous Wellness Program ARP also improves availability of pediatric, obstetric and psychiatric care.

Other initiatives described under Outcomes 2 and 4 include expanded access to a continuum of mental health and addiction services to provide a range of appropriate support for individuals, families and communities, as well as funding to support Indigenous communities in assessing their continuing care needs.

Performance Metrics

Performance Indicator 1.a

Unplanned medical readmissions to hospital within 30 days of discharge

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
14.6%	13.8%	13.7%	14.0%	13.5%

Source: Canadian Institute for Health Information (CIHI). Alberta Health Discharge Abstract Database (DAD); Hospital Morbidity Database (HMDB); National Ambulatory Care Reporting System (NACRS).

The indicator tracks the percentage of medical patients with unplanned readmission to hospital within 30 days of leaving the hospital. Results exclude hospital admissions for surgery, pregnancy, childbirth, mental health diseases and disorders, palliative care and cancer therapy.

Results Analysis

The percentage of unplanned readmissions of patients recently hospitalized because of a medical condition has remained relatively stable over the past five years. The 2019-20 result of 13.5 per cent is 1.1 percentage point lower than in 2015-16. A lower percentage means fewer patients have been readmitted to hospital within one month of discharge.

Although readmission may involve external factors, lower readmission rates are supported by discharge planning and continuity of services after discharge. Rates may also be impacted due to the nature of the population served by a hospital facility, such as elderly patients or patients with complex health needs, or by the accessibility of post-discharge health care services in the community.

The Canadian Institute for Health Information ranked Alberta seven out of 12 provinces/territories for this indicator in 2018-19. AHS continues to monitor hospital readmissions and a range of initiatives have been undertaken to address the needs specific to each zone. For example, patients with multiple chronic conditions can now be linked to Community Health Navigators to assist them with system navigation, understanding information, locating community resources, and supporting self-management. Coordination of care is also improving because of recent enhancements to the health information system that enables electronic notification of primary care doctors when their patient is admitted or discharged from hospital.

Performance Indicator 1.b**Median number of days hospital stay extended until home care services or supports were ready**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
25	16	12	8	8 (preliminary)

Source: Alberta Health Discharge Abstract Database (DAD).

This indicator tracks timely access to home care by counting the median number of days an Albertan remains in an acute care hospital when they no longer require it and they are waiting for home care services or supports to be available to them for discharge. The median number of days is the mid-point between the minimum and maximum number of days a person's hospital stay was extended for this reason. A patient with instructions to return to their doctor or referral to a specialist (as part of a routine discharge plan) is not counted in this metric.

Results Analysis

The preliminary 2019-20 median result of eight days is consistent with the 2018-19 result. The desired result is to lower the number of days a person is waiting for discharge from hospital until home care services and support are ready. A number of factors can explain an extended stay, including availability of services, and complexity of post-discharge needs that require more time to plan appropriate services at home. Other factors that can influence the indicator results include accessibility to comprehensive home care services, such as through AHS' Intensive Home Care program, or client-driven delays due to availability of a friend or family caregiver at home.

Performance Indicator 1.c**Ambulatory Care Sensitive Condition hospitalization rate**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
373	360	351	341 ¹	321

Source: Numerator: Alberta Health Discharge Abstract Database (DAD); Denominator: Alberta Health Population Estimate.

¹The 2018-19 result is updated.

This indicator measures the number of people (under 75 years of age) per 100,000 population who were hospitalized for health conditions known as ambulatory care sensitive conditions, which include: grand mal status and other epileptic convulsions; chronic obstructive pulmonary diseases; asthma; heart failure and pulmonary edema; hypertension; angina; and, diabetes.

Results Analysis

In 2019-20, 321 people (under 75 years of age) per 100,000 population were hospitalized for health conditions that are routinely treated outside of a hospital inpatient setting by a range of health professionals. This year's result is a significant improvement on 2018-19 results.

While not all hospital admissions for ambulatory care sensitive conditions are avoidable, it is assumed that appropriate ambulatory care could help prevent the onset of this type of illness or condition, control an acute episodic illness or condition, or manage a chronic disease or condition. Optimizing management of these health conditions in the community, including primary health care settings, and support for people in managing their health conditions can contribute to both improved patient outcomes and reduced health care costs.

The ministry continues to focus on increasing access to care in the community and developing a coordinated approach across the continuum of care, to help people prevent and manage chronic health conditions and diseases and improve their quality of life. The ministry is working collaboratively with community partners to coordinate the planning and delivery of supports and services across the province in all AHS health zones. In addition, Alberta's Strategic Clinical Networks focus on specific areas of health such as cardiovascular health and stroke; diabetes, obesity and nutrition; and, respiratory health to implement clinical best practices and ensure consistent care across the province.

Performance Indicator 1.d Wait times for surgical procedures

Surgery (national benchmark)	Prior Years' Results				2019-20 Actual
	2015-16	2016-17	2017-18	2018-19	
Hip replacement (benchmark 182 days)	80.5%	80.2%	70.5%	68.5%	65.5%
Knee replacement (benchmark 182 days)	77.7%	75.2%	64.6%	65.0%	61.5%
Cataract surgery (benchmark 112 days)	60.6%	56.8%	53.3%	48.2%	45.1%

Source: Alberta Health Services

This indicator tracks the percentage of selected common surgical procedures undertaken within established national benchmarks. The results are based on the time (in days) within which people had their surgical procedures performed in Alberta as planned. The wait time calculations do not include persons who received emergency surgical care.

Results Analysis

The 2019-20 results for all three types of surgical procedure included in this indicator show a decline, meaning that fewer Albertans are receiving the surgical procedure within national benchmark wait times when compared to 2018-19 results. Shorter wait times are expected in the future as the Alberta Surgical Initiative is implemented over the next three years. Alberta's Surgery Strategic Clinical Network is also implementing initiatives to improve access and decrease wait times, reduce variation in clinical practice, and increase quality of surgical care provincially.

The following table provides results published by the Canadian Institute for Health Information. For the reporting time period (April 1-September 30, 2018), more Albertans receive surgical procedures within national benchmark wait times than those waiting for surgeries in a number of other provinces. However, a higher proportion of people in comparator provinces of Ontario and Quebec

receive the surgeries within benchmark wait times than in Alberta and a higher proportion of people in British Columbia than in Alberta receive cataract surgery within the benchmark wait time.

Percentage of Surgical Procedures undertaken within national benchmark wait times ¹ (April 1 – September 30, 2018) ²											
	AB ³	BC ³	SK	MB	ON ³	QC ³	NB	NS	PEI	NL	Canada
Hip replacement	70	67	66	49	84	80	55	49	49	88	75
Knee replacement	66	59	56	37	79	77	43	47	26	75	69
Cataract surgery	49	64	62	29	70	83	67	68	40	77	70

Source: Canadian Institute for Health Information. Waittimes.cih.ca

¹National Benchmark wait times: Hip Replacement (182 days); Knee Replacement (182 days); Cataract Surgery (112 days)

²The most recent data available for provincial comparison. Provinces: Alberta, British Columbia, Saskatchewan, Manitoba, Ontario, Quebec, New Brunswick, Nova Scotia, Prince Edward Island, and Newfoundland and Labrador.

³Comparator provinces: Alberta, British Columbia, Ontario, Quebec.

Performance Measure 1.e

Percentage of scheduled surgeries performed in chartered surgical facilities (formerly known as non-hospital surgical facilities)

Prior Years' Results				2019-20 Target	2019-20 Actual
2015-16	2016-17	2017-18	2018-19		
Not Available ¹	Not Available ¹	Not Available ¹	15%	17%	15%

Source: Alberta Health Services Discharge Abstract Database (DAD); National Ambulatory Care Reporting System (NACRS).

¹Results not available as this is a new performance measure.

This indicator tracks the percentage of all surgeries in Alberta that were performed in chartered surgical facilities (CSFs). Currently, 43 CSFs provide surgeries under contract with AHS. Most are in Edmonton and Calgary. All facilities must be accredited by the College of Physicians and Surgeons of Alberta and follow safety and quality standards.

Results Analysis

In the past year, accredited CSFs in Alberta provided 15 per cent of all publicly funded surgeries, with 85 per cent of surgeries provided in hospitals. The result for 2019-20 is consistent with 2018-19 and is two per cent lower than the 2019-20 target. The desired result is to increase the percentage of scheduled publicly funded surgeries performed in CSFs. The 2019-20 result was inhibited by the pause in scheduled surgical procedures in March due to the pandemic. Up to mid-March, AHS was on target to meet the 2019-20 CSF performance target.

Alberta has set aggressive targets to move surgeries deemed safe and appropriate to CSFs over the course of the Alberta Surgical Initiative. Targets have been set that will see incremental increases up to the 2022-23 target of 30 per cent of scheduled surgeries performed in CSFs. This will result in expanded surgical capacity in the provincial health care system and reduction in wait times for less complex scheduled surgeries.

Outcome Two: A high-quality health system based on sound fiscal stewardship, clear accountabilities, efficient service delivery, and value for investment

Outcome statement |

Alberta's health system is based on high-quality, reliable and cost-effective services where all stakeholders are accountable and contribute to positive health outcomes. Responsible fiscal management, supported by evidence and innovation, will ensure the long-term sustainability of Alberta's health system.

Key Objectives

2.1 Develop and implement strategies that support the fiscal sustainability of the health system, bringing Alberta's health spending down to national norms by 2022-23. This includes a performance review of Alberta Health Services, to identify efficiencies and savings in administration and other areas that could be directed towards front-line services.

Alberta has the largest integrated provincial health care system in Canada, which provides quality care and allows for opportunities to develop new ways to improve patient care and health outcomes. Despite its strengths, Alberta's health system, like all health systems, faces challenges. The biggest challenge is delivering high-quality, patient-centred care while ensuring sustainability. Alberta spends significantly more per capita on health care than comparable provinces. However, Albertans have not seen the health outcomes they should expect from this level of investment. In fact, some results are worse than elsewhere in Canada.

Alberta is facing rising demand on its health care system. Alberta's health care spending has been increasing and so have wait times. The province's population continues to grow, and like in other jurisdictions, there has been a sustained increase in the number of seniors and individuals with complex and chronic medical conditions. The province is now also facing unprecedented financial and economic challenges due to the COVID-19 pandemic and low oil prices. This means that it is more important than ever for the ministry to find ways to transform the health system, making it more efficient and effective.

An independent panel (MacKinnon Panel on Alberta's Finances) commissioned by the government noted that Alberta continues to spend more on health care on a per capita basis than the comparator provinces of Ontario, British Columbia and Quebec. The panel's report, released in September 2019, recommended using other provinces as models, and engaging nurses, doctors, other health professionals, stakeholders and the public where appropriate, to make improvements to Alberta's health system. The report is available at <https://open.alberta.ca/publications/report-and-recommendations-blue-ribbon-panel-on-alberta-s-finances>.

Recommendations included making better use of Alberta's health system resources, such as providing more health care in community settings rather than in hospitals; adjusting the mix of Alberta's health professionals; and limiting the increasing cost of physician services by providing incentives for physicians to move to alternative compensation plans. Progress is being made in these areas and performance metric results are included in this annual report.

The 2019 Capital Plan committed \$3.55 billion in capital funding over four years for health facilities, equipment and IT infrastructure. This funding allows investment of almost \$2.9 billion to deliver quality health care through building and upgrading health care facility infrastructure across the province. The plan included continuation of several previously approved health capital projects and programs, including the Calgary Cancer Centre; the Grande Prairie Regional Hospital; expansion of

continuing care capacity across the province; and capital investment in province-wide IT systems. The plan deferred the proposed Child and Adolescent Mental Health building in Edmonton and delayed opening of the new south Edmonton hospital to 2030, which is consistent with the expected growth in demand for acute care beds in the region. In 2019-20, a number of health facilities were completed and/or opened including a new Detoxification and Residential Treatment Facility for the Protection of Children Abusing Drugs (PChAD) program in Red Deer; comprehensive building envelope repairs to the Northern Lights Regional Health Centre in Fort McMurray; and seven Affordable Supportive Living Initiative (ASLI) projects representing 438 affordable supportive living and long-term care spaces.

In 2019-20, \$30 million of the capital budget was spent on the annual Medical Equipment Replacement and Upgrade Program. The 2019 Capital Plan also committed \$533 million over four years to Health's Infrastructure Maintenance and Renewal Program. This funding represents government's ongoing commitment to maintaining and preserving existing health facilities with the intent of reducing overall deferred maintenance.

In 2019, the government commissioned Ernst & Young LLP to examine the operations and organizational efficiency of Alberta Health Services. The review included significant engagement with Albertans, including patients, staff and physicians working in AHS, as well as key stakeholder groups such as patient advocates, regulatory bodies and associations, municipalities and universities. Changes stemming from the review will increase health system efficiency, while improving quality and delivering better results for Albertans. The review provided clear evidence and opportunities where the health system can do better in terms of costs and results. The AHS Review Report identified that AHS is well-positioned to achieve a greater level of system performance. Enhancing standardization and reducing variation in care will improve patient flow through the health system and patients will be cared for in the right place at the right time. The report is available at <https://open.alberta.ca/publications/alberta-health-services-performance-review-summary-report>.

The government accepted the report's recommendations, with two exceptions: there will be no hospital closures and no consolidation of urban trauma centres. AHS will develop an implementation plan by August 2020 with a detailed analysis that will include timing and sequencing of changes to the system, a clear line of sight between expected savings and reallocation to priority areas, and metrics that provide evidence that access and quality of care are being maintained or improved.

2.2 Ensure investments in information technology are effective, efficient, and improve the secure exchange of information among health professionals and with patients, while reducing potential duplication and overlap.

Sharing health information is foundational to team-based and community-based care, and involving Albertans as participating members of their health care teams. Recognizing that different health care professionals may have unique information sharing needs or constraints, the government has responded by evolving and adapting to a changing system with increasing demands. This requires innovation to work differently, while maintaining quality and safety, and addressing barriers to information sharing across IT platforms. Not addressing these barriers can impact a health care providers' ability to make informed decisions when providing care, impede data collection, limit online access to health information for Albertans and affect broad system policy development.

The ministry has three key IT platforms for sharing health information – Connect Care, Alberta Netcare and MyHealth Alberta. In 2019-20, the province spent \$137 million on AHS' Connect Care clinical information system. This major initiative aims to significantly reduce the number of systems AHS uses, stabilize failing IT infrastructure and provide provincial laboratory information services. Efficient service delivery is expected through Connect Care, which will offer a

central access point to patient information for services provided by AHS and its affiliated partners, common clinical standards and enable the use of best health care practices. Health care providers practicing outside of AHS will have access to more robust, consistent and complete clinical information from AHS through enhancements to Alberta Netcare, which is the provincial electronic health record system. MyHealth Records enables Albertans to access some of their personal health information online through a secure portal.

In fall 2019, the ministry commissioned an independent third-party to review Connect Care, Alberta Netcare and MyHealth Records. Ernst and Young LLP were engaged to conduct the review to identify potential duplication of services and ensure maximum effectiveness, efficiency, and value for investment in these three platforms. The ministry received the final report in March 2020. The final report is available at <https://open.alberta.ca/publications/review-of-connect-care-alberta-netcare-and-myhealth-records-final-report>.

2.3 Improve the measuring, monitoring and reporting of health system performance to drive health care improvements.

Evidence-informed decisions are the basis of high-quality care in Alberta's health care system. The ministry continues to increase capacity for evidence-informed practice and policy through enhanced data sharing, research, innovation, health technology assessment, and knowledge translation.

On March 5, 2020, the government announced plans to develop an enhanced data-sharing environment to store valuable provincial health and non-health data in a secure environment that will safeguard and protect personal information. This world-class health data warehouse will strengthen health analytics and drive research and innovation for health-system improvement.

The department is leveraging its existing data warehouse and up to 35 years of health data to enable other ministries to access data to ensure cross-ministry policies, programs and services support the health and well-being of all Albertans. The academic sector, as well as organizations such as AHS, the Alberta Medical Association and the College of Physicians and Surgeons of Alberta, will also have secure access to the data, providing significant opportunities to use data to make meaningful improvements in care and outcomes for patients. For example, Strategic Clinical Networks (SCNs) have brought together clinicians, researchers, patients and policy-makers to improve the quality and standardization of care in specific areas of health, such as cancer care, seniors care and cardiovascular health.

The ministry has established a \$10 million Health Innovation Implementation and Spread Fund to support implementation of proven innovations in the health system, such as new health technologies, services or ways of providing care. The fund has enabled AHS to implement five projects aimed at different aspects of health care, including timely cancer diagnosis; improved access to gastroenterology specialist advice; training for parents to support earlier discharge of their baby from hospital; and extra training for primary care providers – in pediatric mental health, and care for patients infected with Hepatitis C. More projects are expected to be selected in 2020-21.

Grants totaling \$2.2 million annually have been committed to fund internationally recognized external partners at the University of Calgary, University of Alberta, and the Institute of Health Economics, to conduct rigorous, independent health evidence reviews. The findings of these reviews enable the ministry and community health providers to implement policies and best practices in the Alberta context to achieve better health outcomes at a reduced cost.

Ongoing collaboration with ministry partners, including the Health Quality Council of Alberta and AHS, aims to improve how the province measures and reports on patient and client experiences, which helps ensure patient needs are at the centre of health care improvements.

2.4 Create new continuing care spaces where they are most needed, in an efficient and cost-effective manner.

Alberta is facing an increasing demand for facility-based continuing care spaces, due to an aging population combined with the rising prevalence of chronic conditions. It is likely that more and more Albertans will need the increasing levels of support offered through home care and in long-term care and designated supportive living facilities to maintain their independence and quality of life.

The government has initiated the implementation of a new approach to adding, developing, refurbishing and replacing continuing care spaces, which builds on the best aspects of the former Affordable Supportive Living Initiative (ASLI) program. The new approach uses ASLI's approach to partnerships to ensure new continuing care units are added and developed more quickly and cost-effectively. The ministries of Health, and Seniors and Housing are also working together to include continuing care spaces where needed in lodge developments for a flexible range of care. Funding through this partnership has been committed for projects in Spirit River, Manning, Camrose, Drumheller, and Oyen. Federal funding provided by a Canada-Alberta bilateral agreement under the National Housing Strategy has also been committed to support the planning for new affordable seniors housing for three of these projects in Oyen, Spirit River and Manning.

The government worked with AHS to identify communities with priority need for new continuing care spaces to ensure that spaces are developed where they are most needed. In 2019-20, 37 Indigenous communities/organizations received funding to support collaboration with AHS to conduct needs assessments and to support business case development for continuing care capacity in their communities. Additionally, construction is underway to expand and redevelop the Gene Zwozdesky Centre at Norwood in Edmonton. With an investment of \$379 million, the Centre will provide specialized rehabilitative programs, long-term care and dementia care to help seniors and those with complex care needs maintain the best quality of life.

AHS reported that by the end of 2019, nine new continuing care facilities had been opened – four in the Calgary Zone, four in the Edmonton Zone and one in the North Zone. As of March 31, 2020, AHS had opened 611 net new continuing care spaces, made up of 68 long-term care, 536 designated supportive living (including 179 for dementia care), and seven palliative care spaces.

2.5 As a member of the pan-Canadian Pharmaceutical Alliance, collaborate with other provinces to achieve cost-effective and affordable coverage of prescription drugs.

Government spending on prescription drugs is one of the fastest growing costs in the health system, with Alberta spending considerably more on a per capita basis than most provinces. Alberta participates in the pan-Canadian Pharmaceutical Alliance with other provinces to reduce prescription drug costs and increase access to clinically effective and cost-effective drug treatment options.

Alberta is implementing a number of changes to publicly funded prescription drug programs that better align with other provinces. For example, family members under the age of 65 are no longer eligible under the coverage for seniors drug program, and more cost-effective biosimilars are covered instead of their (more expensive) originator brands. Meanwhile, Alberta is targeting funding to those Albertans most in need - for example through the initiation of the Opioid Agonist Therapy Gap Coverage Program and by reducing out-of-pocket costs for patients on the Palliative Coverage Program.

The ministry relies on health technology assessments by the Canadian Agency for Drugs and Technologies in Health, the advice provided by the Alberta Expert Committee in Drug Evaluation and

Therapeutics, and AHS' drug review processes to inform program decisions. These subject matter experts provided critical evidence-based advice to inform government's decision to move to clinically proven lower cost alternatives, while ensuring publicly funded drug programs deliver the best possible health outcomes in the most cost-effective manner.

Effective December 12, 2019, the Alberta government, under the Alberta Biosimilars Initiative, changed the funding of select biologic drugs for patients on Alberta government-sponsored drug plans. Affected adult patients (except pregnant women) were advised they must switch to the biosimilar version of higher cost biologic medications by July 1, 2020. An exception can be requested by a physician if there is a medical reason that prevents a patient from switching to a biosimilar drug. The requirement to switch is now postponed until January 15, 2021, due to increased demands on health providers and the uncertainty facing Albertans as a result of the pandemic.

Another strategy to keep prescription drug costs low is the Maximum Allowable Cost (MAC) policy, which was expanded in November 2019, to include three additional drug categories. MAC is the maximum amount Alberta's government-sponsored drug plans will pay for a specific drug product within a grouping of therapeutically equivalent drug products. Expanding this policy encourages cost-effective prescribing without compromising patient care. Government estimates savings of approximately \$15 million annually from changes to the MAC policy.

Changes to eligibility criteria for the seniors' drug program were introduced this past year, with expected savings of approximately \$36 million annually. Effective March 1, 2020, only Albertans aged 65 years or older are eligible to receive coverage under the seniors' drug program, and seniors' spouses, partners and dependants under the age of 65 are no longer covered under the plan. Seniors' family members under the age of 65 who require ongoing health benefits coverage were advised to consider other coverage options, including private insurance or public options, such as the government-sponsored Non-Group Coverage program.

Performance Metrics

Performance Indicator 2.a Provincial per capita spending on health care

Category	Prior Years' Results				2019-20 Forecast ¹
	2015-16	2016-17	2017-18	2018-19 Forecast ¹	
Total - Nominal	\$4,836	\$4,977	\$5,113	\$5,254	\$5,118
Hospitals	\$1,954	\$1,982	\$1,987	\$2,007	\$1,956
Physicians	\$1,108	\$1,146	\$1,184	\$1,201	\$1,170
Drugs	\$359	\$370	\$365	\$366	\$356

Source: Canadian Institute for Health Information (CIHI), National Health Expenditure Trends (NHEx), 1975 to 2019. Statistics Canada, Demography Division (population estimates).

¹There is a two-year lag in actual results.

This indicator is used to monitor the trend of financial resources used for health care for each person covered by Alberta's publicly funded health care system. The total provincial government per capita spending on health care includes spending by the Ministry of Health, including AHS and health-related spending by other government departments and agencies, and is a gauge of the overall success of cost containment initiatives.

Results Analysis

Government and health system partners, such as health professional colleges and associations, the pan-Canadian Pharmaceutical Alliance and AHS, work together to manage the three biggest health care costs— hospital services, physician compensation, and pharmaceuticals.

The 2019-20 forecast results show per capita provincial government health care spending was reduced for all categories in comparison to 2018-19 forecast results. The desired result is for the per capita provincial government health care spending figure to decrease over time to close the gap between Alberta and the comparator provinces of British Columbia (BC), Ontario (ON) and Quebec (QC). The results provided in the following table of provincial health care spending show Alberta's total per capita health spending is more than these comparator provinces. Alberta also spends more than these three provinces on hospitals and physicians and more than BC and QC on publicly funded pharmaceutical drugs.

2019-20 Per Capita Provincial Government Health Expenditure (Forecast Results)										
	AB ¹	BC ¹	SK	MB	ON ¹	QC ¹	NB	NS	PEI	NL
Total	\$5,118	\$4,275	\$4,806	\$4,759	\$4,385	\$4,643	\$4,718	\$4,851	\$5,177	\$6,022
Hospitals	\$1,956	\$1,611	\$1,804	\$1,892	\$1,491	\$1,648	\$1,979	\$2,147	\$2,169	\$2,381
Physicians	\$1,170	\$957	\$994	\$1,014	\$1,068	\$1,000	\$883	\$908	\$956	\$954
Drugs	\$356	\$228	\$282	\$251	\$400	\$306	\$284	\$310	\$257	\$290

Source: Canadian Institute for Health Information (CIHI), National Health Expenditure Trends (NHEX), 1975 to 2019. Statistics Canada, Demography Division (population estimates).

Provinces: Alberta, British Columbia, Saskatchewan, Manitoba, Ontario, Quebec, New Brunswick, Nova Scotia, Prince Edward Island, and Newfoundland and Labrador.

¹**Comparator provinces:** Alberta, British Columbia, Ontario, Quebec.

As highlighted in the MacKinnon Panel report, when comparing expenditures with other Canadian provinces, there are opportunities to explore whether similar initiatives used in other provinces can help lower costs in Alberta. The AHS Performance Review identified opportunities for AHS to reduce costs and improve health outcomes by using hospital resources more efficiently. The ministry is also pursuing opportunities to better align physician compensation arrangements, and if appropriate align government-sponsored health benefit coverage to that of comparable provinces, as well as continue to reduce drug costs through the work of the pan-Canadian Pharmaceutical Alliance.

Performance Indicator 2.b Cost of standard hospital stay

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
\$8,034	\$8,067	\$7,893	\$7,988	Not Available

Source: Canadian Institute for Health Information (CIHI): Canadian MIS Database (CMDB); Alberta Health Discharge Abstract Database (DAD).

This indicator is used to monitor the cost of a hospital stay after adjusting for differences in the acuity of patients a hospital serves, using nationally calculated adjustments. This standardization

allows direct comparison across sites and jurisdictions of how efficiently and effectively acute care hospital services are provided. It does not compare quality or satisfaction for the services delivered.

Results Analysis

The prior years' results indicate that the cost of a standard hospital stay in Alberta remains around \$8,000. As shown in the following table, Alberta's hospital stay cost is higher than other provinces and substantially higher than the comparator provinces of British Columbia (BC), Ontario (ON) and Quebec (QC).

2018-19 Cost of Standard Hospital Stay by Province									
AB ¹	BC ¹	SK	MB	ON ¹	QC ¹	NB	NS	PEI	NL
\$7,988	\$6,464	\$7,263	\$6,332	\$5,484	\$5,922	\$5,904	\$6,082	\$6,558	\$6,248

Source: Canadian Institute for Health Information

Provinces: Alberta, British Columbia, Saskatchewan, Manitoba, Ontario, Quebec, New Brunswick, Nova Scotia, Prince Edward Island, and Newfoundland and Labrador.

¹**Comparator provinces:** Alberta, British Columbia, Ontario, Quebec.

When assessing whether Albertans are receiving good value for dollars spent on the publicly funded health system, it is important to also consider quality of care and health outcome indicators. AHS will be taking these factors into consideration when moving forward on implementing cost savings initiatives arising from the AHS Performance Review.

Performance Indicator 2.c Alternate level of care days

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
13.7%	15.3%	17.3%	16.2%	15.5% (preliminary)

Source: Alberta Health Discharge Abstract Database (DAD).

This indicator is defined as the percentage of all hospital inpatient days when a patient no longer requires the intensity of care in the hospital setting and the patient's care could be provided in an alternate setting in the community. Community settings that better meet the needs of these hospitalized patients include personal residences, continuing care facilities and rehabilitation facilities.

Results Analysis

This indicator show modest improvement in 2019-20 results (15.5 per cent) when compared to 2018-19 results (16.2 per cent), however, it is higher than 2015-16 (13.7 per cent) and 2016-17 (15.3 per cent). The ministry is committed to reducing the time patients wait in hospital for the appropriate level of care. It is anticipated that Alberta's rate of alternate level of care days will continue to improve as more continuing care spaces are opened and access to enhanced home care supports in all health zones is improved.

Performance Indicator 2.d Facility-based beds in community settings

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
76.7%	77.2%	77.5%	78.2%	78.5%

Source: Alberta Health Services.

This indicator tracks the percentage of facility-based spaces in a community setting (mental health and addiction, long-term care, designated supportive living and sub-acute care outside acute care) as a proportion of all spaces (including both community facilities and acute care hospitals).

Results Analysis

AHS is responsible for implementing operational contracts and annually reporting the facility spaces in service. As of March 31, 2020, 78.5 per cent of facility-based spaces are outside of an acute care hospital setting. This result represents 2,785 mental health and addiction spaces, 15,665 long-term care spaces, 11,853 designated supportive living spaces, 256 community palliative and hospice spaces, and 472 sub-acute spaces in auxiliary hospitals. The percentage of spaces in the community is expected to rise as the government's investments expand capacity across the range of facility-based care available to Albertans.

Adding new continuing care spaces can reduce the number of people waiting in hospital for an appropriate space and/or decrease the amount of time people spend waiting, improving an individual's quality of life, freeing-up hospital spaces for those who need them, and decreasing overall health system costs.

The government is committed to developing new continuing care spaces quickly and cost-effectively and has begun to implement a new approach in order to achieve this. Government will partner with community operators to support the cost-effective addition of new continuing care spaces, including providing an opportunity for operators with existing spaces to enter into contracts for the provision of publicly funded care in these spaces. Communities with priority need for new continuing care spaces will be identified, ensuring new spaces are added or developed where they are most needed.

The government is also making progress on adding facility-based mental health and addiction spaces in the community to expand options to support Albertans in choosing an option for care that best meets their needs.

Performance Measure 2.e Annual rate of operational expenditures

Prior Years' Results				2019-20 Target	2019-20 Actual
2015-16	2016-17	2017-18	2018-19		
4.2%	3.1%	2.7%	3.3%	1.0%	2.1%

Source: Ministry Consolidated Statement of Revenue and Expenses as presented in the Financial Information section of the Ministry of Health Annual Report.

This measure is defined as the year-over-year percentage change in the Ministry Total Expenses per published Statement of Revenue and Expenses less infrastructure support. It is used to monitor the trend of financial resources used for the Ministry of Health and is an indicator of fiscal sustainability and efficiency of the health system.

Results Analysis

The 2019-20 result shows operational expenditures grew by 2.1 per cent when compared to 2018-19 operational expenditures. This was a lower annual rate of growth than shown under Prior Year' Results, but is higher than the 2019-20 target of one per cent. Additional information about 2019-20 ministry expenditures is provided in the Financial Information section of the annual report.

The government is committed to controlling costs by improving the efficiency and effectiveness of the health system and supporting Albertans in maintaining their health and wellbeing. This includes implementing recommendations from the AHS Performance Review and examining best practices from other provinces to identify opportunities for improving health outcomes, while achieving cost savings.

Outcome Three: Albertans have increased access to health care professionals and the mix of professionals that best meets their needs

Outcome statement |

Albertans can choose their care from competent and accountable health care professionals who are delivering high-quality and safe care. This may include physicians, nurses, pharmacists, paramedics, psychologists, dietitians, dentists, mental health counsellors, rehabilitation therapists, social workers and others. The right number, mix and distribution of professionals, working to their full scope of practice, align with health needs across the province and are supported by fiscally sustainable compensation models.

Key Objectives

3.1 Improve Albertans' choice of health professionals by increasing access to nurse practitioners and midwives, while also expanding scopes of practice of other professionals, and removing barriers that limit health care providers from working to their full scope of practice.

The Ministry of Health is committed to working with primary health care partners toward the shared goal of building a primary health care system that puts people, families and communities at the centre of the ministry's work. In 2019-20, the ministry invested \$240.6 million in Alberta's 41 Primary Care Networks (PCNs) to support delivery of team-based primary care. In PCNs, family physicians collaborate with teams of health care professionals (such as nurses, dietitians, psychologists, social workers, and pharmacists) to meet the primary health care needs of their communities. Approximately 80 per cent of Alberta's primary care doctors are part of a PCN, which range in size from five physicians with 4,568 patients, to 477 physicians with 327,900 patients. PCNs implement processes and standards that support shared accountability for comprehensive primary care that best serves the needs of the community, including coordinated health services across the continuum of care.

Over the past year, AHS and PCNs collaborated to develop Zone Service Plans for each of the five health zones covering the province. The plans support PCNs, AHS and community-based organizations to identify and prioritize opportunities to align, integrate and share services across

Zones. In particular, the Zone Service Plans incorporated better coordination of services for five patient populations – maternal, well-at-risk, chronic comorbid, addiction and mental health, and frail elderly. Key areas of focus include specialty access, care transitions, addiction and mental health services and opioid response. The plans were approved by the ministry in March 2020, and will move to implementation in 2020-21. Implementation of these service plans is expected to lead to less duplication and fewer gaps in primary and community services across the Zones.

In 2019, the ministry implemented the PCN Nurse Practitioner (NP) Support Program to increase the presence of NPs in primary and community care. As of September 2019, there were 50 NPs working in PCNs. Budget 2019 committed \$3 million in 2019-20 to support PCNs in hiring and funding the equivalent of an additional 30 full-time NPs. The NP program aims to increase access to primary health care by helping extend PCNs' operating hours to reduce reliance on emergency departments, improving access in rural/remote areas and for underserved populations, and improving support for Albertans in managing chronic disease.

The government has committed to making changes so that Albertans can receive care from the most appropriate health care professionals who are working to their full scope of practice. This includes increasing the number of NPs and midwives, and expanding the scope of practice of nursing professionals where appropriate, as well as making better use of paramedics' full scope of practice. For example, in fall 2019, government removed funding restrictions for midwifery services, and Community Paramedicine programs are ongoing in AHS' Central and North Zones, including Mobile Integrated Health Teams, who work with physicians and community health care providers to deliver on-site, non-emergency care to residents of pre-selected supportive living facilities.

Amendments to Alberta's *Licensed Practical Nurses Profession Regulation* are helping to create a more efficient and cost-effective health system by expanding the scope of practice for licensed practical nurses (LPNs). The changes, which came into effect on February 1, 2020, allow LPNs to offer more services, including administering blood or blood products and dispensing certain drugs. More than 16,000 LPNs work in Alberta – most are AHS staff working in a variety of settings, including hospitals, community health centres, continuing care facilities, urgent care centres and home care.

Albertans want to be confident that their health care providers are working to the highest standards and accountability. Throughout the year the ministry collaborated with a number of regulatory colleges regarding proposed amendments to standards of practice for their membership.

3.2 Develop and implement innovative and fiscally-sustainable approaches to provider compensation and resourcing that supports high-quality care and collaborative practice within interdisciplinary team-based environments.

Provider compensation accounts for a large proportion of the costs of delivering health care to Albertans. With increasing financial pressure on the health system, the ministry is exploring new cost-effective approaches to health service delivery and new provider compensation models that incorporate best practices from other jurisdictions across Canada. For example, AHS' Operational Best Practice initiative uses comparative data from other provinces to establish targets, such as worked hours per patient day and supply cost per patient day, to measure progress towards reducing costs while maintaining or improving care.

To keep Alberta's health system sustainable, there is a pressing need for the government to bring costs in line with other provinces, including keeping physician spending at current levels. Expenditures on physician compensation and development alone make up about 25 per cent of the provincial health budget – approximately \$5.4 billion annually. The MacKinnon Panel recommended the government limit the increasing cost of physician services by moving away from the

fee-for-service (FFS) model for physician compensation, for example, and towards alternative relationship plans.

The FFS model is the most common remuneration model for physicians in Alberta and across Canada. Approximately 83 per cent of Alberta's expenditures on physician services use the FFS model, which is the highest proportion in Canada. The amount paid to the physician for each service or procedure is set out in Alberta's Schedule of Medical Benefits.

Alberta and other jurisdictions also use physician compensation models that are different than the FFS model to allow for more comprehensive, patient-centred care, while providing physicians predictable funding. In Alberta these arrangements include Academic or Clinical Alternative Relationship Plans (ARPs), including a Blended Capitation Model (BCM). An independent review of the Academic Medicine Health Services Program (AMHSP) was completed in 2019-20 to identify improvements to governance of the program and in preparation for a new AMHSP master agreement.

Clinical ARPs pay clinicians for delivering a defined set of clinical services to a target population, for example, people diagnosed with Chronic Obstructive Pulmonary Edema. A BCM arrangement pays a fee for a bundle of services for each person under the regular care of a physician, combined with FFS as necessary, for any additional services provided. The goal of the BCM model is to provide Albertans with increased access to primary health care by enabling a "medical home" concept through strong relationships to their primary care physicians and improved continuity of care, similar to other Clinical ARPs. Modernizing physician funding through ARPs provides opportunities to move away from a focus on quantity of care embedded in the FFS model of compensation towards a greater focus on quality of care. To encourage more physicians into the Clinical ARP model, the ministry is streamlining the application process.

Negotiation and consultation between the government and the Alberta Medical Association (AMA) on a new agreement began in November 2019 and continued into February 2020, including mediation commencing in January. Discussions ceased without an agreement and faced with projected cost overruns of \$2 billion over the next three years the government implemented a new physician funding framework on March 31, 2020, which maintained the budget at \$5.4 billion.

The new physician funding framework is being carefully monitored for impacts to physicians and health care delivery, and necessary adjustments – for example enabling provision of care through virtual means and incentivizing rural physicians by removing the cap on premiums under the Rural Remote Northern Program to improve access in rural and remote communities. Of eleven proposals, eight have been fully implemented, one rescinded (complex modifiers), and two delayed (overhead and clinical stipends to allow further review by AHS). The new framework, also established a Physician Compensation Advisory Committee with physician and public members to examine all aspects of the physician services funding model and provide recommendations to the Minister, supporting the delivery of high-quality, patient-focused health services.

The AHS Performance Review report recommended that in alignment with government's physician negotiations, AHS should address radiologist compensation to bring it more in line with other provinces. On March 12, 2020, AHS moved forward on this recommendation by providing a one-year notice that existing contracts and agreements would terminate on March 31, 2021, and that a Request for Proposal (RFP) would be issued to ensure transparent and fair compensation across Alberta. During this period, service will continue and patient care will not be affected.

Performance Metrics

Performance Indicator 3.a

Percentage of Alternative Relationship Plan payments of total physician payments

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
13%	13%	13%	13%	Not Available

Source: Canadian Institute for Health Information (CIHI): National Physician Database (NPDB); Alternative Payment Information File for Alberta.

This indicator tracks the percentage of total physician payments (clinical and non-clinical) that are made under the terms of an Alternative Relationship Plan (ARP), which is an arrangement that compensates physicians by a method other than fee-for-service. The intent is to increase the percentage as a way to diversify physician payments and achieve greater cost predictability for both physicians and government and improved patient care.

Results Analysis

ARP payments as a percentage of total physician payments has remained at 13 per cent over the prior four years. The ministry is developing strategies to increase physician interest and participation in ARPs, including streamlined application and approval processes.

Performance Indicator 3.b

Percentage of Albertans who had access to a regular health provider

Prior Years' Results				2019 Actual
2015	2016	2017	2018	
80.5%	82.0%	83.6%	83.7%	Not Available ¹

Source: Canadian Community Health Survey, Statistics Canada.

¹The 2019 Survey release date was postponed due to the COVID-19 pandemic.

This indicator tracks the percentage of Albertans aged 12 or older who participated in the Canadian Community Health Survey and responded that they have a regular health care provider. The survey describes a regular health care provider as one health professional that a person regularly sees or talks to when a person needs care or health advice.

Results Analysis

Prior years' results show a small year-over-year improvement and in 2018, 83.7 per cent of Albertans had access to a regular health provider.

Having a regular health care provider is important for early screening, prevention through health and wellness advice, diagnosis, and treatment of a health issue, as well as ensuring good continuity of care and connections to other health and social services. The desired result is to increase the percentage of Albertans that have a regular health care provider. This is consistent with progress towards the following provincial primary health care goals:

- Timely access to appropriate primary care services delivered by a regular health care provider or team;
- Co-ordinated, seamless delivery of primary care services through a patient's 'medical home' and integration of primary care with other levels of the health care system;
- Efficient delivery of high-quality, evidence-informed primary care services; and
- Involvement of Albertans as active partners in their own health and wellness.

Alberta's Primary Care Networks are involved in a variety of initiatives that support provincial and health zone primary care goals, including adopting a 'medical home' approach in their practices. This approach strengthens the connection between a patient and regular health care provider to improve access to care, chronic disease prevention and management, continuity of care, and innovations in primary health care including telemedicine and virtual care.

Performance Indicator 3.c

Percentage of Licensed Practical Nurses relative to Registered Nurses

Prior Years' Results				2019 Actual
2015	2016	2017	2018	
34.2%	36.5%	38.0%	40.0%	41.6%

Source: Canadian Institute for Health Information (CIHI).

This indicator measures Licensed Practical Nurses (LPNs) relative to Registered Nurses (RNs), which includes Registered Psychiatric Nurses but excludes Nurse Practitioners. LPNs and RNs are different types of nursing professionals with different scopes of practice that align to their training. Decisions on health workforce employment and utilization are the responsibility of AHS and other employers. The AHS Performance Review identified opportunities to optimize the skill mix and better leverage the various skill levels of RNs, LPNs and Health Care Aides to ensure efficiency and effectiveness of AHS' complement of nursing staff.

Results Analysis

In 2019, 41.6 per cent of all licensed and registered nurses (excluding Nurse Practitioners) in Alberta were LPNs. This is consistent with the annual increase in the percentage of LPNs relative to RNs over the past five years in both Alberta and Ontario, as provided in the following table.

Percentage of LPNs relative to RNs (Alberta and comparator provinces)				
	Alberta	British Columbia	Ontario	Quebec
2015	34.2%	30.6%	42.6%	41.0%
2016	36.5%	30.1%	45.3%	39.5%
2017	38.0%	29.9%	47.1%	38.7%
2018	40.0%	30.1%	48.8%	38.6%
2019	41.6%	30.2%	50.8%	38.5%

Source: Canadian Institute for Health Information (CIHI) Nursing in Canada, 2019 data tables.

In the past year, both RNs and LPNs have had their scope of practice expanded in Alberta. These changes strengthen the health system by enabling health professionals to practice to the full extent of their competence and training.

Performance Indicator 3.d

Percentage of Nurse Practitioners relative to Family Medicine Physicians

Prior Years' Results				2019 Actual
2015	2016	2017	2018	
7.9%	8.4%	8.7%	9.6%	Not Available

Source: Canadian Institute for Health Information (CIHI).

This indicator tracks the percentage of Nurse Practitioners (NPs) relative to Family Medicine Physicians. NPs are registered nurses with post-graduate degrees and advanced knowledge and skills. They are trained to assess, diagnose, treat, order diagnostic tests, prescribe medications, make referrals to specialists and manage overall care. NPs often work closely with physicians and other health professions as part of a team. Some NPs work independently and manage their own clinics. Family medicine physicians include general practitioners, as well as family medicine specialists and emergency family medicine specialists certified by the College of Family Physicians of Canada.

Results Analysis

In 2018, the percentage of NPs relative to Family Medicine Physicians was 9.6 per cent, which is an increase over the previous year and consistent with the four-year trend. In the following table, comparator provinces of British Columbia, Ontario and Quebec also show a year-over-year increase in the percentage of NPs relative to Family Medicine Physicians.

Percentage of NPs relative to Family Medicine Physicians (Alberta and comparator provinces)				
	Alberta	British Columbia	Ontario	Quebec
2015	7.9%	5.4%	16.7%	3.1%
2016	8.4%	6.2%	18.0%	3.9%
2017	8.7%	6.6%	18.7%	4.2%
2018	9.6%	6.9%	19.1%	4.9%

Source: Canadian Institute for Health Information (CIHI).

The government is encouraging an increase in the number of NPs in primary care settings through the PCN NP Support Program. Enabling NPs to provide care in community-based settings helps to achieve the five identified strategic outcomes for primary health care: access to primary health care; safety and quality of care; continuity of care; care transitions; and, person experience and outcomes. The ministry is working with the Nurse Practitioner Association of Alberta to understand implications of funding models on NP recruitment for primary care and to enable optimal NP scope of practice.

Performance Indicator 3.e**Average wait time from general practitioner to orthopedic surgeon consult**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
22.4	27.7	28.6	29.0	31.0

Source: Alberta Bone and Joint Health Institute.

The indicator tracks the provincial average wait time (in weeks) from the day a surgeon's office receives the referral from the patient's doctor to the patient's first appointment with the orthopedic surgeon. Orthopedic surgeons treat the musculoskeletal system, which includes bones, joints, ligaments, muscles, nerves and tendons. Common orthopedic surgical procedures include knee and hip replacements.

Results Analysis

Results for 2019-20 show that Albertans waited an average of 31 weeks for their first appointment with an orthopedic surgeon. This is two weeks longer than in 2018-19.

The ministry is reviewing every phase of the surgical journey that patients experience, from the consultations with primary care physicians and specialists to receiving their required procedures, including improved referral wait times.

The ministry is also working with primary health care partners to implement telephone and electronic advice between primary care providers and specialists. This will provide appropriate and timely care to Albertans, and will decrease the time that Albertans wait for an appointment with a specialist.

Outcome Four: Albertans are supported by accessible and coordinated mental health and addiction services

Outcome statement |

Preventing, stabilizing and supporting recovery from mental health issues or addiction will reduce the significant negative impact of these challenges on individuals, families, caregivers and communities. The ministry will continue to work with its partners to address mental health and addiction challenges.

Key Objectives

4.1 Implement a mental health and addiction strategy to move toward a quality, coordinated, connected and easily accessible mental health and addiction system.

The government's commitment to the mental health and well-being of Albertans was established through the appointment of an Associate Minister of Mental Health and Addictions and allocation of funds for mental health and addiction services and supports. Budget 2019 committed \$140 million over four years to expand access to a continuum of mental health and addiction services including awareness, prevention, intervention, treatment and recovery.

In November 2019, the Minister of Health appointed the Mental Health and Addictions Advisory Council to develop strategic recommendations to increase access to recovery-oriented addiction and mental health services. The Council is to provide their final report with recommended strategic actions to the Minister of Health and the Associate Minister of Mental Health and Addictions in fall 2020, which will inform the development of a new provincial addiction and mental health strategy.

The advisory council, made up of 23 members with a range of personal and professional experience, is building on the work completed in past years, namely the *Valuing Mental Health: Report of the Alberta Mental Health Review Committee* and the Minister's Opioid Emergency Response Commission recommendations. The new Council engaged with stakeholders from existing mental health and addictions advisory and integration committees, people with lived experience, representatives from Indigenous communities, subject matter experts, community service providers and industry representatives, among others.

Mental health and addiction services are delivered in various settings by a mix of public and private service providers, and are funded by government, philanthropic organizations, and by individuals through user fees or private insurance. It is important to ensure that legislation supports Albertans' health and safety in all settings and situations, while complying with the Canadian Charter of Rights and Freedoms.

In July 2019, the Alberta Court of Queen's Bench found that sections of the *Mental Health Act* did not comply with the Canadian Charter of Rights and Freedoms. Following the Court's decision, government launched a public engagement initiative to gather feedback to inform changes to the Act and ensure it meets legal requirements. Input was gathered from nearly 3,400 individuals via a public online survey. Focused meetings were held with stakeholders such as police organizations; family and patient advocacy representatives; Indigenous representatives; Mental Health Review Panel members; health professions; other ministries; and, AHS (including input from clinicians as required). Changes aim to more effectively protect the rights of patients, improve the care provided to them, and modernize the Act, as well as responding to some long-standing concerns of key partners.

Alberta's Mental Health Services Protection Act also provides a foundation to ensure safe, quality addiction and mental health care. The Act provides authority to license residential addiction treatment facilities and outlines core requirements that must be met. Effective November 1, 2019, all residential addiction treatment providers in Alberta must be licensed in accordance with the Act. The licensing requirements ensure people seeking recovery and their families have safe and quality choices for treatment and recovery options.

4.2 Expand access to a continuum of mental health and addiction services to provide a range of appropriate supports for individuals, families and communities.

Mental health and addiction services may be clinical or non-clinical. Clinical care by health care professionals encompasses diagnosing, treating, or managing a mental health disorder, and may include treatment with medication and intensive or specialized care. Non-clinical care includes health promotion and prevention, harm reduction strategies, awareness-raising, screening, brief intervention and referrals to treatments. The government is committed to supporting a continuum of accessible and coordinated mental health and addiction services, including awareness, recovery-oriented prevention, harm reduction and treatment, to support and promote the recovery journey and reduce the significant negative impact these challenges have on individuals, families, and communities.

The ministry funds prevention initiatives such as Alberta 211, Honouring Life, and Recovery Colleges and Peer Support through the Canadian Mental Health Association. Initiatives such as these

help to increase access to critical supports and services. At the end of 2019, a new online eReferral Advice Request process through Alberta Netcare was established to enable primary care physicians to request advice from a Child and Adolescent Psychiatry clinical specialist to support timely patient care in the community. Primary care physicians play a key role in identifying and assessing mental health conditions that can impact Albertans of all ages, and in coordinating their care, including care provided by other physicians or other mental health care providers or supports.

On November 27, 2019, government announced a \$22 million investment to deliver more mental health supports for post-secondary students. Students at the 26 post-secondary institutions across Alberta will have more access to mental health services available through AHS. Additional supports for students include text and chat platforms, and professional counselling by phone through a partnership between Alberta 211 and Kids Help Phone.

Alberta 211 also provides referrals to mental health and addiction services across Alberta. An increasing number of mental health services and supports can be accessed online or by phone, which is particularly effective in improving access for those in rural and remote areas as well as during physical distancing measures implemented during the pandemic. Phone help and crisis lines have seen a surge during the pandemic, and demand is expected to continue in 2020-21.

Government is working with partners to develop a new digital navigation and crisis hub that will provide system navigation and seamless transfers to help lines such as the Kids Help Line and the family violence information line, and other digital crisis supports. This will ensure service navigation and crisis support will be available to all Albertans, regardless of where they live. The integrated nature of 211 and crisis support is critical to ensure Albertans receive the services that best meet their immediate needs.

According to the Centre for Suicide Prevention, in 2019 the rate of deaths by suicide in Alberta was 12.7 per 100,000 population. Certain groups in Alberta are disproportionately affected by suicide, including Indigenous communities and youth. The ministries of Health and Children's Services are continuing projects that aim to reduce the youth suicide rate by providing supports and services; training, education and awareness; and research, data and knowledge.

In September 2019, the Centre for Suicide Prevention announced the launch of a community-led resource toolkit to help develop, with ministry funding support, life promotion strategies and activities for Indigenous youth and communities. The government is also providing \$12.5 million over four years to AHS to expand Honouring Life funding to all interested First Nations and Métis Settlements to support resiliency, empowerment, and holistic suicide prevention initiatives. AHS is working with more than 50 Indigenous communities and organizations on program applications. More than 36 community projects are currently underway.

Published in September 2019, two new graphic novels developed for First Nations and Métis youth will help start conversations about suicide prevention. More than 100 Indigenous youth from across Alberta, in addition to First Nations and Métis producers, writers and artists, were engaged in the development of the novels.

In 2019-20, almost \$900,000 was invested to support Indigenous Albertans and communities in establishing a continuum of addiction and mental health services. As part of government's commitment to establish 4,000 new publicly funded addiction and mental health treatment beds, on December 4, 2019, the province announced \$1.4 million funding over three years to Poundmaker's Lodge Treatment Centres to add space to treat 900 more people over three years. These treatment centres blend Indigenous-healing methodologies with western practices in alignment with best and wise practice standards. The centres employ elders, addiction physicians, nurses, pharmacists, therapists, psychologists and other health professionals. The new funding will help create 28 new

residential addiction treatment beds and fund seven existing beds at the Iskwew Healing Lodge as well as upgrading five medically supported detoxification beds.

In 2019-20, \$6 million was dedicated to expand programs to support Albertans in crisis, including police and crisis teams, provincial family violence treatment programs, diversion programs, and by developing mental health court models.

Supporting Albertans in crisis will ensure that more people have access to early intervention to reduce the mental health impact of trauma. Diversion programs, drug treatment courts and mental health courts will also reduce the need for more costly criminal justice interventions by diverting Albertans to the appropriate addiction and mental health services.

4.3 Develop and implement opioid response strategies that support increased access to opioid treatment and recovery services for individuals and their families.

The opioid crisis is having an enormous impact on individuals, families, friends and communities. Reducing deaths related to opioid use requires a coordinated, recovery-oriented, public health response that spans prevention, harm reduction services, treatment and post-treatment supports. The government continues to monitor demands on the health care system from the opioid crisis, and substance-use more broadly, and is working with partners and civil society groups to meet and, where possible, reduce these demands through recovery supports and prevention efforts. In particular, the ministry is working with First Nations groups in Alberta to improve access to culturally sensitive treatment and recovery supports.

Budget 2019 included a commitment of \$10 million annually over four years to address the opioid crisis through programs such as a virtual opioid dependence program, training for primary care physicians, and improvement in addressing opioid addictions in emergency departments and corrections services. By September 2020, more Albertans will be able to access psychosocial supports via opioid dependency programs including addiction counsellors and therapists, and peer support programs. In addition, \$71 million over four years will fund publicly funded residential addiction treatment and mental health spaces and \$28 million will support the creation of new medically assisted detoxification spaces as part of the government's commitment to create more than 4,000 publicly funded spaces to support recovery from opioid addictions, with expanded spaces starting in summer 2020.

In Edmonton, as part of the Addiction and Mental Health Day Hospital initiative, patients can benefit from a therapeutic setting while being able to remain in their home. The ministry is reviewing needs and implementation options to ensure that home care service enhancements such as this will be most beneficial for Albertans.

Developmental Pathways (formerly called InRoads) are online resources that support health professionals providing addiction and mental health services in primary care and other settings. Care providers continue to access the learning modules to enhance skills in promotion, prevention, and harm reduction.

Work continues with physicians to ensure general practitioners have training in opioid replacement therapy. The College of Physicians and Surgeons of Alberta launched a course in mid-June 2019 to train its members on how to safely reduce the prescription of powerful opioid drugs that have a high incidence of addiction.

AHS' Virtual Opioid Dependency Program (VODP), accessible from anywhere in the province, enables same day access to begin medication, if appropriate, and offers transition supports for moves between emergency departments, detoxification centres, and corrections services. In

addition, by March 2020, 107 emergency departments across the province had implemented treatment and rapid follow-up in the community for clients with opioid use disorder. Sixty per cent of patients accessing this program were found to have continued with their treatment four months after treatment was initiated in the emergency department.

Beginning in 2019-20, Canadian Mental Health Association (CMHA)-Calgary received \$3 million in grant funding over three years to spread the successful model of recovery colleges and peer support to Alberta's five health zones. CMHA-Calgary is collaborating with communities to establish the required networks and infrastructure. Recovery college programs have been established in Calgary, Edmonton, Red Deer, Wood Buffalo, and Lethbridge.

On August 19, 2019, the Supervised Consumption Services (SCS) Review Committee was established to evaluate the social and economic impacts of current and proposed SCS sites. The committee heard from thousands of Albertans through public town halls and meetings with local leaders, front-line responders, community groups, business owners, harm reduction agencies and others impacted by this work. Input was also collected through online surveys and email submissions.

The final report, *Impact: A Socio-Economic Review Of Supervised Consumption Sites in Alberta*, released in March 2020, was informed by public input and pre-existing, third party data and research. The public report can be viewed online at www.alberta.ca/supervised-consumption-services-review.aspx.

The report provides valuable information about the issues and impacts SCS sites have on those living and working in the communities surrounding them. The report found that Albertans want their family, friends and community members struggling with addiction to be safe and have access to appropriate care. The government will make sure the path forward on this issue strikes an appropriate balance to not only protect communities, but ensure Albertans struggling with substance use disorder have access to a continuum of care, including recovery-oriented treatment services.

Some key findings of the report included inadequate oversight and a lack of accountability mechanisms, lack of focus on referrals to recovery oriented services, a need for improved needle debris and needle distribution policies, issues with erratic behaviour near the sites and inadequate community consultation prior to sites opening. Findings will inform the development of a comprehensive, long-term approach to protect communities and improve recovery-oriented supports for Albertans dealing with addiction. Government has extended grant funding for existing SCS and Opioid Prevention Services sites until September 30, 2020, while determining the best way to move forward on a city-by-city basis and improve consistency across all sites in 2020-21.

The opioid crisis has had a significant financial impact on all governments across Canada. Alberta's *Opioid Damages and Health Care Costs Recovery Act*, which received Royal Assent on December 5, 2019, will help Alberta hold opioid manufacturers and wholesalers accountable for opioid-related health-care costs and damages. The Act enables Alberta to participate in a proposed national opioid class action lawsuit.

Performance Metrics

Performance Indicator 4.a

Unplanned mental health readmissions to hospital

Prior Years' Results				2019-20 Target	2019-20 Actual
2015-16	2016-17	2017-18	2018-19		
8.6%	8.8%	8.8%	9.8%	8.6%	10.4% (April – December)

Source: Alberta Health Discharge Abstract Database (DAD).

This measure represents the proportion of non-elective (unplanned) readmissions to an acute care hospital in Alberta for selected mental illness within 30 days of a patient being discharged from a hospital stay for their mental illness. The measure excludes patients who have mental health disorders that require scheduled follow-up care.

Visits to facilities and programs not designated as acute inpatient care facilities (e.g., hospital emergency departments, urgent care centres, community clinics) are not included. The selected mental illnesses include: substance use disorders; schizophrenia, delusional, and non-organic psychotic disorders; mood/affective disorders; anxiety disorders; and, disorders of adult personality and behavior.

Results Analysis

Available results for 2019-20 show that 10.4 per cent of those patients previously discharged from a hospital stay for mental illness had an unplanned mental health readmission within 30 days. This is higher than prior years' results and almost two per cent higher than the target of 8.6 per cent.

Unplanned readmissions within 30 days for mental health patients is used as an indicator of continuity of care and accessibility of care in the community. It should be noted, however, that readmission may involve other external factors including the nature of the population served by the facility and the nature of addiction and mental health issues. Frequent hospitalizations may be an indication there are challenges with appropriateness of care, medication access and use, and community supports. Readmissions may indicate a lack of follow-up care or transition to care in the community.

While not all readmissions can be avoided, monitoring readmissions can assist in planning appropriate follow-up care after hospitalization. Hospital care for people diagnosed with a mental illness typically aims to stabilize acute symptoms. Once stabilized the individual can be discharged, and subsequent care and support to prevent relapse or complications are offered through primary care and outpatient and community programs.

The government is committed to improving care in the community, so Albertans have access to services closer to home. Going forward, government will develop a provincial mental health and addiction strategy that will improve access to care and continuity of care.

Performance Indicator 4.b**Emergency department visits where a mental health issue was identified for the first time in the past two years**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
31.0	28.7	27.5	26.2	25.4 (preliminary)

Source: Alberta Health Discharge Abstract Database (DAD), Alberta's physician claims database, Alberta Ambulatory Care Reporting System (AACRS), Provincial Population Registry.

This indicator tracks the number of number of mental health-related emergency department (ED) visits (per 100 mental health ED visits) where the patient had received no mental health services from a physician, or in a hospital or emergency department in the past two years.

Results Analysis

The current preliminary results show that just over a quarter of mental health ED visits (25.4 per 100 visits) were patients where their ED visit identified a mental health issue for the first time in the past two years. This is a slight improvement on the 2018-19 result (26.2 per 100 visits).

First identification of a mental health issue in the emergency department may indicate a lack of access to services available in the community, especially early intervention and screening. Government is investing in improving access to a range of mental health and addiction services and supports in the community. Implementation of actions arising from the new Mental Health and Addiction Strategy will support continued improvement in this indicator, by ensuring Albertans experiencing a mental health issue have options other than EDs to help them improve their mental health.

Performance Indicator 4.c**Emergency department visits due to alcohol use**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
959	948	939	941	928 (preliminary)

Source: Alberta Ambulatory Care database; Alberta Health Care Insurance Plan, End of Fiscal-Year (March 31) Adjusted Population Registry Files Statistics Canada; Canadian population, 2011.

This indicator provides the number of visits to emergency departments (EDs) and urgent care centres related to use of alcohol per 100,000 population. The population excludes members of the Armed Forces, RCMP, inmates in federal penitentiaries, or those who have opted out of the Alberta Health Care Insurance Plan.

Results Analysis

The preliminary 2019-20 result (928 ED visits due to alcohol per 100,000 population) shows a slight improvement when compared to the 2018-19 result (941 ED visits). It should be noted that the 2019-20 results are preliminary and are subject to change. It is possible that decreasing use of

alcohol among youth may be a contributing factor to the drop in ED visits. The most recent results from the Canadian Student Tobacco, Alcohol and Drug Survey among youth in grades 7-12 show that fewer youth were using alcohol in the past 12 months in 2018-19 (38.6%) compared to 2016-17 (46.8%). High risk drinking among students in grade 7-12 also decreased in 2018-19 to 23.4 per cent from 27.4 per cent in 2016-17.

Emergency department clinicians can use a standardized and consistent evidence-based care pathway to ensure patients receive the best treatment required for their condition. Albertans can receive help through the Addiction Helpline (1-866-332-2322), a toll free, confidential service that provides support for those experiencing problems with alcohol, tobacco, other drugs, or gambling, including information and referral to services. Government is committed to improving access to a range of mental health and addiction services and supports.

The ministry and the Alberta Gaming, Liquor and Cannabis agency work collaboratively to implement the province's Alcohol Strategy, which aims to prevent and reduce alcohol-related harm by developing a culture of moderation. The issue of alcohol use is complex and misuse impacts costs associated with health care, criminal justice and lost productivity.

Outcome Five: The well-being of Albertans is supported through population health initiatives

Outcome statement |

Healthy populations and communities are shaped through a range of social, economic and physical environmental factors, also known as the social determinants of health. The ministry will continue to work with its partners on the promotion of health and wellness to encourage Albertans to stay healthy through policies, programs and initiatives focused on reducing risks to prevent injury and disease.

Key Objectives

5.1 Prevent chronic conditions, injuries and infections by developing policies that reduce risk from environmental and individual risk factors.

AHS' public health programs provide health promotion as well as disease and injury prevention programs to individuals, high-risk groups and the broader community. These programs support health action from pre-conception through old age via a wide range of initiatives including health promotion education and support and population health surveillance for emerging public health concerns. Online resources can be accessed through the MyHealth Alberta website (myhealth.alberta.ca), which offers resources from AHS and Alberta-based partner organizations.

In the fall of 2019, the government initiated a review of Alberta's *Tobacco and Smoking Reduction Act* to assess the effectiveness of the current legislative framework and consider options to further protect Albertans from the harms of tobacco, vaping, and tobacco-like products. Current legislation does not explicitly address vaping, which is an emerging health concern. There is an ongoing rise in the use of these products, especially by young people. Alberta recorded its first case of severe vaping illness in January 2020 after the Public Health Agency of Canada alerted provincial health officials to this potential public health concern and asked provinces to report probable and confirmed cases as part of a national investigation underway.

Public feedback from the review was gathered via an online survey and through meetings with interest groups, including health experts, academics, educators, law enforcement and

businesses/manufacturers. Information on the review, including survey highlights and summary meeting notes are posted on the government website at www.alberta.ca/tobacco-and-smoking-reduction-review.aspx.

The Chronic Condition and Disease Prevention and Management (CCDPM) task group, under the direction of the Provincial Primary Care Network Committee, conducted a survey of the current state of CCDPM programs and services offered in Primary Care Networks across the province. Results of this project are under review to ensure Albertans are receiving the best care possible and up-to-date and consistent information about prevention and management of chronic conditions and diseases.

In 2019-20, \$1.4 million was provided to the Injury Prevention Centre (IPC) to support programs and services that work to reduce the incidence and severity of unintentional injury through prevention efforts. Funding also supported the IPC in administering the Alberta Community Injury Control Fund, which provides communities with resources to increase awareness of preventable injuries and improve the well-being of individuals. The IPC funded 21 projects, reaching 78 communities in Alberta, and focused on reducing unintentional injuries from motor vehicle collisions, falls, drowning, farming, fire, poisoning, or concussion.

Increased cancer prevention and screening efforts (as well as new treatments, technology and research) are helping more Albertans receive an earlier diagnosis, enter treatment sooner and live longer. There is more work to be done, and increased focus on innovative cancer prevention and screening services will help the ministry's ongoing cancer-related investments go further.

In 2019-20, the government invested \$2.74 million in Alberta's Tomorrow Project to support development of a database of ongoing health and lifestyle data for 55,000 Albertans, which is critical to determining why some people get chronic diseases like cancer. Evidence-informed cancer risk factor information, as well as other health information and resources are available through the MyHealth Alberta website. Albertans can also keep track of their screening appointments and results through the MyHealth Records portal (myhealth.alberta.ca/myhealthrecords), which was launched in March 2019.

Government released the *Alberta Air Quality Notification Protocol: What you need to know* to support Albertans during wildfire or other poor air quality events, and the *Alberta Safe Beach Protocol* is available to help beach operators keep recreational waters safe for Albertans.

The First Nations Cancer Prevention and Screening Practices program supports First Nations communities to develop, implement, and evaluate comprehensive prevention and screening plans. In the past year, three pilot communities continued implementing initiatives like traditional sweat ceremonies led by project elders, prevention and screening awareness events, and community walking groups.

5.2 Safeguard Albertans from communicable diseases through initiatives aimed at decreasing sexually transmitted and blood-borne infections, and through immunization for vaccine-preventable diseases, such as influenza and measles.

Requirements for reporting infectious diseases are mandated by Alberta's *Communicable Diseases Regulation*. The collection, analysis and interpretation of communicable disease data assists Alberta's Chief Medical Officer of Health in fulfilling the duties prescribed in the *Public Health Act*. The government monitors communicable disease information, and develops and maintains notifiable disease management guidelines for health professionals, with input to the guidelines from medical officers of health, public health nurses, public health inspectors, laboratories, and medical infectious disease specialists.

Immunization is one of the world's most successful and cost-effective mechanisms for preventing disease and protecting the health of the population. Immunization helps individuals develop immunity to those infectious pathogens (bacteria and viruses) that can cause severe disease and death. When people are immunized they protect not only themselves, but also help reduce the likelihood of outbreaks and epidemics which can severely affect more vulnerable people around them, like seniors and those with existing health conditions, as seen with influenza outbreaks. Alberta has one of the most comprehensive immunization programs across Canada and in 2019-20, the Government of Alberta spent \$51.25 million purchasing vaccines against 15 vaccine-preventable diseases (chicken pox, diphtheria, haemophilus influenza type B, hepatitis B, human papillomavirus, influenza, measles, meningococcal, mumps, pertussis, pneumococcal, polio, rotavirus, rubella and tetanus).

During the initial portion of the 2019-20 school year, AHS public health nurses offered immunization in the school setting to students in grades six and nine, as per Alberta's routine immunization schedule, and to children in grades one to nine with incomplete immunizations. In response to the COVID-19 pandemic, public health nurses and other staff were redeployed to assist in testing and contact tracing. Schools were closed to students beginning March 16, 2020. The immunization program is expected to resume, once public health staffing capacity has been restored and in-class learning resumes across Alberta.

An outbreak of pertussis (whooping cough) in communities in AHS' South Zone was declared on October 15, 2019. The outbreak of this vaccine-preventable disease was contained and declared over on February 6, 2020.

In the past number of years, sexually transmitted infections, such as chlamydia, gonorrhea, and syphilis have been an ongoing public health concern in Alberta. Alberta's Chief Medical Officer of Health declared a provincial syphilis outbreak in July 2019, due to a rapid increase in syphilis cases. A provincial outbreak coordination committee made up of provincial health officials was formed in response, and this work is ongoing. The committee developed a coordinated strategy and determined concrete actions to increase testing for sexually transmitted infections and follow-up, promote public awareness about prevention and treatment, and reduce the overall number of syphilis cases in Alberta, including cases of congenital syphilis in newborns.

5.3 Improve maternal, infant and child health by supporting initiatives that foster maternal-infant health and early childhood development.

Maternal, infant and child health care is delivered by physicians, midwives and AHS, either through facilities or through specific programs. These services are publicly funded through either direct funding to AHS or through the Alberta Health Care Insurance Plan. Maternal-infant health initiatives foster healthy birth outcomes, including healthy birth weights, Fetal Alcohol Spectrum Disorder prevention, and support for overall maternal and infant health.

The Maternal Newborn Child & Youth Strategic Clinical Network supports various initiatives including the antenatal care pathway, providing clinicians in rural communities with up-to-date information, standards of care and decision-making tools. An acute care neonatal abstinence syndrome pathway, which supports babies of mothers who have been using opioids and other drugs is nearing completion. The pathway guideline was completed by the end of 2019 and implementation planning and education resource development has commenced.

The Alberta Newborn Metabolic Screening Program provides timely screening and diagnosis to improve newborn health and save lives. Following a \$2 million investment, the screening panel was expanded from 17 to 21 conditions and includes screening for treatable conditions such as sickle cell

disease and cystic fibrosis. In 2018-19 (the most recent information available), the program successfully screened 99.4 per cent of Alberta-born infants.

Government also committed to increasing access to midwifery services, allowing this important option to be available to more women. In 2019-20, government, AHS and the Alberta Association of Midwives reached an agreement to increase the number of courses of care available each year, and to enable midwives to work to their full capacity of up to 40 courses of care annually. AHS has integrated midwifery services in all five of Alberta's health zones and continues to work with the Alberta Association of Midwives to align midwifery services to the needs and priorities of Albertans and maternity health service delivery, including in rural communities.

Access to maternity services for Indigenous, vulnerable, and rural populations remains a priority area for AHS provincially. Government increased funding to support vulnerable mothers and their babies to access prenatal and postnatal care and social supports. Programs operate in Medicine Hat, Grande Prairie, Edmonton, Calgary and Red Deer. In July 2019, a mobile health van started delivering health services and supports to rural and remote areas.

AHS' Merck for Mothers initiative is community-based programming to enhance the supports available to pregnant Indigenous women to overcome barriers to prenatal care. For example, in the Central Zone, Maskwacis focuses on culturally appropriate education programs for pregnant women and health care providers; the Pregnancy Pathways program in inner city Edmonton provides safe housing for pregnant Indigenous homeless women and support services, such as opportunities to attend traditional women's teachings; and, in the North Zone, Little Red River Cree Nation community relationships are strengthened through teepee teachings and community home visits.

Performance Metrics

Performance Measure 5.a

Percentage of seniors aged 65 or older and long-term care facility residents immunized for influenza

Population	Prior Years' Results				2019-20 Target	2019-20 Actual
	2015-16	2016-17	2017-18	2018-19		
Seniors aged 65 or older	63%	62%	60%	61%	66%	61%
Residents of long-term care facilities	90%	89%	89%	87%	93%	88%

Source: Alberta Health Services; Alberta Health's weekly pharmacists data; First Nations and Inuit Health Branch, Indigenous Services Canada, Alberta Region.

This performance measure tracks efforts to reach immunization targets for Albertans in high-risk groups, specifically seniors (aged 65 or over) and residents of long-term care facilities. Influenza immunization targets are set by the ministry and are based on national immunization targets as set by the National Immunization Strategy and agreed to by the Pan-Canadian Public Health Network Council. National targets are based on epidemiological evidence to decrease disease incidence and complications from disease.

Results Analysis

In 2019-20, 61 per cent of Alberta's seniors received immunization against influenza, which is the same percentage as in 2018-19 and is five per cent below the target (66%). In 2019-20, 88 per cent of residents in long-term care facilities received immunization, which is one percentage point above the 2018-19 result (87%) and is five per cent below the target (93%) for these Albertans.

Influenza has a significant impact on the health of Albertans and tends to be most severe among older Albertans, residents of long-term care facilities, infants, young children, and those with certain chronic medical conditions. More than 1.4 million doses of influenza vaccine were administered during the 2019-20 influenza season, covering 33 per cent of Alberta's population. This is the highest population-based influenza immunization rate achieved to date since the universal influenza program was established in 2009.

The 2020-21 Influenza Immunization Policy (IIP) has an added objective of increasing influenza immunization coverage rates for targeted populations, including individuals 65 years of age and older. Influenza immunization service delivery is shifting to prioritize high-risk populations, which is now defined in the 2020-21 IIP. Access to influenza vaccine continues to increase with over 1,400 pharmacies now offering provincially funded vaccine.

In order to better protect residents of long-term care, in 2020-21 the federal government is procuring an influenza high-dose vaccine for use in all long-term care facilities across Canada that should offer better protection for the most vulnerable Albertans.

The ministry is working with health practitioners in all health zones to ensure a consistent approach to reporting influenza immunization and adverse events following immunization to the Provincial Immunization Repository (Imm/ARI) as per the new reporting requirements outlined in Alberta's *Immunization Regulation*.

Performance Measure 5.b

Children by age two immunized for DTaP-IPV-Hib (diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b) and for MMRV (measles, mumps, rubella, varicella)

Immunization	Prior Years' Results				2019-20 Target	2019 Actual
	2015	2016	2017	2018		
Diphtheria, tetanus, pertussis, polio, Hib	76%	77%	77%	78%	95% ¹	79%
Measles, mumps, rubella, varicella	88%	88%	88%	87%	95% ¹	88%

Source: Alberta Health Care Insurance Plan (AHCIP) Quarterly Population Registries; Immunization/Adverse Reactions to Immunization (Imm/ARI); Alberta Vital Statistics, Birth Files.

¹National targets.

This indicator reports the percentage of children in Alberta who by two years of age have received the required immunization for the following vaccine preventable diseases: diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b, measles, mumps, rubella and varicella.

Results Analysis

This performance indicator shows efforts towards protecting children from a number of vaccine preventable diseases, some of which can be fatal or produce permanent disabilities. A high rate of immunization reduces the incidence of these diseases and serves to control outbreaks.

In 2019, by age two, 79 per cent of Albertans had received immunization with DTaP-IPV-Hib vaccine and 88 percent had received immunization with MMRV vaccine. These immunization rates are both lower than the national target of 95 per cent for both vaccines.

Provincial immunization rates for both vaccines for this age group of children have remained stable over the past five years. Vaccine hesitancy can contribute to lower rates of immunization and is related to a general lack of understanding about vaccines, perceived risk of serious adverse events related to injections, and lack of appreciation for the severity of vaccine preventable diseases. Accessing vaccine services can also be a barrier.

AHS has implemented five of the seven recommendations of the Canadian Pediatric Society to increase immunization levels, including the development of a provincial immunization registry, proactive reminders for parents, enhanced immunization clinic hours, and an augmented school-based immunization program.

The ministry is working with health practitioners in all health zones to ensure a consistent approach to disease outbreak reporting, notification, and management.

Performance Indicator 5.c

Infant mortality rate per 1,000 live births for First Nations people, compared to non-First Nations

Infant Population ¹	Prior Years' Results				2019 Actual
	2015	2016	2017	2018	
First Nations	8.3	7.8	9.2	13.8	9.1
Non-First Nations	4.2	3.7	4.4	4.4	3.8

Sources: Alberta Vital Statistics Death File (infant deaths); Newborn Metabolic Screening Database (live births); First Nations Status Registry.

¹Infants less than one year of age.

This indicator compares the infant mortality rate for the First Nations population to that of the non-First Nations population in Alberta. The infant mortality rate provides the rate of death of children less than one year of age, per 1,000 live births. Infant mortality is often used as an indicator to measure the health status of a general population, because factors affecting the health of entire populations can also impact the mortality rate of infants.

Results Analysis

There is a significant gap in the infant mortality rate for the First Nations population in Alberta in comparison to the non-First Nations population. Infant mortality rates are based on relatively small numbers that result in annual fluctuations, making trends over time difficult to interpret. Compared to 2018, the infant mortality rate for First Nations people is lower in 2019; however, it remains higher compared to 2015 and 2016, while the rate for non-First Nations people has remained relatively consistent.

Rates of death among infants under one year of age are associated with premature birth, lower birth weight, and injury. These outcomes may be impacted by maternal health, and may be driven by socio-environmental factors such as income, housing, education, and employment conditions. Importantly, for First Nations people, infant mortality rates may be influenced by a complex history that includes intergenerational trauma from residential schools, higher rates of poverty, and systemic racism.

To address the health status gaps between First Nations and non-First Nations populations in Alberta, the ministry is committed to respectfully engaging with Indigenous leadership, communities, and peoples in the design, delivery and stewardship of health services. Engagement and coordination processes with Provincial Indigenous Organizations including the Métis Nation of Alberta, Metis Settlements General Council, Confederacy of Treaty 6, Treaty 8 First Nations of Alberta, Blackfoot Confederacy and the Stoney Nakoda Tsuut'ina Tribal Council build upon the foundation of reconciliation and relationships.

The ministry continues to work in partnership with Indigenous people and federal and provincial partners to strengthen health services for Indigenous people in our province, and is supporting key initiatives aimed at reducing disparities between Indigenous and non-Indigenous Albertans, including supporting expectant Indigenous mothers.

Performance Indicator 5.d **Percentage of Albertans who smoke cigarettes**

Prior Years' Results				2019-20 Actual
2015-16	2016-17	2017-18	2018-19	
16.4%	16.6%	15.3%	15.5%	15.0% (preliminary)

Source: Alberta Community Health Survey

This indicator tracks the percentage of Albertans aged 18 or older who took part in the Alberta Community Health Survey and indicated that they smoke cigarettes daily or occasionally at the present time.

Results Analysis

The preliminary survey results for 2019-20 show that 15 per cent of Albertans aged 18 or older currently smoke cigarettes daily or occasionally. This is less than one percentage point drop when compared to the previous two years. The desired result is to decrease the percentage of Albertans who smoke cigarettes daily or occasionally.

The government recognizes the health harms of cigarette smoking and tobacco products. In addition, the increase in vaping, particularly by youth, is a significant emerging health concern. The review of Alberta's *Tobacco and Smoking Reduction Act*, which commenced in 2019, provided government with an opportunity to assess the effectiveness of the current legislative framework and consider options to further protect Albertans.

Public education remains an important tool to discourage the use of tobacco and vaping products. A variety of information, tips, tools and resources are available through the AlbertaQuits website (www.albertaquits.ca) to raise awareness about smoking and vaping, and to support a decrease in the percentage of Albertans who smoke cigarettes daily or occasionally. These supports are particularly important during the pandemic because infected individuals who smoke or vape are at higher risk of developing more severe COVID-19 symptoms.

Performance Measure and Indicator Methodology

Performance Indicator 1.a:

Unplanned medical readmissions to hospital within 30 days of discharge

Methodology

Numerator: Total number of medical patients with unplanned readmission to hospital within 30 days of discharge.

- Inclusion Criteria: Residents of Alberta covered by the Alberta Health Care Insurance Plan. Admission day of subsequent readmission is less than or equal to 30 days of initial discharge date from an acute care hospital.
- Exclusion Criteria: Transfers (admitted within 6 hours of discharge from another hospital, or 6-12 hours after transfer from or to an acute care facility); newborn, still birth, cadaver admissions; non-acute care admissions; pediatric (less than 20 years of age), surgery, obstetrics, palliative care, mental health admissions; hospital admissions for cancer therapy; non-urgent, planned admissions.

Denominator: Total number of medical patients discharged.

- Inclusion Criteria: Residents of Alberta covered by the Alberta Health Care Insurance Plan; discharge from an acute care hospital, within a reporting period.
- Exclusion Criteria: Admissions where patients died in hospital; admissions from March 2 to March 31; transfers (admitted within 6 hours of discharge from another hospital, or 6-12 hours after transfer from or to an acute care facility); newborn, still birth, cadaver admissions; non-acute care admissions; pediatric (less than 20 years of age), surgery, obstetrics, palliative care, mental health admission.

$$\text{Hospital Readmission Percentage (unadjusted) = } \frac{\text{Number of discharged patients readmitted to hospital within 30 days of index discharge}}{\text{Number of patients discharged}} \times 100$$

Risk Adjustment: Accounts for differences in patient characteristics that may vary over years. Based on the list of risk factors (e.g. age, sex, case mixing grouping, Charlson comorbidity score) published by the Canadian Institute for Health Information, Alberta built provincial specific logistic regression model using past five-year data to estimate the Alberta average and expected readmissions based on patients' risk profile.

The risk-adjusted rate is calculated using the following formula:

$$\text{Risk-adjusted rate = } \frac{\text{observed cases}}{\text{expected cases}} \times \text{Alberta rate}$$

Source

Canadian Institute for Health Information (CIHI); Alberta Health Discharge Abstract Database (DAD); Hospital Morbidity Database (HMDDB); National Ambulatory Care Reporting System (NACRS).

Performance Indicator 1.b**Median number of days hospital stay extended until home care services or supports were ready****Methodology**

The indicator measures the median (50th percentile) number of days patients spend in an inpatient acute care hospital bed when they don't need acute care, also known as alternate level of care (ALC), before they are discharged to home care.

- Inclusion Criteria: Records from acute care hospitals in Alberta; patients discharged to home or a home setting with home care services available to the patient; sex recorded as male or female; and, records with a valid ALC length of stay greater than 0.
- Exclusion Criteria: Records with admission category of cadaveric donor or stillbirth.

Median # days = The mid-point on the distribution curve of the # days spent in hospital in ALC before discharged with home care available.

Source

Alberta Health Discharge Abstract Database (DAD).

Performance Indicator 1.c**Ambulatory Care Sensitive Condition hospitalization rate****Methodology**

Numerator: Total number of acute care hospitalizations for patients under 75 years of age for ambulatory care sensitive conditions (ACSCs).

- **Inclusion Criteria:** Any most responsible diagnosis code relating to grand mal status and other epileptic convulsions; chronic obstructive pulmonary diseases (COPD); acute lower respiratory infection, only when there is a secondary diagnosis of J44; asthma; heart failure and pulmonary edema; hypertension; angina; and, diabetes; and, gender specified as male or female. Age is determined as of discharge date. Only people deemed as “Alberta resident” (as defined by the Ministry of Health) will be included. This includes people that are homeless and people from Lloydminster, discharged from an acute care hospital.
- **Exclusion Criteria:** Individuals 75 years of age or older; deaths; stillbirths and newborns; cases with a specified cardiac procedure; and, records where the fiscal year was omitted.

Denominator: The total end-of-fiscal year population younger than age 75. The population is age-adjusted using the 2011 Canada Census.

The overall age-standardized rate is obtained by first calculating crude rates for each age group multiplied by its corresponding age groups weight (derived from the 2011 Canadian Census). The sum across all age groups for the year is the age-standardized ACSC.

$$\text{Hospitalization rate} = \frac{\text{total \# of acute care hospitalizations for ACSCs under age 75}}{\text{total end-of-fiscal year population under age 75 years}} \times 100,000$$

Source

Numerator: Alberta Health Discharge Abstract Database (DAD); Denominator: Alberta Health population estimate.

Performance Indicator 1.d

Wait times for surgical procedures

Methodology

Ready-to-treat (RTT) Wait time (in days) is calculated for each relevant record meeting specified inclusion/exclusion criteria.

- Inclusion Criteria: All elective hip, knee replacement and cataract surgeries based on surgery procedure catalogue descriptions. All urgency levels.
- Exclusion Criteria: Persons who received emergency surgical care. Cases with an invalid RTT date. When cataract surgery is required for a patient's both eyes, only the wait time for surgery on the first eye is included in the wait time calculations.

RTT Date = The date that the surgeon determines the patient is ready for the surgical intervention. The RTT Date excludes patient delays or voluntary waits.

Treatment Date = The date the treatment (surgical procedure) took place.

RTT Wait time = Treatment (surgical procedure) Date minus RTT Date

National RTT wait time benchmarks:

- Hip replacement national benchmark is 182 days
- Knee replacement national benchmark is 182 days
- Cataract surgery national benchmark (for first eye) is 112 days.

$$\text{Percentage} = \frac{\text{\# of cases completed with a RTT wait time less than or equal to the national RTT wait time benchmark}}{\text{\# of completed cases with valid RTT dates}} \times 100$$

Source

Alberta Health Services. Multiple Operating Room Systems across Alberta (PICIS, Meditech, VAX, and Manual Data Collection).

Performance Indicator 1.e**Percentage of scheduled surgeries performed in chartered surgical facilities (formerly known as non-hospital surgical facilities)**

Methodology	
Numerator: Non-emergency Main Operating Room cases completed in chartered surgical facilities (CSFs).	
Denominator: Non-emergency Main Operating Room cases completed in CSFs and hospitals .	
Emergency surgeries and pregnancy terminations are excluded from the calculation.	
Percentage = $\frac{\text{\# surgeries performed in CSFs under contract with AHS}}{\text{total \# surgeries in CSFs and hospitals from April 1 to Mar 31}} \times 100$	
Source	
Alberta Health Services Discharge Abstract Database (DAD); National Ambulatory Care Reporting System (NACRS).	

Performance Indicator 2.a**Provincial per capita spending on health care**

Methodology	
Health expenditure includes any type of expenditure for which the primary objective is to improve or prevent the deterioration of health status.	
Data is extracted annually from provincial and territorial government public accounts. Programs and/or program items are classified into health expenditure categories according to accepted and standardized methods and definitions used in estimating national health expenditure. Data from the public accounts is supplemented with information from provincial and territorial government department annual reports and annual statistical reports when available, as well as information provided by provincial and territorial government department officials.	
Adjustments for regional health authority and/or hospital deficits or surpluses are not made in the National Health Expenditures Trends (NHEX) report unless the provincial government assumes them. If deficits or surpluses are assumed by the provincial government, they are allocated to the years when the regional health authority and/or hospitals accumulated them.	
As part of the preparation of the NHEX report, the Canadian Institute for Health Information estimates of provincial and territorial government health expenditures were submitted to provincial and territorial departments of health for review.	
Per capita provincial government health expenditure = $\frac{\text{provincial government health expenditure}}{\text{population estimates}}$	
Source	
Canadian Institute for Health Information (CIHI), National Health Expenditures Trends (NHEX), 1975 to 2019. Statistics Canada, Demography Division (population estimates).	

Performance Indicator 2.b

Cost of standard hospital stay

Methodology

The Cost of standard hospital stay (CSHS) is constructed as a ratio. CSHS is comparable across provinces since data for the Canadian MIS Database (CMDB) and Discharge Abstract Database (DAD) are submitted by provinces according to national standards.

Numerator: Total inpatient costs captured from all areas of a hospital. As a full-cost indicator, the CSHS includes not only direct expenses incurred in the provision of care (e.g., nursing compensation, drugs, meals, etc) but also the indirect operational expenses (e.g., finance, administrative services). The costs are retrieved from the CMDB, which contains financial and statistical operations information from hospitals and regional health authorities across Canada.

Note: The CSHS calculation excludes physician compensation. Physician compensation within the hospital environment is treated and reported differently across jurisdictions due to varying provincial and territorial policies. It has been removed from the CSHS calculation for better comparability across jurisdictions.

Denominator: The sum of all acute care weighted cases, or Resource Intensity Weights (RIWs). To arrive at this number, each inpatient case is weighted by an RIW factor indicating the expected resource intensity of the stay. All RIWs are relative to the average typical inpatient case, which is assigned an RIW of 1.0. For example, a patient with an RIW of 2.0 would be expected to require twice as many resources during his or her hospital stay as the average typical inpatient. The RIW calculated for each stay takes into account a patient's age, disease status, comorbidities, and types of interventions received. Data required for the denominator is retrieved from the DAD, which captures administrative, clinical and demographic information about patients when they are discharged from the hospital.

$$\text{Cost of a standard hospital stay} = \frac{\text{Total inpatient costs}}{\text{Total acute inpatient weighted cases}}$$

Source

Canadian Institute for Health Information (CIHI): Canadian MIS Database (CMDB) and Alberta Health Discharge Abstract Database (DAD).

Performance Indicator 2.c

Alternate level of care days

Methodology

Alternative Level of Care (ALC) days are recorded at any point during the hospital stay for patients covered by the publicly funded Alberta Health Care Insurance Plan.

- Inclusion Criteria: Records from acute care hospitals in Alberta; sex recorded as male or female; and, records with a valid ALC length of stay greater than 0.
- Exclusion Criteria: Records with admission category of cadaveric donor or stillbirth.

$$\text{Percentage} = \frac{\text{\# of days as an inpatient in hospital classified as ALC days}}{\text{total \# of inpatient days in hospital}} \times 100$$

Source

Alberta Health Discharge Abstract Database (DAD).

Performance Indicator 2.d

Facility-based beds in community settings

Methodology

Facility-based beds in the community as a proportion of all beds.

Numerator: Number of facility-based beds in a community setting.

- Inclusion Criteria:
 - Long-Term Care (Auxiliary and Nursing Home)
 - Designated Supportive Living (DSL3, DSL4, DSL4D)
 - Community Palliative and Hospice (outside hospitals)
 - Sub-Acute in Long-Term Care (Auxiliary Hospitals)
 - Addiction and Mental Health (Addiction Treatment, Community Mental Health, stand-alone psychiatric facilities)

Denominator: Number of facility-based beds in a community setting and number of beds in acute care hospitals.

$$\text{Percentage} = \frac{\text{\# facility-based beds in a community setting}}{\text{\# facility-based beds in a community setting} + \text{\# acute care beds}} \times 100$$

Source

Alberta Health Services.

Performance Indicator 2.e

Annual rate of operational expenditures

Methodology	
Calculation of percentage growth in ministry operational expenses when compared to operational expenses in the previous fiscal year.	
Operational expenses is defined as the Ministry of Health total expenses per published Statement of Revenue and Expenses less infrastructure support.	
Percentage =	$\frac{\text{total 2019/20 operational expenses} - \text{total 2018/19 operational expenses}}{\text{total 2018/19 operational expenses}} \times 100$
Source	
Ministry Consolidated Statement of Revenue and Expenses as presented in the Financial Information section of the Ministry of Health Annual Report.	

Performance Indicator 3.a

Percentage of Alternative Relationship Plan payments of total physician payments

Methodology	
Alternative Relationship Plans (ARPs) are arrangements to compensate physicians by methods other than fee-for-service. Information comes from the Canadian Institute for Health Information – National Physician Database (NPDB). NPDB data is derived from physicians' billings, including fee codes, which provincial and territorial medicare programs submit.	
Claims data and associated physician data are submitted quarterly, usually within six months of the end of the fiscal quarter. The ARP payment data is derived from payments made to ARP programs through the Claims Assessment System in the specified fiscal year.	
Percentage =	$\frac{\$ \text{physician payments made under an ARP}}{\$ \text{physician payments}} \times 100$
Source	
Canadian Institute for Health Information (CIHI) - National Physician Database (NPDB); Alternative Payment Information File for Alberta.	

Performance Indicator 3.b**Percentage of Albertans who had access to a regular health care provider****Methodology**

Statistics Canada's Canadian Community Health Survey (CCHS) is a cross-sectional survey that collects information related to health status, health care utilization, and health determinants for the Canadian Population (including regional and provincial levels).

- Inclusion Criteria: Population age 12 and older.
- Exclusion Criteria: Persons living on Indigenous reserves or settlements, full-time members of the Canadian Forces, institutionalized population (i.e., those who live in an institutional collective dwelling, such as a hospital, a nursing home or a prison; residents under care or custody, such as patients or inmates), and children age 12-17 living in foster care.

Population estimates are based on weighted survey responses to reflect the total population. The results of the survey are based on a 95% confidence interval.

$$\text{Percentage} = \frac{\text{\# survey respondents who reported having a regular health care provider}}{\text{\# survey respondents}} \times 100$$

Source

Canadian Community Health Survey, Statistics Canada.

Performance Indicator 3.c**Percentage of Licensed Practical Nurses relative to Registered Nurses****Methodology**

Prior to 2019, data pertaining to registered nurses (RNs) reported by the Canadian Institute for Health Information (CIHI) included Nurse Practitioners (NPs). In the 'Nursing in Canada, 2019 data tables', CIHI separated NP counts from RN counts. In calculation of the percentage of licensed practical nurses (LPNs) relative to RNs, the current methodology excludes NPs.

Nurse supply refers to those nurses who are eligible to practice in the given year, including those employed and those not employed at the time of registration. Note that secondary registrants (also known as interprovincial duplicates) are excluded from provincial nurse supply counts.

$$\text{Percentage} = \frac{\text{\# LPN supply}}{\text{\# RN supply} + \text{\# registered psychiatric nurse supply}} \times 100$$

Source

Canadian Institute for Health Information (CIHI).

Performance Indicator 3.d**Percentage of Nurse Practitioners relative to Family Medicine Physicians****Methodology**

Numerator: Nurse Practitioner (NP) supply refers to all NPs who are eligible to practice in the given year, including those employed and those not employed at the time of registration. Note that secondary registrants (also known as interprovincial duplicates) are excluded from provincial NP supply counts.

Denominator: Family medicine physicians include general practitioners, as well as family medicine specialists and emergency family medicine specialists who are certificants of the College of Family Physicians of Canada.

$$\text{Percentage} = \frac{\text{\# nurse practitioner supply}}{\text{\# family medicine physicians}} \times 100$$

Source

Canadian Institute for Health Information (CIHI).

Performance Indicator 3.e**Average wait time from general practitioner to orthopedic surgeon consult****Methodology**

Referrals and surgeon consult records are transmitted from participating surgeon office electronic medical record system to Alberta Bone and Joint Health Institute monthly.

$$\text{Average wait time} = \frac{\text{Number of weeks from the day a surgeon's office receives the referral to the patient's first appointment with the orthopedic surgeon}}{\text{Number of first appointments completed by orthopedic surgeons}}$$

Source

Alberta Bone and Joint Health Institute.

Performance Indicator 4.a

Unplanned mental health readmissions to hospital

Methodology

The patient population is aged 15 years or older.

The unit of analysis is an inpatient encounter within a single acute inpatient facility. Discharges to transfer between acute inpatient facilities are excluded, although the discharge from the final facility after transfers is included. In this way, episodes of care are identified with the reporting facility identified as the final discharging facility.

Readmission rate reporting always lags by a fiscal year quarter. Information is available once data from the Discharge Abstract Database (DAD) is collected by all facilities in the province and loaded into the provincial database. Enforced reporting lag is applied (90 days) to allow for completion of stay and load of the abstract record for the readmission stay.

Readmission rates are attributed to the fiscal year and quarter in which a patient is originally discharged from an acute care hospital. This requires that patients be tracked for 30 days after the end of the quarter to allow sufficient time from the date of initial discharge to determine whether a readmission will occur.

Since transfer is excluded from readmission and there are several non-standardized ways to determine whether a transfer has occurred, the readmission rates published elsewhere could differ. Since there is not a standard method to identify unplanned readmissions (e.g., admissions through emergency ambulatory care), readmission rates published elsewhere may differ. Unplanned admission is defined as admit category 'U' which is urgent or emergent admission. The data reliability is highly dependent on the accuracy of this field.

Crude Readmission Rate (CRR):

$$\text{CRR} = \frac{\text{\# discharged patients readmitted within 30 days of index discharge}}{\text{\# index hospital discharges with MRDx as selected mental illness}} \times 100$$

Risk Adjusted Rate (RAR):

The observed number of cases is the actual count of readmissions to a hospital. The expected number of cases is based on the sum of the probabilities of readmissions to a hospital. Coefficients from the Canadian Institute for Health Information used for calculating the probability of readmissions were from logistic regression models on the following independent variables – age, sex, multiple previous admissions for a selected mental illness (two or more) during the past 12 months, discharges against medical advice, substance abuse related disorder, schizophrenia, anxiety disorder, and personality disorder.

$$\text{RAR} = \frac{\text{\# of observed cases}}{\text{\# of expected cases}} \times \text{the Canadian CRR}$$

Source

Alberta Health Discharge Abstract Database (DAD).

Performance Indicator 4.b

Emergency department visits where a mental health issue was identified for the first time in the past two years.

Methodology

This indicator examines first-contact visits to an emergency department (ED) for mental health conditions. The calculation includes the number of mental health related ED visits (per 100 mental health ED visits) for those patients, aged 15 or older, where the patient had received no mental health services from a physician, or in a hospital or emergency department in the preceding two years.

Data is sourced from ministry databases pertaining to physician billing claims data and hospital discharge abstracts, and the National Ambulatory Care Reporting System. Current fiscal year results are reported as preliminary to allow for all provincial reporting to be completed.

$$\text{\# visits (per 100 mental health ED visits)} = \frac{\text{\# individuals treated in an ED for a mental health reason for the first time in two years}}{\text{\# individuals treated in an ED for a mental health reason}} \times 100$$

Source

Alberta Health Discharge Abstract Database (DAD), Alberta Ambulatory Care Reporting System (AACRS), Alberta physician claims database, Provincial Population Registry.

Performance Indicator 4.c
Emergency department visits due to alcohol use

Methodology	
This indicator provides the age-standardized rate of visits to emergency departments and urgent care centres related to use of alcohol. Emergency visits are any hospital discharges beginning with any of the following MIS codes: 71310 (Ambulatory Care Services described as emergency), 71513 (Community Urgent Care Centre), and 71514 (Community Advanced Ambulatory Care Centre). A discharge or emergency visit occurs when a patient leaves the hospital – by death, transfer to another facility, discharge to home, or against medical advice. Only Alberta residents are included in the numerator. Emergency visit rates based on all diagnostic fields were used. The following ICD10 codes were selected: F10 (mental and behavioral disorders due to use of alcohol) and T51 (toxic effects due to alcohol). The date of birth in the fiscal year population registry file is used to calculate the age of the individual as of March 31 each year. The population excludes members of the Canadian Armed Forces, RCMP, inmates in federal penitentiaries, and those who have opted out of the Alberta Health Care Insurance Plan.	
Age-standardized rate of visits: $\text{Rate} = \sum_{i = \text{first age group}}^{\text{last age group}} \frac{\text{\# ED visits (alcohol) in age group}}{\text{Alberta pop'n in that age group}} \times \frac{\text{reference pop'n in age group}}{\text{total reference population}} \times 100,000$	
Source	
Alberta Ambulatory Care database; Alberta Health Care Insurance Plan, End-of-fiscal year (March 31) adjusted Population Registry Files Statistics Canada; Canadian population, 2011.	

Performance Indicator 5.a**Percentage seniors aged 65 or older and long-term care facility residents immunized for influenza.**

Methodology	
Seniors aged 65 or over:	
Immunization rate =	$\frac{\text{\# of seniors aged 65 or over who received one dose of the influenza vaccine}}{\text{mid-year population estimate of age category}} \times 100$
Residents of long-term care (LTC) facilities:	
Immunization rate =	$\frac{\text{\# of LTC residents on Dec 15, 2019 who received one vaccine dose during the period October 1 – December 15, 2019}}{\text{\# of LTC residents on Dec 15, 2019}} \times 100$
It is necessary to define the number of residents of long-term care facilities on December 15 each year, due to the high turnover in this population. Otherwise, the result would be an immunization rate over 100 per cent.	
Source	
Numerator: Number of individuals immunized by age category: Alberta Health Services zones; Alberta Health's weekly pharmacists data; First Nations and Inuit Health Branch, Indigenous Services Canada, Alberta Region.	
Denominator: For seniors, the denominator is the ministry's population estimates, based on mid-year registration population estimates. For residents of long-term care facilities the denominator is the number of residents as of December 15, 2019 provided by Alberta Health Services.	

Performance Indicator 5.b**Children by age two immunized for DTaP-IPV-Hib (diphtheria, tetanus, pertussis (whooping cough), polio, Haemophilus influenza type b), and for MMRV (measles, mumps, rubella, varicella)**

Methodology
Using data from the Alberta Health Care Insurance Plan population registries, children in Alberta are followed through time (i.e., from date of birth to study end date). Exclusions include individuals leaving Alberta, individuals who died, individuals who do not belong to the study period, First Nations individuals, and residents of Lloydminster.
Coverage rates are based on a birth cohort and reported at age two. Once established the population-based birth cohort is linked to Imm/ARI using the Unique Lifetime Identifier to get immunization information.
Calculation: Childhood immunization coverage is calculated using a survival analysis (time-to-immunization) method based on the specified population-based birth cohort. The analysis measures the probability that the child will receive required vaccines by age two.
Source
Alberta Health Care Insurance Plan (AHCIP) Quarterly Population Registries; Immunization/Adverse Reactions to Immunization (Imm/ARI), Alberta Vital Statistics, Birth Files.

Performance Indicator 5.c**Infant mortality rate per 1000 live births for First Nations people, compared to non-First Nations**

Methodology	
Infant deaths are identified from the Alberta Vital Statistics Death file, while live births are identified from the Newborn Metabolic Screening Database. Infants are defined as less than one year of age (birth to 364 days).	
Infant mortality rate =	$\frac{\text{\# of infant deaths during a calendar year}}{\text{\# of live births during a calendar year}} \times 1,000$
Source	
Alberta Vital Statistics Death File (infant deaths); Newborn Metabolic Screening Database (live births); First Nations Status Registry.	

Performance Indicator 5.d**Percentage of Albertans who smoke cigarettes**

Methodology	
The Alberta Community Health Survey is a telephone based survey aimed to collect data on specific determinants of health among Albertans 18 years and older.	
The smoking indicator is based on the following survey question: "At the present time, do you smoke cigarettes: i) Daily, ii) Occasionally, or iii) not at all"?	
Sampling Methodology: Goal of sampling frame is to have high-quality geographically representative estimates	
• Telephone (cell phone and landed line) or web based survey among Albertans 18 years and older • Cell phone sampling with geographic specificity began halfway through the 2014 cycle • A web-based survey (Jan 2017 onwards) was available to those who did not elect to do the telephone survey • Sample size in 2015-2016 to 2019-20 was 7152, 6410, 6474, 6716 and 3191 (preliminary), respectively.	
The percentage of cigarette smokers is estimated by tabulating a weighted proportion of those that responded as daily or occasional cigarette smokers and based on a confidence interval of 95%.	
Geographic assignment is based on the postal code of residence reported by the respondent. The geographic areas are obtained by linking the postal code with the Postal Code Translator File.	
Smoking rate =	$\frac{\text{weighted total of daily or occasional cigarette smokers}}{\text{weighted total (all responses)}} \times 100$
Source	
Alberta Community Health Survey (ACHS).	

Financial Information

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Reporting Entity and Method Consolidation

The consolidated ministry financial information is prepared in accordance with Canadian Public Sector Accounting Standards.

The reporting entity is the Ministry of Health, for which the Minister of Health is accountable. The accounts of the Department of Health are fully consolidated with Alberta Health Services (AHS) and Health Quality Council of Alberta on a line-by-line basis.

Accounts of entities that are consolidated by AHS are listed in note 2a (i) and (ii) of AHS consolidated financial statements.

Under the line-by-line basis, accounting policies of the consolidated entities are adjusted to conform to government accounting policies and the results of each line item in their financial statements (revenue, expense, assets, and liabilities) are included in government's results. Revenue and expense, capital, investing and financing transactions and related asset and liability balances between the consolidated entities have been eliminated.

Ministry of Health

Ministry Financial Highlights

Statement of Revenues and Expenses (unaudited)

Year ended March 31, 2020

(in thousands)

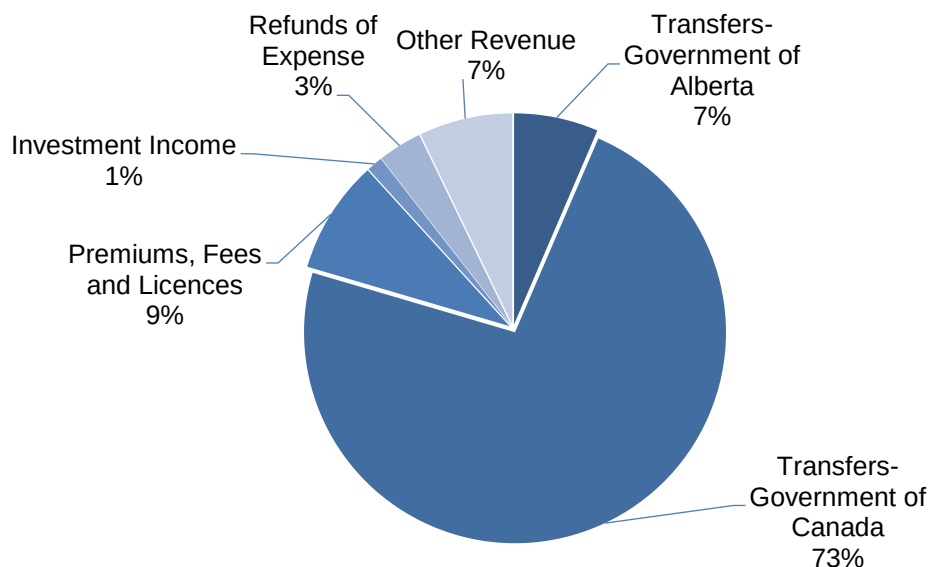
	2020		2019	Change from	
	Budget	Actual	Actual (Restated)	Budget	2019 Actual
Revenues					
Government Transfers					
Government of Alberta	\$ 417,000	\$ 425,845	\$ 420,622	\$ 8,845	\$ 5,223
Government of Canada	4,840,370	4,815,833	4,569,848	(24,537)	245,985
Premiums, Fees and Licences	554,001	572,658	580,542	18,657	(7,884)
Investment Income	65,020	80,269	68,544	15,249	11,725
Refunds of Expense	169,105	222,414	201,826	53,309	20,588
Other Revenue	410,922	472,103	457,080	61,181	15,023
Ministry Total	6,456,418	6,589,122	6,298,462	132,704	290,660
Inter-Ministry Consolidation Adjustments	(447,800)	(458,438)	(453,626)	(10,638)	(4,812)
Adjusted Ministry Total	6,008,618	6,130,684	5,844,836	122,066	285,848
Expenses - Directly Incurred					
Ministry Support Services	61,803	61,297	60,349	(506)	948
Physician Compensation and Development	5,431,717	5,456,359	5,365,361	24,642	90,998
Drugs and Supplemental Health Benefits	2,250,009	2,335,872	2,202,674	85,863	133,198
Population and Public Health	690,514	640,212	624,831	(50,302)	15,381
Acute Care	4,051,171	4,150,743	4,138,866	99,572	11,877
Continuing Care	1,145,000	1,163,433	1,126,904	18,433	36,529
Ambulance Services	514,000	527,041	525,530	13,041	1,511
Community Care	1,475,000	1,475,178	1,417,257	178	57,921
Home Care	688,000	716,023	688,040	28,023	27,983
Diagnostic, Therapeutic & Other Patient Services	2,451,569	2,493,967	2,475,531	42,398	18,436
Administration	537,533	492,031	542,397	(45,502)	(50,366)
Support Services	2,245,000	2,267,016	2,241,441	22,016	25,575
Information Technology	664,473	660,838	584,459	(3,635)	76,379
Research and Education	97,579	127,253	137,941	29,674	(10,688)
Debt Servicing	16,000	14,755	15,353	(1,245)	(598)
Infrastructure Support	33,026	9,727	44,360	(23,299)	(34,633)
Cancer Research and Prevention Investment	12,390	2,449	6,567	(9,941)	(4,118)
COVID-19 Pandemic Response	-	25,837	-	25,837	25,837
Ministry Total	22,364,784	22,620,031	22,197,861	255,247	422,170
Inter-Ministry Consolidation Adjustments	(260,108)	(276,146)	(277,663)	(16,038)	1,517
Adjusted Ministry Total	22,104,676	22,343,885	21,920,198	239,209	423,687
Adjusted Annual Deficit	\$ (16,096,058)	\$ (16,213,201)	\$ (16,075,362)	\$ (117,143)	\$ (137,839)

Ministry of Health

Revenue and Expense Highlights

Revenues

Consolidated Revenues (prior to inter-ministry consolidation adjustments)



Actual revenue was higher than the budget by \$133 million mainly due to:

- Refunds of Expense being higher than anticipated for aggregate assessment and refund of unutilized grant funding from previous years.
- Other Revenue due to unbudgeted cancer drugs received at no cost from suppliers, and more than anticipated recoveries from Pharmaceutical Pricing Policy and external entities.

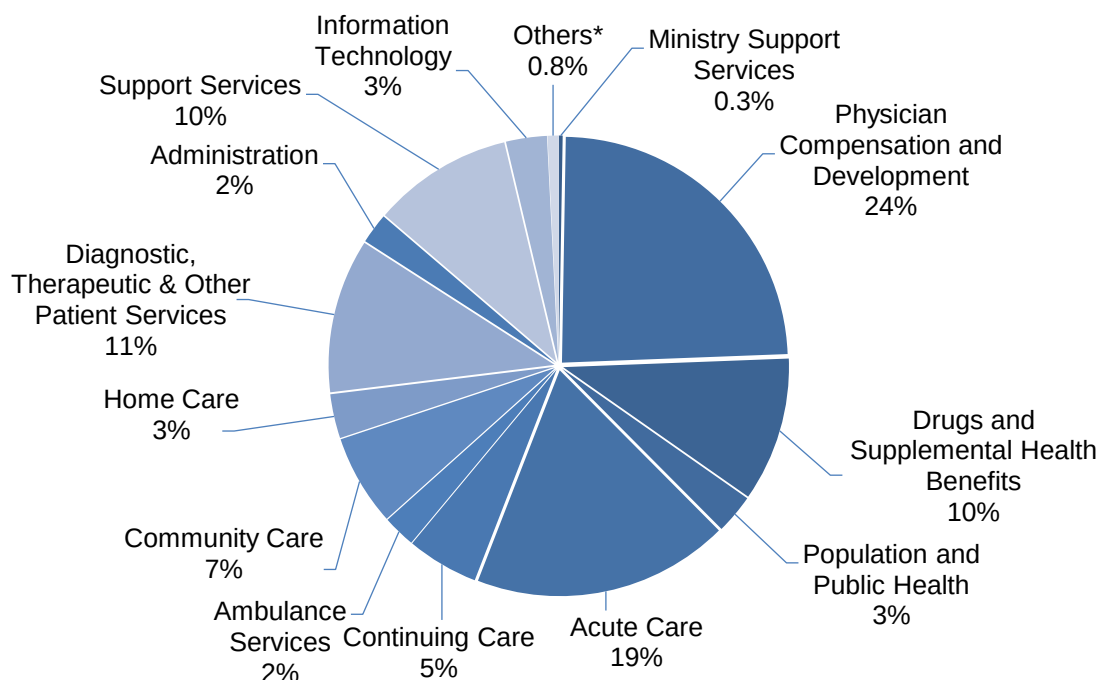
Actual revenue increased by \$291 million over prior year mainly due to:

- Federal Government Transfers due to annualized increase in Canada Health Transfer and funding increase for Home and Community Care Services and Mental Health and Addiction Services.

Ministry of Health

Expenses

Consolidated Expenses (prior to inter-ministry consolidation adjustments)



* includes Research and Education, Debt Servicing, Infrastructure Support, Cancer Research and Prevention Investment, and COVID-19 Pandemic Response.

Actual expenses was higher than the budget by \$255 million mainly due to:

- Drugs and Supplemental Health Benefits as a result of accounting for utilization of unbudgeted cancer drugs received at no cost, higher utilization of high cost cancer drugs, and increase in senior drug benefits due to volume growth;
- Acute Care due to higher than anticipated demand for services resulting in higher staffing levels and costs; and
- Diagnostic, Therapeutic and Other Patient Services due to lower than planned achieved savings and increased access for Magnetic Resonance Imaging (MRI) and Computed Tomography (CT) scans.

The deficit was partially offset by surplus in:

- Population and Public Health due to deferral of spending as part of in-year cost savings initiatives and reduced funding resulting from unutilized prior year funds; and
- Administration due to vacancies and discretionary spending reduction strategies resulting in reduced expenses.

Actual expenses increased by \$422 million over prior year mainly due to:

- Physician Compensation and Development attributed to volume growth in Fee for Service;
- Drugs and Supplemental Health Benefits due to increased utilization of high cost cancer drugs and increased population eligible for drugs, support and health benefits;
- Community Care as a result of full implementation of initiatives related to Continuing Care Capacity Plan and Health System Integration that started partway during prior year;
- Information Technology due to development and implementation of Connect Care and retirement of obsolete operating systems.

The increase was partially offset by:

- Infrastructure Support due to deferral of spending as part of in-year cost savings initiatives, and project delays for continuing care facilities; and
- Administration due to lower costs of liability insurance.

Ministry of Health

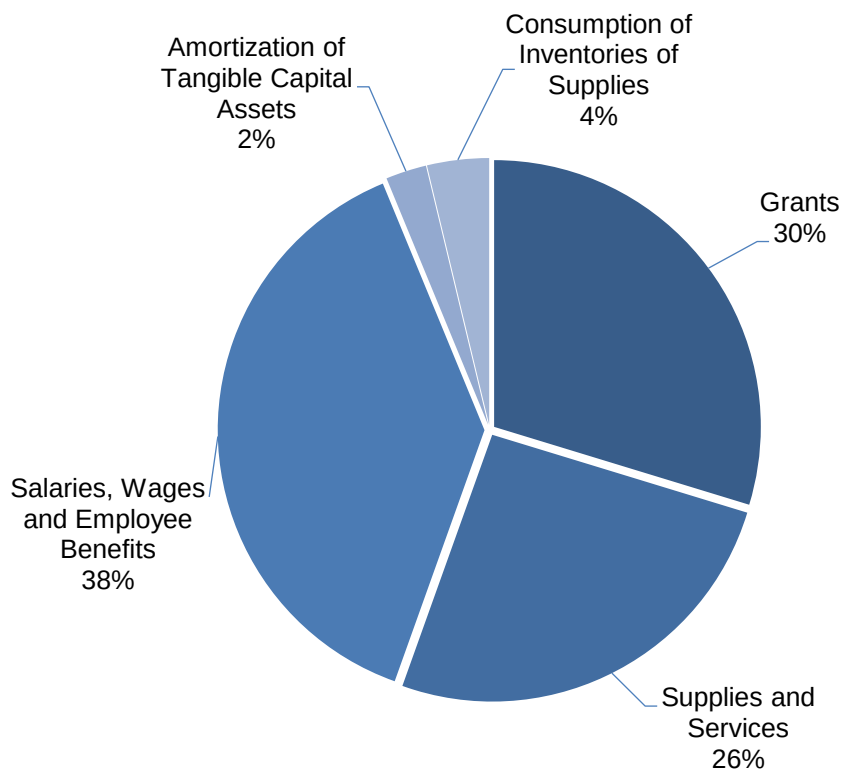
Expenses – Directly Incurred Detailed by Object (unaudited)

(in thousands)

	2020		2019
	Budget	Actual	Actual
Grants	\$ 6,662,737	\$ 6,694,349	\$ 6,579,024
Supplies and Services	5,787,102	5,800,962	5,781,909
Salaries, Wages and Employee Benefits	8,430,783	8,629,931	8,420,613
Amortization of Tangible Capital Assets	572,522	557,249	544,239
Consumption of Inventories of Supplies	893,500	844,955	807,468
Financial Transactions and Other	18,140	92,585	64,608
	<u>\$ 22,364,784</u>	<u>\$ 22,620,031</u>	<u>\$ 22,197,861</u>

Ministry of Health

2020 Actual (unaudited)



- Ministry expenses incurred were primarily for Grants, Supplies and Services, and Salaries, Wages and Employee benefits, which accounted for 94% of the total expenses.
- Salaries, Wages, and Employee benefits totaled 38% (\$8.6 billion) of the expenses primarily for delivery and provision of health services.
- Grants comprised 30% (\$6.7 billion) of the expenses primarily for Physician Compensation and Development, and Drugs and Supplemental Health Benefits. Other grant expenses includes restricted funding to support organization and communities through grant programs, and funding for out-of-province health services.
- Supplies and services resulted in 26% (\$5.8 billion) of the expenses mainly attributed to contracts with voluntary and private health service providers, contract payments to physicians for referred-out services, purchased services, and home support contracts.
- Amortization of tangible capital assets (\$557 million), consumption of inventories of supplies (\$845 million), and financial transactions and other expenses (\$93 million) comprises the residual ministry expenses.

Ministry of Health

Supplemental Financial Information

Tangible Capital Assets (unaudited)

Year ended March 31, 2020

(in thousands)

	2020						2019
	Land	Buildings ⁽¹⁾	Improvements	Equipment	Computer Hardware and Software	Leasehold Assets	Total
Estimated Useful Life	Indefinite	10-40 years	5-40 years	3-20 years	3-10 years	Term of Lease	
Historical Cost ⁽²⁾							
Beginning of year	\$ 116,823	\$ 11,481,121	\$ 94,188	\$ 2,546,862	\$ 2,010,087	\$ 248,124	\$ 16,497,205
Additions ⁽³⁾	133	683,383	11,473	103,060	247,072	32,231	1,077,352
Disposals, including write-downs	(30)	(30,216)	(80)	(46,220)	(53,396)	(1,069)	(131,011)
	116,926	12,134,288	105,581	2,603,702	2,203,763	279,286	17,443,546
Accumulated Amortization							
Beginning of year	-	4,349,705	68,851	1,923,929	1,530,633	192,369	8,065,487
Amortization expense	-	305,988	3,280	146,443	93,014	8,524	557,249
Effect of disposals	-	(2,813)	(80)	(45,228)	(41,914)	(1,067)	(91,102)
	-	4,652,880	72,051	2,025,144	1,581,733	199,826	8,531,634
Net Book Value at March 31, 2020	\$ 116,926	\$ 7,481,408	\$ 33,530	\$ 578,558	\$ 622,030	\$ 79,460	\$ 8,911,912
Net Book Value at March 31, 2019	\$ 116,823	\$ 7,131,416	\$ 25,337	\$ 622,933	\$ 479,454	\$ 55,755	\$ 8,431,718

⁽¹⁾ Buildings include parking lots.⁽²⁾ Historical cost includes work-in-progress at March 31, 2020 totaling \$1,770,125 (2019 - \$1,633,567).⁽³⁾ Additions include total contributed capital assets of \$523,196 (2019 - \$285,368) consisting of \$523,196 from Ministry of Infrastructure (2019 - \$285,322) and \$nil from other sources (2019 - \$46).

Ministry of Health

Portfolio Investments (unaudited)

Year ended March 31, 2020

(in thousands)

	2020		2019	
	Book Value	Fair Value	Book Value	Fair Value
Cash held for investing purposes	\$ 112,072	\$ 112,072	\$ 110,887	\$ 110,887
Interest bearing securities:				
Money market securities	38,076	38,076	177,199	177,199
Fixed income securities	888,059	902,690	1,467,856	1,485,637
	926,135	940,766	1,645,055	1,662,836
Equities:				
Canadian equities	121,175	111,491	157,852	175,391
Global equities	256,183	278,175	288,454	329,954
	377,358	389,666	446,306	505,345
Real estate pooled funds	40,267	40,837	-	-
Total Portfolio Investments	\$ 1,455,832	\$ 1,483,341	\$ 2,202,248	\$ 2,279,068

The following is a breakdown of portfolio investments:

	2020		2019	
	Cost	Fair Value	Cost	Fair Value
Operating	\$ 1,380,394	\$ 1,407,903	\$ 2,127,091	\$ 2,203,911
Endowments	75,438	75,438	75,157	75,157
Total Portfolio Investments	\$ 1,455,832	\$ 1,483,341	\$ 2,202,248	\$ 2,279,068

Financial Statements of Other Reporting Entities

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Alberta Health Services

Financial Statements

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Management's Responsibility for Financial Reporting

The accompanying consolidated financial statements for the year ended March 31, 2020 are the responsibility of management and have been reviewed and approved by senior management. The consolidated financial statements were prepared in accordance with Canadian Public Sector Accounting Standards and the financial directives issued by Alberta Health, and of necessity include some amounts based on estimates and judgment.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains systems of financial management and internal control which give consideration to costs, benefits and risks that are designed to:

- provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded so as to maintain accountability of public money;
- safeguard the assets and properties of the "Province of Alberta" under Alberta Health Services' administration

Alberta Health Services carries out its responsibility for the consolidated financial statements through the Audit & Risk Committee (the Committee). The Committee meets with management and the Auditor General of Alberta to review financial matters, and recommends the consolidated financial statements to the Alberta Health Services Board for approval upon finalization of the audit. The Auditor General of Alberta has free access to the Committee.

The Auditor General of Alberta provides an independent audit of the consolidated financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures which allow him to report on the fairness of the consolidated financial statements prepared by management.

[Original signed by Dr. Verna Yiu, MD, FRCPC]

Dr. Verna Yiu, MD, FRCPC
President and Chief Executive Officer
Alberta Health Services

[Original signed by Robert Hawes, CPA, CA, ICD.D]

Robert Hawes, CPA, CA, ICD.D
Interim Vice President Corporate Services and Chief
Financial Officer
Alberta Health Services

June 24, 2020

Independent Auditor's Report

To the Members of the Alberta Health Services Board and the Minister of Health



**Auditor
General**
OF ALBERTA

Report on the Consolidated Financial Statements

Opinion

I have audited the consolidated financial statements of Alberta Health Services (the Group), which comprise the consolidated statement of financial position as at March 31, 2020, and the consolidated statements of operations, change in net debt, remeasurement gains and losses, and cash flows for the year then ended and notes to the consolidated financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying consolidated financial statements present fairly, in all material respects, the consolidated financial position of the Group as at March 31, 2020, and the results of its operations, its remeasurement gains and losses, its changes in net debt, and its cash flows for the year then ended in accordance with Canadian public sector accounting standards.

Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Consolidated Financial Statements* section of my report. I am independent of the Group in accordance with the ethical requirements that are relevant to my audit of the consolidated financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

Other information

Management is responsible for the other information. The other information comprises the information included in the *Annual Report*, but does not include the consolidated financial statements and my auditor's report thereon. The *Annual Report* is expected to be made available to me after the date of this auditor's report.

My opinion on the consolidated financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the consolidated financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the consolidated financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

Responsibilities of management and those charged with governance for the consolidated financial statements

Management is responsible for the preparation and fair presentation of the consolidated financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the consolidated financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the consolidated financial statements, management is responsible for assessing the Group's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and

using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the Group's financial reporting process.

Auditor's responsibilities for the audit of the consolidated financial statements

My objectives are to obtain reasonable assurance about whether the consolidated financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these consolidated financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgment and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the consolidated financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Group's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Group's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my auditor's report to the related disclosures in the consolidated financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit evidence obtained up to the date of my auditor's report. However, future events or conditions may cause the Group to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the consolidated financial statements, including the disclosures, and whether the consolidated financial statements represent the underlying transactions and events in a manner that achieves fair presentation.
- Obtain sufficient appropriate audit evidence regarding the financial information of the entities or business activities within the Group to express an opinion on the consolidated financial statements. I am responsible for the direction, supervision and performance of the group audit. I remain solely responsible for my audit opinion.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Original signed by W. Doug Wylie FCPA, FCMA, ICD.D]
Auditor General

June 24, 2020
Edmonton, Alberta

Alberta Health Services

Consolidated Statement of Operations

Year ended March 31, 2020

(thousands of dollars)

CONSOLIDATED STATEMENT OF OPERATIONS YEAR ENDED MARCH 31			
	2020		2019
	Budget (Note 3)	Actual	Actual
Revenues:			
Alberta Health transfers			
Base operating	\$ 12,600,000	\$ 12,598,000	\$ 12,485,595
One-time base operating	-	-	29,558
Other operating	1,294,000	1,290,302	1,192,862
Recognition of expended deferred capital revenue	61,000	61,354	65,104
Other government transfers (Note 4)	427,000	434,768	429,665
Fees and charges	508,000	532,250	538,721
Ancillary operations	136,000	129,129	133,513
Donations, fundraising, and non-government contributions (Note 5)	154,000	195,980	181,319
Investment and other income (Note 6)	185,000	226,752	218,367
TOTAL REVENUE	15,365,000	15,468,535	15,274,704
Expenses:			
Continuing care	1,157,000	1,176,468	1,137,363
Community care	1,521,000	1,525,789	1,445,283
Home care	689,000	716,561	688,295
Acute care	4,940,000	5,065,807	5,018,467
Ambulance services	517,000	530,662	528,045
Diagnostic and therapeutic services	2,495,000	2,539,566	2,505,618
Population and public health	361,000	357,117	349,669
Research and education	319,000	344,634	347,239
Information technology	588,000	597,005	507,326
Support services (Note 7)	2,265,000	2,287,205	2,260,421
Administration (Note 8)	513,000	473,544	525,609
TOTAL EXPENSES (Schedules 1 and 3)	15,365,000	15,614,358	15,313,335
ANNUAL OPERATING DEFICIT	-	(145,823)	(38,631)
Accumulated surplus, beginning of year	1,278,000	1,278,424	1,317,055
Accumulated surplus, end of year (Note 19)	\$ 1,278,000	\$ 1,132,601	\$ 1,278,424

The accompanying notes and schedules are part of these consolidated financial statements.

Alberta Health Services

Consolidated Statement of Financial Position

As at March 31, 2020

(thousands of dollars)

CONSOLIDATED STATEMENT OF FINANCIAL POSITION AS AT MARCH 31		
	2020 Actual	2019 Actual (Note 26)
Financial Assets:		
Cash	\$ 538,778	\$ 60,610
Portfolio investments (Note 10)	1,483,341	2,279,068
Accounts receivable (Note 11)	610,571	366,722
	2,632,690	2,706,400
Liabilities:		
Accounts payable and accrued liabilities (Note 12)	1,627,584	1,505,873
Employee future benefits (Note 13)	707,471	688,496
Unexpended deferred operating revenue (Note 14)	405,951	453,219
Unexpended deferred capital revenue (Note 15)	108,823	128,394
Debt (Note 17)	481,551	347,642
	3,331,380	3,123,624
NET DEBT	(698,690)	(417,224)
Non-Financial Assets:		
Tangible capital assets (Note 18)	8,855,960	8,381,004
Inventories for consumption	127,298	106,509
Prepaid expenses, deposits, and other non-financial assets	211,480	167,722
	9,194,738	8,655,235
NET ASSETS BEFORE EXPENDED DEFERRED CAPITAL REVENUE	8,496,048	8,238,011
Expended deferred capital revenue (Note 16)	7,359,615	6,925,118
NET ASSETS	1,136,433	1,312,893
Net Assets is comprised of:		
Accumulated surplus (Note 19)	1,132,601	1,278,424
Accumulated remeasurement gains	3,832	34,469
	\$ 1,136,433	\$ 1,312,893

Contractual Obligations and Contingent Liabilities (Note 20)
Impact of COVID-19 (Note 25)

The accompanying notes and schedules are part of these consolidated financial statements.

Approved by the Alberta Health Services Board:

[Original signed by David Weyant]

David Weyant, Q.C.
Board Chair

[Original signed by David Carpenter]

David Carpenter, FCPA, FCA
Audit & Risk Committee Chair

Alberta Health Services

Consolidated Statement of Change in Net Debt

Year ended March 31, 2020

(thousands of dollars)

CONSOLIDATED STATEMENT OF CHANGE IN NET DEBT YEAR ENDED MARCH 31			
	2020		2019
	Budget (Note 3)	Actual	Actual
Annual operating deficit	\$ -	\$ (145,823)	\$ (38,631)
Effect of changes in tangible capital assets:			
Acquisition of tangible capital assets (Note 18):			
Purchased tangible capital assets	(542,000)	(533,483)	(593,957)
Contributed tangible capital assets	(496,000)	(523,196)	(285,368)
Amortization and loss on disposals/write-downs of tangible capital assets (Note 18)	554,000	581,723	529,628
Effect of other changes:			
Net increase in expended deferred capital revenue	448,000	434,497	189,664
Net decrease (increase) in inventories for consumption	22,000	(20,789)	(9,936)
Net decrease (increase) in prepaid expenses and other non-financial assets	1,000	(43,758)	(2,001)
Net remeasurement gains (losses) for the year	22,000	(30,637)	13,634
Decrease (increase) in net debt for the year	9,000	(281,466)	(196,967)
Net debt, beginning of year	(417,000)	(417,224)	(220,257)
Net debt, end of year	\$ (408,000)	\$ (698,690)	\$ (417,224)

The accompanying notes and schedules are part of these consolidated financial statements.

Alberta Health Services

Consolidated Statement of Remeasurement Gains and Losses

Year ended March 31, 2020

(thousands of dollars)

CONSOLIDATED STATEMENT OF REMEASUREMENT GAINS AND LOSSES YEAR ENDED MARCH 31			
	2020		2019
	Budget (Note 3)	Actual	Actual
Unrestricted unrealized gains (losses) attributable to:			
Derivatives	\$ -	\$ 539	\$ 253
Portfolio investments	30,000	(5,933)	17,751
Amounts reclassified to the Consolidated Statement of Operations:			
Portfolio investments	(8,000)	(25,243)	(4,370)
Net remeasurement gains (losses) for the year	22,000	(30,637)	13,634
Accumulated remeasurement gains, beginning of year	35,000	34,469	20,835
Accumulated remeasurement gains, end of year (Note 10)	\$ 57,000	\$ 3,832	\$ 34,469

The accompanying notes and schedules are part of these consolidated financial statements.

Alberta Health Services

Consolidated Statement of Cash Flows

Year ended March 31, 2020

(thousands of dollars)

CONSOLIDATED STATEMENT OF CASH FLOWS YEAR ENDED MARCH 31		
	2020	2019
	Actual	Actual (Note 26)
Operating transactions:		
Annual operating deficit	\$ (145,823)	\$ (38,631)
Non-cash items:		
Amortization and loss on disposals/write-downs of tangible capital assets	581,723	529,628
Recognition of expended deferred capital revenue	(404,405)	(383,405)
Decrease (increase) in:		
Accounts receivable related to operating transactions	(254,073)	48,304
Inventories for consumption	(20,789)	(9,936)
Prepaid expenses and other non-financial assets	(43,758)	(2,001)
Increase (decrease) in:		
Accounts payable and accrued liabilities related to operating transactions	100,838	76,173
Employee future benefits	18,975	15,360
Unexpended deferred operating revenue	(60,847)	(31,957)
Cash (applied to) provided by operating transactions	(228,159)	203,535
Capital transactions:		
Purchased tangible capital assets	(533,483)	(593,957)
Increase in accounts payable and accrued liabilities related to capital transactions	44,406	37,368
Cash applied to capital transactions	(489,077)	(556,589)
Investing transactions:		
Purchase of portfolio investments	(2,683,128)	(3,161,266)
Proceeds on disposals of portfolio investments	3,429,544	3,220,278
Cash provided by investing transactions	746,416	59,012
Financing transactions:		
Restricted capital contributions received	343,010	331,363
Unexpended deferred capital revenue returned	(4,398)	(250)
Proceeds from debt	157,000	-
Principal payments on debt	(23,091)	(22,133)
Payments on obligations under capital leases	(23,814)	(19,985)
Net receipt (repayment) of life lease deposits	281	(596)
Cash provided by financing transactions	448,988	288,399
Increase (decrease) in cash	478,168	(5,643)
Cash, beginning of year	60,610	66,253
Cash, end of year	\$ 538,778	\$ 60,610

The accompanying notes and schedules are part of these consolidated financial statements.

Alberta Health Services

Notes to the Consolidated Financial Statements

For the year ended March 31, 2020

(thousands of dollars)

Note 1 Authority, Purpose and Operations

Alberta Health Services (AHS) was established under the *Regional Health Authorities Act* (Alberta), effective April 1, 2009, as a result of the merger of 12 formerly separate health entities in Alberta.

Pursuant to Section 5 of the *Regional Health Authorities Act* (Alberta), AHS is responsible in Alberta to:

- promote and protect the health of the population and work toward the prevention of disease and injury;
- assess on an ongoing basis the health needs of the population;
- determine priorities in the provision of health services and allocate resources accordingly;
- ensure that reasonable access to quality health services is provided and;
- promote the provision of health services in a manner that is responsive to the needs of individuals and communities and supports the integration of services and facilities.

Additionally, AHS is accountable to the Minister of Health (the Minister) for the delivery and operation of the public health system.

The AHS consolidated financial statements include the revenue and expenses associated with its responsibilities. These consolidated financial statements do not reflect the complete costs of provincial health care. For a complete picture of the costs of provincial health care, readers should consult the consolidated financial statements of the Government of Alberta (GOA).

Note 2 Significant Accounting Policies and Reporting Practices

(a) Basis of Presentation

These consolidated financial statements have been prepared in accordance with Canadian Public Sector Accounting Standards and the financial directives issued by Alberta Health (AH).

These consolidated financial statements reflect the assets, liabilities, revenues, and expenses of the reporting entity, which is comprised of the organizations controlled by AHS as noted below:

(i) Controlled Entities

AHS controls the following three entities:

- Alberta Precision Laboratories Ltd. (APL) (formerly Alberta Public Laboratories Ltd.) - provides medical diagnostic services throughout Alberta. AHS owns 100% of the Class A voting shares.
- CapitalCare Group Inc. (CCGI) - manages continuing care programs and facilities in the Edmonton area. AHS owns 100% of the Class A voting shares.
- Carewest - manages continuing care programs and facilities in the Calgary area. AHS owns 99% of the Class A voting shares and 1% of the Class A voting shares are held in trust for the benefit of AHS by the Chair of the Board of Directors.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

AHS has majority representation on, or the right to appoint, the governance boards indicating control of the following entities:

- Foundations:

- Airdrie Health Foundation - Alberta Cancer Foundation - American Friends of the Calgary Health Trust Foundation (<i>inactive</i>) - Bassano and District Health Foundation - Bow Island and District Health Foundation - Brooks and District Health Foundation - Calgary Health Trust - Canmore and Area Health Care Foundation - Cardston and District Health Foundation - Claresholm and District Health Foundation - Crowsnest Pass Health Foundation - David Thompson Health Trust (<i>inactive</i>) - Fort Macleod and District Health Foundation - Fort Saskatchewan Community Hospital Foundation - Grande Cache Hospital Foundation - Grimshaw/Berwyn and District Hospital Foundation - Jasper Health Care Foundation - Lac La Biche Regional Health Foundation	- Lacombe Health Trust - Medicine Hat and District Health Foundation - Mental Health Foundation - North County Health Foundation - Oyen and District Health Care Foundation - Peace River and District Health Foundation - Ponoka and District Health Foundation - Rocky Mountain House & Area Health Services Foundation - Stettler Health Services Foundation - Strathcona Community Hospital Foundation - Tofield and Area Health Services Foundation - Two Hills Health Centre Foundation - Vermillion and Region Health and Wellness Foundation (<i>inactive</i>) - Viking Health Foundation - Vulcan County Health and Wellness Foundation - Windy Slopes Health Foundation
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- Provincial Health Authorities of Alberta Liability and Property Insurance Plan (LPIP)

LPIP's main purpose is to share the risks of general and professional liability to lessen the impact on any one subscriber. Effective April 1, 2020, LPIP ceased providing new liability coverage and continues in operation for the limited purpose of winding up its affairs.
- Queen Elizabeth II Hospital Child Care Centre

All inter-entity accounts and transactions between these organizations and AHS are eliminated upon consolidation.

(ii) Government Partnerships

AHS proportionately consolidates its 50% interests in Primary Care Network (PCN) partnerships with physician groups, its 50% interest in the Northern Alberta Clinical Trials Centre (NACTRC) partnership with the University of Alberta, and its 33.33% interest in the Institute for Reconstructive Sciences in Medicine (iRSM) partnership with the University of Alberta and Covenant Health (Note 22).

AHS entered into local primary care initiative agreements to jointly manage and operate the delivery of primary care services to achieve the PCN plan objectives, and to contract and hold property interests required in the delivery of PCN services.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

The following PCNs are included in these consolidated financial statements on a proportionate basis:

Alberta Heartland Primary Care Network	Lloydminster Primary Care Network
Aspen Primary Care Network	McLeod River Primary Care Network
Big Country Primary Care Network	Mosaic Primary Care Network
Bighorn Primary Care Network	Northwest Primary Care Network
Bonnyville Primary Care Network	Palliser Primary Care Network
Bow Valley Primary Care Network	Peace Region Primary Care Network
Calgary Foothills Primary Care Network	Peaks to Prairies Primary Care Network
Calgary Rural Primary Care Network	Provost Primary Care Network
Calgary West Central Primary Care Network	Red Deer Primary Care Network
Camrose Primary Care Network	Rocky Mountain House Primary Care Network
Chinook Primary Care Network	Saddle Hills Primary Care Network (formerly
Cold Lake Primary Care Network	Sexsmith / Spirit River Primary Care Network and
Drayton Valley Primary Care Network	West Peace Region Primary Care Network)
Edmonton North Primary Care Network	Sherwood Park/Strathcona County Primary Care
Edmonton Oliver Primary Care Network	Network
Edmonton Southside Primary Care Network	South Calgary Primary Care Network
Edmonton West Primary Care Network	St. Albert & Sturgeon Primary Care Network
Grande Prairie Primary Care Network	Wainwright Primary Care Network
Highland Primary Care Network	WestView Primary Care Network
Kalyna Country Primary Care Network	Wetaskiwin and Area Primary Care Network
Lakeland Primary Care Network	Wolf Creek Primary Care Network
Leduc Beaumont Devon Primary Care Network	Wood Buffalo Primary Care Network

All inter-entity accounts and transactions between these organizations and AHS are eliminated upon consolidation.

(iii) Trusts under Administration

These consolidated financial statements do not include trusts administered on behalf of others (Note 23). AHS provides services to certain entities not included in these consolidated financial statements.

(iv) Other

AHS is responsible for the delivery and operation of the public health system in Alberta (Note 1), and contracts with various voluntary and private health service providers to provide health services throughout Alberta. The largest of these service providers is Covenant Health, a denominational health care organization, providing a full spectrum of care. Covenant Health is an independent, separate legal entity with a separate Board of Directors and accordingly, these consolidated financial statements do not include their assets, liabilities or results of operations. However, the payments for contracts with health service providers such as Covenant Health are recorded as expenses in the Consolidated Statement of Operations.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(b) Revenue Recognition

Revenue is recognized in the year in which the transactions or events that give rise to the revenue as described below occur. All revenue is recorded on an accrual basis, except when the accrual cannot be determined within a reasonable degree of certainty or when estimation is impracticable.

(i) Government Transfers

Transfers from AH, other GOA ministries and agencies, and other government entities are referred to as government transfers.

Government transfers and the associated externally restricted investment income are recorded as deferred revenue if the eligibility criteria for the use of the transfer, or the stipulations together with AHS' actions and communications as to the use of the transfer, create a liability. These transfers are recognized as revenue as the stipulations are met and, when applicable, AHS complies with the communicated use of the transfer.

All other government transfers, without stipulations for the use of the transfer, are recorded as revenue when the transfer is authorized and AHS meets the eligibility criteria.

Deferred revenue consists of unexpended deferred operating revenue, unexpended deferred capital revenue, and expended deferred capital revenue. The term deferred revenue in these consolidated financial statements refers to the components of deferred revenue as described.

Unallocated costs comprising of materials and services contributed by related parties in support of AHS' operations, are not recognized in these consolidated financial statements.

(ii) Donations, Fundraising, and Non-Government Contributions

Donations, fundraising, and non-government contributions are received from individuals, corporations, and other not-for-profit organizations. Donations, fundraising, and non-government contributions may be unrestricted or externally restricted for operating or capital purposes.

Unrestricted donations, fundraising, and non-government contributions are recorded as revenue in the year received or in the year the funds are committed to AHS if the amount can be reasonably estimated and collection is reasonably assured.

Externally restricted donations, fundraising, non-government contributions, and the associated externally restricted investment income are recorded as deferred revenue if the terms for their use, or the terms along with AHS' actions and communications as to their use create a liability. These resources are recognized as revenue as the terms are met and, when applicable, AHS complies with the communicated use.

In-kind donations of services and materials from non-related parties are recorded at fair value when such value can reasonably be determined. While volunteers contribute a significant amount of time each year to assist AHS, the value of their services is not recognized as revenue and expenses in the consolidated financial statements because fair value cannot be reasonably determined.

(iii) Transfers and Donations related to Land

Transfers and donations for the purchase of land are recognized as deferred revenue when received and as revenue when the land is purchased.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(iv) Fees and Charges, Ancillary Operations, and Other Income

Fees and charges, ancillary operations, and other income are recognized in the year that goods are delivered or services are provided by AHS. Amounts received for which goods or services have not been provided by year end are recorded as deferred revenue.

(v) Investment Income

Investment income includes dividend income, interest income, and realized gains or losses on the sale of portfolio investments. Unrealized gains and losses on portfolio investments exclusive of restricted transfers or donations are recognized in the Consolidated Statement of Remeasurement Gains and Losses until the related portfolio investments are sold. When realized, these gains or losses are recognized in the Consolidated Statement of Operations. Investment income and unrealized gains and losses from restricted transfers or donations are allocated to their respective balances according to the provisions within the individual funding agreements.

(c) Expenses

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed. Interest expense includes debt servicing costs.

Expenses include grants and transfers under shared cost agreements. Grants and transfers are recorded as expenses when the transfer is authorized and eligibility criteria have been met by the recipient.

(d) Financial Instruments

Financial instruments comprise financial assets and liabilities. Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations. Liabilities are present obligations of AHS to others arising from past transactions or events occurring before the year end, the settlement of which is expected to result in the future sacrifice of economic benefits.

All of AHS' financial assets and liabilities are initially recorded at their fair value. The following table identifies AHS' financial assets and liabilities and identifies how they are subsequently measured:

Financial Assets and Liabilities	Subsequent Measurement and Recognition
Portfolio investments	Measured at fair value with unrealized changes in fair values recognized in the Consolidated Statement of Remeasurement Gains and Losses or deferred revenue until realized, at which time the cumulative changes in fair value are recognized in the Consolidated Statement of Operations.
Accounts receivable, accrued vacation pay, accounts payable and accrued liabilities and debt	Measured at amortized cost.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

AHS records equity investments quoted in an active market to be recorded under the fair value category and AHS may choose to record other financial assets under the fair value category if there is an investment strategy to evaluate the performance of a group of these financial assets on a fair value basis. AHS has elected to record all portfolio investments at fair value. The three levels of information that may be used to measure fair value are:

- Level 1 – Unadjusted quoted market prices in active markets for identical assets or liabilities;
- Level 2 – Observable or corroborated inputs, other than level 1, such as quoted prices for similar assets or liabilities in inactive markets or market data for substantially the full term of the assets or liabilities; and
- Level 3 – Unobservable inputs that are supported by little or no market activity and that are significant to the fair value of the assets and liabilities.

AHS measures and recognizes embedded derivatives separately from the host contract when the economic characteristics and risk of the embedded derivative are not closely related to those of the host contract, when it meets the definition of a derivative and, when the entire contract is not measured at fair value.

Derivatives are recorded at fair value in the Consolidated Statement of Financial Position. Derivatives with a positive or negative fair value are recognized as increases or decreases to portfolio investments. Unrealized gains and losses from changes in the fair value of derivatives are recognized in the Consolidated Statement of Remeasurement Gains and Losses.

All financial assets are assessed for impairment on an annual basis. When a decline is determined to be other than temporary, the amount of the loss is reported as a realized loss on the Consolidated Statement of Operations.

Transaction costs associated with the acquisition and disposal of portfolio investments are expensed as incurred. Investment management fees are expensed as incurred. The purchase and sale of portfolio investments are accounted for using trade date accounting.

(e) Cash

Cash is comprised of cash on hand. Cash on hand includes amounts in interest bearing accounts held for the purpose of meeting short-term commitments rather than for investment purposes.

(f) Inventories For Consumption

Inventories for consumption are valued at lower of cost (defined as moving average cost) and replacement cost.

(g) Tangible Capital Assets

Tangible capital assets and work in progress are recorded at cost, which includes amounts that are directly related to the acquisition, design, construction, development, improvement, or betterment of the assets. Cost includes overhead directly attributable to construction and development as well as interest costs that are directly attributable to the acquisition or construction of the asset. Costs incurred by Alberta Infrastructure (AI) to build tangible capital assets on behalf of AHS are recorded by AHS as work in progress as AI incurs costs.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

Contributed tangible capital assets from non-related entities are recognized at their fair value at the date of the contribution when fair value can be reasonably determined. When AHS cannot determine the fair value, it records such contributions at nominal value.

The costs less residual values of tangible capital assets, excluding land, are amortized over their estimated useful lives on a straight-line basis as follows:

	<u>Useful Life</u>
Facilities and improvements	10-40 years
Equipment	3-20 years
Information systems	3-15 years
Building service equipment	5-40 years
Land improvements	5-40 years

Work in progress, which includes facility and improvement projects and development of information systems, is not amortized until after a project is substantially complete and the tangible capital assets are available for or in use.

Leases transferring substantially all benefits and risks of tangible capital asset ownership are classified as capital leases and reported as tangible capital assets. Capital leases and leasehold improvements are amortized over the term of the lease. Capital lease obligations associated with these capital leases are recorded at the present value of the minimum lease payments excluding executory costs (e.g. insurance, maintenance costs, etc.) and reported as obligations under capital leases. The discount rate used to determine the present value of the lease payments is the lower of AHS' rate for incremental borrowing and the interest rate implicit in the lease.

Tangible capital assets are written down to their net recoverable amounts when conditions indicate that they no longer contribute to AHS' ability to provide goods and services or when the value of future economic benefits associated with the tangible capital assets are less than their net book value. Net write-downs are accounted for as expenses in the Consolidated Statement of Operations.

Works of art, historical treasures, and collections are not recognized as tangible capital assets.

(h) Employee Future Benefits

(i) Defined Benefit Pension Plans

Local Authorities Pension Plan (LAPP) and Management Employees Pension Plan (MEPP)

AHS participates in LAPP and MEPP which are registered defined benefit pension plans. These multi-employer public sector defined benefit plans provide pensions for participants for each year of pensionable service based on the average salary of the highest five consecutive years, up to the average Canada Pension Plan's Year's Maximum Pensionable Earnings (YMPE), over the same five consecutive year period. Benefits for post-1991 service payable under these plans are limited by the *Income Tax Act* (Canada). Prior to March 1, 2019, the President of Alberta Treasury Board and Minister of Finance was the legal trustee and administrator for LAPP and MEPP. Although there has been no change in MEPP governance, effective March 1, 2019, LAPP Corporation became the legislated administrator and trustee of LAPP. As a participating government organization, AHS accounts for these plans on a defined contribution basis. Accordingly, the pension expense recorded for these plans in these consolidated financial statements is comprised of the employer contributions that AHS is required to pay for its employees during the fiscal year, which are calculated based on actuarially pre-determined amounts that are expected to provide the plan's future benefits.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

Supplemental Executive Retirement Plan (SERP)

The SERP covers certain employees and supplements the benefits under AHS' registered plans that are limited by the *Income Tax Act* (Canada). The SERP has been closed to new entrants since April 1, 2009. The SERP provides future pension benefits to participants based on years of service and earnings.

As required under the *Income Tax Act* (Canada), approximately half of the assets are held in a non-interest bearing Refundable Tax Account with the Canada Revenue Agency. The remaining assets of the SERP are invested in a combination of Canadian equities and Canadian fixed income securities.

(ii) Defined Contribution Pension Plans

Group Registered Retirement Savings Plans (GRRSPs)

AHS sponsors GRRSPs for certain employee groups. Under the GRRSPs, AHS matches a certain percentage of any contribution made by plan participants up to certain limits. AHS also sponsors a defined contribution pension plan for certain employee groups where the employee and employer each contribute specified percentages of pensionable earnings.

Supplemental Pension Plan (SPP)

Subsequent to April 1, 2009, staff that would have otherwise been eligible for SERP have been enrolled in a defined contribution SPP. The SPP supplements the benefits under AHS registered plans that are limited by the *Income Tax Act* (Canada). AHS contributes a percentage of an eligible employee's pensionable earnings, in excess of the limits of the *Income Tax Act* (Canada). The SPP provides participants with an account balance at retirement based on the contributions made to the plan and investment income earned on the contributions based on investment decisions made by the participant.

(iii) Other Benefit Plans

Accumulating Non-Vesting Sick Leave

Sick leave benefits accumulate with employees' service and are provided by AHS to certain employee groups of AHS, as defined by employment agreements, to cover illness related absences that are outside of short-term and long-term disability coverage. Benefit amounts are determined and accumulate with reference to employees' final earnings at the time they are paid out. The cost of the accumulating non-vesting sick leave benefits is expensed as the benefits are earned.

AHS recognizes a liability and expense for accumulating non-vesting sick leave benefits using an actuarial cost method as the employees render services to earn the benefits. The liability and expense is determined using the projected benefit method pro-rated for service and management's best estimates of expected discount rate, inflation, rate of compensation increase, termination and retirement dates, and mortality. Actuarial gains and losses are amortized on a straight-line basis over the expected average remaining service life of the related employee groups.

AHS does not record a liability for sick leave benefits that do not accumulate beyond the current reporting year as these are renewed annually.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

Other Benefits

AHS provides its employees with basic life, accidental death and dismemberment, short-term disability, long-term disability, extended health, dental, and vision benefits through benefits carriers. AHS fully accrues its obligations for employee non-pension future benefits.

(i) Liability for Contaminated Sites

Contaminated sites are a result of contamination being introduced into air, soil, water, or sediment of a chemical, organic or radioactive material or live organism that exceeds an environmental standard. A liability for remediation of contaminated sites is recognized when all of the following criteria are met:

- (i) an environmental standard exists;
- (ii) contamination exceeds the environmental standard;
- (iii) AHS is directly responsible or accepts responsibility;
- (iv) it is expected that future economic benefits will be given up; and
- (v) a reasonable estimate of the amount can be made.

(j) Foreign Currency Translation

Transaction amounts denominated in foreign currencies are translated into their Canadian dollar equivalents at exchange rates prevailing at the transaction dates. Carrying values of monetary assets and liabilities and non-monetary items included in the fair value category reflect the exchange rates at the Consolidated Statement of Financial Position date. Unrealized foreign exchange gains and losses are recognized in the Consolidated Statement of Remeasurement Gains and Losses.

In the year of settlement, foreign exchange gains and losses are reclassified to the Consolidated Statement of Operations, and the cumulative amount of remeasurement gains and losses are reversed in the Consolidated Statement of Remeasurement Gains and Losses.

(k) Reserves

Certain amounts, as approved by the AHS Board, may be set aside in accumulated surplus for use by AHS for future purposes. Transfers to, or from, are recorded to the respective reserve account when approved. Reserves include Invested in Tangible Capital Assets, Internally Restricted Surplus for Future Purposes, and Internally Restricted Surplus for Insurance Equity Requirements and Foundations.

(l) Measurement Uncertainty

The consolidated financial statements, by their nature, contain estimates and are subject to measurement uncertainty. Measurement uncertainty exists when there is a difference between the recognized or disclosed amount and another reasonably possible amount. These estimates and assumptions are reviewed at least annually. Actual results could differ from the estimates determined by management in these consolidated financial statements, and these differences could require adjustment in subsequent reporting years. Significant estimates include: amortization of tangible capital assets, recognition of expended deferred capital revenue, employee future benefits, provision for unpaid claims, accrued liabilities and allowance for doubtful accounts.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(m) Future Accounting Changes

The following accounting standards are applicable in future years:

- PS 3280 – Asset Retirement Obligations (effective April 1, 2021)**
 PS 3280 provides guidance on how to account for and report a liability for retirement of a tangible capital asset. AHS has completed an initial review of the standard and conducted a preliminary assessment of tangible capital assets impacted. Based on the scoping work performed to date, AHS has identified the asset categories of Facilities and Equipment as the assets primarily impacted by this standard. In-depth discussions continue as AHS considers the nature and related cost estimates of, the applicable retirement obligations for these assets. The overall impact of adopting this standard on the future consolidated financial statements continues to be assessed.
- PS 3400 – Revenue (effective April 1, 2022)**
 PS 3400 provides guidance on how to account for and report revenue, and specifically, it differentiates between revenue arising from exchange and non-exchange transactions. AHS is currently assessing what the impact of this standard will have on future consolidated financial statements.

Note 3 Budget

The AHS Health Plan and Business Plan, which included the 2019-20 annual budget, was approved by the AHS Board on December 11, 2019 for submission to the Minister who approved it on January 28, 2020.

Note 4 Other Government Transfers

	Budget	2020	2019
Unrestricted operating	\$ 32,000	\$ 34,035	\$ 32,790
Restricted operating (Note 14)	113,000	92,152	114,269
Recognition of expended deferred capital revenue (Note 16)	282,000	308,581	282,606
	\$ 427,000	\$ 434,768	\$ 429,665

Other government transfers include \$425,845 (2019 – \$420,622) transferred from the GOA, \$8,923 (2019 – \$9,043) from government entities outside the GOA, and exclude amounts from AH as these amounts are separately disclosed on the Consolidated Statement of Operations.

Note 5 Donations, Fundraising, and Non-Government Contributions

	Budget	2020	2019
Unrestricted operating	\$ 3,000	\$ 3,940	\$ 4,073
Restricted operating (Note 14)	124,000	157,289	141,088
Recognition of expended deferred capital revenue (Note 16)	27,000	34,470	35,695
Endowment contributions	-	281	463
	\$ 154,000	\$ 195,980	\$ 181,319

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 6 Investment and Other Income

	Budget	2020	2019
Investment income	\$ 65,000	\$ 80,243	\$ 68,521
Other income:			
GOA (Note 21)	34,000	28,077	30,847
AH	-	12,562	12,698
Other	86,000	105,870	106,301
	\$ 185,000	\$ 226,752	\$ 218,367

The Other balance of \$105,870 (2019 - \$106,301) mainly relates to recoveries for services provided to third parties.

Note 7 Support Services

	Budget	2020	2019
Facilities operations	\$ 913,000	\$ 905,659	\$ 893,599
Patient: health records, food services, and transportation	439,000	436,326	424,288
Housekeeping, laundry, and linen	224,000	220,500	217,296
Materials management	177,000	177,684	174,916
Support services expense of full-spectrum contracted health service providers	157,000	152,542	154,714
Ancillary operations	108,000	97,099	103,122
Fundraising expenses and grants awarded	48,000	48,635	45,689
Emergency preparedness services	6,000	5,847	5,255
Other	193,000	242,913	241,542
	\$ 2,265,000	\$ 2,287,205	\$ 2,260,421

Note 8 Administration

	Budget	2020	2019
General administration	\$ 258,000	\$ 220,679	\$ 272,241
Human resources	116,000	114,865	115,901
Finance	74,000	74,969	73,075
Communications	27,000	24,771	24,415
Administration expense of full-spectrum contracted health service providers	38,000	38,260	39,977
	\$ 513,000	\$ 473,544	\$ 525,609

Note 9 Financial Risk Management

AHS is exposed to a variety of financial risks associated with its financial instruments. These financial risks include market risk, credit risk, and liquidity risk.

(a) Market Risk

Market risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate due to changes in market prices. Market risk is comprised of three types of risk: price risk, interest rate risk, and foreign currency risk.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 9 Financial Risk Management (continued)

In order to earn financial returns at an acceptable level of market risk, the consolidated investment portfolio is governed by investment bylaws and policies with clearly established target asset mixes. The target assets range between 0% and 100% for cash and money market securities, 0% to 80% for fixed income securities and 0% to 70% for equity holdings.

Risk is reduced through asset class diversification, diversification within each asset class, and portfolio quality constraints governing the quality of portfolio holdings.

AHS assesses the sensitivity of its portfolio to market risk based on historical volatility of equity and fixed income markets. Volatility is determined using a ten year average based on fixed income and equity market fluctuations and is applied to the total portfolio. Based on the volatility average of 3.24% (2019 – 2.26%) increase or decrease, with all other variables held constant, the portfolio could expect an increase or decrease in accumulated remeasurement gains and losses and unrealized net gains and losses attributable to unexpended deferred operating revenue of \$28,877 (2019 – \$38,130).

(i) Price Risk

Price risk relates to the possibility that equity portfolio investments will change in fair value due to future fluctuations in market prices caused by factors specific to an individual equity investment or other factors affecting all equities traded in the market. AHS is exposed to price risk associated with the underlying equity portfolio investments held in pooled funds. If equity market indices (S&P/TSX, S&P1500 and MSCI ACWI and their sectors) declined by 10%, and all other variables are held constant, the potential loss in fair value to AHS would be approximately \$42,045 or 2.84% of total portfolio investments (March 31, 2019 – \$50,535 or 2.22%).

In March 2020, the COVID-19 global pandemic resulted in an overall reduction of market prices for equities traded in the market at year-end. AHS is invested primarily in interest bearing securities (money market and fixed income securities) and this diversification served to mitigate price risk for AHS. As a result of this diversification, AHS did not realize significant losses in March 2020.

(ii) Interest Rate Risk

Interest rate risk is the risk that the value of a financial instrument might be adversely affected by a change in market interest rates. AHS manages the interest rate risk exposure of its fixed income securities by management of average duration and laddered maturity dates.

AHS is exposed to interest rate risk through its investments in fixed income securities with both fixed and floating interest rates. AHS has fixed interest rate loans for all debt, thereby mitigating interest rate risk from rate fluctuations over the term of the outstanding debt. The fair value of fixed rate debt fluctuates with changes in market interest rates but the related future cash flows will not change.

In general, investment returns for fixed income securities are sensitive to changes in the level of interest rates, with longer term interest bearing securities being more sensitive to interest rate changes than shorter term bonds.

A 1% change in market yield relating to fixed income securities would have increased or decreased fair value by approximately \$53,634 (March 31, 2019 – \$78,148).

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 9 Financial Risk Management (continued)

Fixed income securities have the following average maturity structure:

	2020	2019
0 – 5 years	56%	77%
6 – 10 years	20%	8%
Over 10 years	24%	15%

	Average Effective Market Yield	
Asset Class	2020	2019
Fixed income securities	1.98%	2.47%

(iii) Foreign Currency Risk

Foreign currency risk is the risk that the fair value of future cash flows of a financial instrument will fluctuate because of changes in foreign exchange rates. The fair value of cash and portfolio investments denominated in foreign currencies is translated into Canadian dollars on a daily basis using the reporting date exchange rate. Both the realized gain/loss and remeasurement gain/loss comprise actual gains or losses on the underlying instrument as well as changes in foreign exchange rates at the time of the valuation. AHS is exposed to foreign exchange fluctuations on its cash denominated in foreign currencies. AHS is also exposed to changes in the valuation on its global equity funds attributable to fluctuations in foreign currency.

Foreign currency risk is managed by the investment policies which limit non-Canadian equities to a maximum of 10% to 45% of the total investment portfolio, depending on the policy. At March 31, 2020, investments in non-Canadian equities represented 18.4% (March 31, 2019 – 14.5%) of total portfolio investments.

Foreign exchange fluctuations on cash balances are mitigated by derivatives and holding minimal foreign currency cash balances. At March 31, 2020, AHS held US dollar forward contracts with ATB Financial to manage currency fluctuations relating to its US dollar accounts payable requirements. As at March 31, 2020, AHS held derivatives in the form of forward contracts for future settlement of \$12,000 (2019 – \$18,000). The fair value of these forward contracts as at March 31, 2020 was \$1,253 (2019 – \$714) and is included in portfolio investments (Note 10).

(b) Credit Risk

Credit risk is the risk of loss arising from the failure of a counterparty to fully honour its contractual obligations. The credit quality of financial assets is generally assessed by reference to external credit ratings. Credit risk can also lead to losses when issuers and debtors are downgraded by credit rating agencies. The investment policies restrict the types and proportions of eligible investments, thus mitigating AHS' exposure to credit risk.

Accounts receivable primarily consists of amounts receivable from AH, other Alberta government reporting entities, patients, other provinces and territories, and the federal government. AHS periodically reviews the collectability of its accounts receivable and establishes an allowance based on its best estimate of potentially uncollectible amounts.

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Note 9 Financial Risk Management (continued)

Under the investment bylaw and policies governing the consolidated investment portfolio, money market securities are limited to a rating of R1 or equivalent or higher, and no more than 10% may be invested in any one issuer unless guaranteed by the Government of Canada or a Canadian province. Investments in corporate bonds are limited to BBB or equivalent rated bonds or higher and no more than 40% of the total fixed income securities. Investments in debt and equity of any one issuer are limited to 5% of the issuer's total debt and equity. AHS holds unrated mortgage fund investments which are classified as part of AHS' fixed income securities. Short selling is not permitted.

The following table summarizes AHS' investment in debt securities by counterparty credit rating at March 31, 2020. The unrated securities consist of low volatility pooled mortgages that are not rated on an active market.

Credit Rating	2020	2019
Investment Grade (AAA to BBB)	89%	87%
Unrated	11%	13%
	100%	100%

(c) Liquidity Risk

Liquidity risk is the risk that AHS will encounter difficulty in meeting obligations associated with financial liabilities that are settled by delivery of cash or another financial asset. Liquidity requirements of AHS are met through funding provided in advance by AH, income generated from portfolio investments, and by investing in liquid assets, such as money market securities, fixed income securities and equities traded in an active market that are easily sold and converted to cash. Short term borrowing to meet financial obligations would be available through established credit facilities as described in Note 17(b).

Note 10 Portfolio Investments

	2020		2019	
	Fair Value	Cost	Fair Value	Cost
Cash held for investing purposes	\$ 112,072	\$ 112,072	\$ 110,887	\$ 110,887
Interest bearing securities:				
Money market securities	38,076	38,076	177,199	177,199
Fixed income securities	902,690	888,059	1,485,637	1,467,856
	940,766	926,135	1,662,836	1,645,055
Equities:				
Canadian equity investments	44,554	47,368	45,866	38,488
Canadian equity funds	66,937	73,807	129,525	119,364
Global equity funds	278,175	256,183	329,954	288,454
	389,666	377,358	505,345	446,306
Real estate pooled funds	40,837	40,267	-	-
	\$ 1,483,341	\$ 1,455,832	\$ 2,279,068	\$ 2,202,248

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Note 10 Portfolio Investments (continued)

	2020	2019
Items at Fair Value		
Portfolio investments designated to the fair value category	\$ 1,437,534	\$ 2,232,488
Portfolio investments in equity instruments that are quoted in an active market	44,554	45,866
Derivatives	1,253	714
	\$ 1,483,341	\$ 2,279,068

Included in portfolio investments is \$233,282 (March 31, 2019 – \$212,323) that is restricted for use as per the requirements in Sections 99 and 100 of the *Insurance Act* of Alberta. Endowment principal included in portfolio investments amounts to \$75,438 (March 31, 2019 – \$75,157).

The following are the total net remeasurement gains on portfolio investments:

	2020	2019
Accumulated remeasurement gains	\$ 3,832	\$ 34,469
Restricted unrealized net gains attributable to unexpended deferred operating revenue (Note 14(b))	23,677	42,351
	\$ 27,509	\$ 76,820

Fair Value Hierarchy

	2020			
	Level 1	Level 2	Level 3	Total
Cash held for investing purposes	\$ 112,072	\$ -	\$ -	\$ 112,072
Interest bearing securities:				
Money market securities	-	38,076	-	38,076
Fixed income securities	-	827,477	75,213	902,690
Equities:				
Canadian equity investments	44,554	66,937	-	111,491
Global equity funds	-	278,175	-	278,175
Real estate pooled funds	-	-	40,837	40,837
	\$ 156,626	\$ 1,210,665	\$ 116,050	\$ 1,483,341
Percent of total	10%	82%	8%	100%

	2019			
	Level 1	Level 2	Level 3	Total
Cash held for investing purposes	\$ 110,887	\$ -	\$ -	\$ 110,887
Interest bearing securities:				
Money market securities	-	177,199	-	177,199
Fixed income securities	-	1,320,899	164,738	1,485,637
Equities:				
Canadian equity investments	45,866	129,525	-	175,391
Global equity funds	-	329,954	-	329,954
	\$ 156,753	\$ 1,957,577	\$ 164,738	\$ 2,279,068
Percent of total	7%	86%	7%	100%

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Notes to the Consolidated Financial Statements

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Note 10 Portfolio Investments (continued)

Reconciliation of Investments classified as level 3.

	2020		
	Fixed income securities	Real estate pooled funds	Total
Balance at beginning of year	\$ 164,738	\$ -	\$ 164,738
Purchases	4,763	40,268	45,031
Sales	(94,716)	-	(94,716)
Gain (loss) included in the Consolidated Statement of Remeasurement Gains and Losses	447	569	1,016
Transfers in (out)	(19)	-	(19)
Balance at end of year	\$ 75,213	\$ 40,837	\$ 116,050

	2019		
	Fixed income securities	Real estate pooled funds	Total
Balance at beginning of year	\$ 147,014	\$ -	\$ 147,014
Purchases	13,861	-	13,861
Sales	-	-	-
Gain (loss) included in the Consolidated Statement of Remeasurement Gains and Losses	1,044	-	1,044
Transfers in (out)	2,819	-	2,819
Balance at end of year	\$ 164,738	\$ -	\$ 164,738

Note 11 Accounts Receivable

	2020			2019
	Gross	Allowance for Doubtful Accounts	Net	Net (Note 26)
Patient accounts receivable	\$ 134,190	\$ 34,214	\$ 99,976	\$ 106,526
AH operating transfers receivable	268,119	-	268,119	36,768
AH capital transfers receivable	40,707	-	40,707	34,356
Other operating transfers receivable	21,667	-	21,667	14,824
Other capital transfers receivable	62,716	-	62,716	79,291
Other accounts receivable	118,188	802	117,386	94,957
	\$ 645,587	\$ 35,016	\$ 610,571	\$ 366,722

Accounts receivable are unsecured and non-interest bearing. At March 31, 2019, the total allowance for doubtful accounts was \$28,966.

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Notes to the Consolidated Financial Statements

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Note 12 Accounts Payable and Accrued Liabilities

	2020	2019
Payroll remittances payable and related accrued liabilities	\$ 611,513	\$ 543,427
Trade accounts payable and accrued liabilities	636,041	608,740
Provision for unpaid claims ^(a)	208,830	202,511
Obligations under capital leases ^(b)	103,990	97,053
Other liabilities	67,210	54,142
	\$ 1,627,584	\$ 1,505,873

Accounts payable and accrued liabilities includes payables related to the purchase of tangible capital assets of \$305,843 (2019 – \$284,970). Of these amounts, \$13,002 (2019 – \$12,721) comprise life lease deposits received from tenants of certain AHS' long term care facilities and amounts payable to the Ministry of Infrastructure of \$109,150 (2019 – \$65,000) related to a project funded by debt.

- (a) Provision for Unpaid Claims is an estimate of liability claims against AHS. It is influenced by factors such as historical trends involving claim payment patterns, pending levels of unpaid claims, claims severity and claim frequency patterns.

The provision has been estimated using the discounted value of claim liabilities using a discount rate of 2.25% (2019 – 2.45%) plus a provision for adverse deviation, based on actuarial estimates. – deviation, based on actuarial estimates.

- (b) Obligations under capital leases include a site lease with the University of Calgary, vehicle leases, obligations related to a clinical information system and site leases for ambulance services.

The obligations will be settled between 2020 and 2036 and have an implicit interest rate payable ranging from 1.93% to 5.07% (2019 – 2.37% to 7.04%).

AHS is committed to making payments for obligations under capital leases as follows:

Year ended March 31	Minimum Contract Payments
2021	\$ 29,041
2022	27,427
2023	14,536
2024	4,738
2025	4,364
Thereafter	40,328
	120,434
Less: interest	(16,444)
	\$ 103,990

(c) Liability for Contaminated Sites

At March 31, 2020, AHS has not identified or accepted any liability for contaminated sites (2019 – \$nil).

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Note 13 Employee Future Benefits

	2020	2019
Accrued vacation pay	\$ 582,819	\$ 566,415
Accumulating non-vesting sick leave ^(a)	124,652	122,081
Registered defined benefit pension plans ^{(b) (c)}	-	-
	\$ 707,471	\$ 688,496

(a) Accumulating Non-Vesting Sick Leave

Sick leave benefits are paid by AHS; there are no employee contributions and no assets set aside to support the obligation.

	2020	2019
Funded status – deficit	\$ 153,419	\$ 103,857
Unamortized net actuarial (loss) gain	(28,767)	18,224
Accrued benefit liability	\$ 124,652	\$ 122,081

Key assumptions used in the determination of the accumulating non-vesting sick leave liability are:

	2020	2019
Estimated average remaining service life	13 years	13 years
Draw down rate of accumulated non-vesting sick leave bank	18.30%	16.50%
Discount rate – beginning of year	3.51%	3.38%
Discount rate – end of year	2.14%	3.51%
Rate of compensation increase per year	2019-20	2018-19
	0.75%	0.75%
	2020-21	2019-20
	0.75%	0.75%
	Thereafter	Thereafter
	2.75%	2.75%

(b) Local Authorities Pension Plan (LAPP)

(i) AHS Participation in the LAPP

The majority of AHS employees participate in the LAPP. AHS' employees comprise approximately 46% (2019-46%) of the total membership in LAPP. AHS is not responsible for future funding of the plan deficit other than through contribution increases. As AHS is exposed to the risk of contribution rate increases, the following disclosure is provided to explain this risk.

The LAPP provides for a pension of 1.4% for each year of pensionable service based on the average salary of the highest five consecutive years up to the average Canada Pension Plan's Year's Maximum Pensionable Earnings (YMPE), over the same five consecutive year period and 2.0% on the excess, subject to the maximum pension benefit limit allowed under the Income Tax Act (Canada). The maximum pensionable service allowable under the plan is 35 years.

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Note 13 Employee Future Benefits (continued)

(ii) LAPP Surplus

LAPP carried out an actuarial valuation as at December 31, 2018 and these results were then extrapolated to December 31, 2019. The LAPP's December 31, 2019 net assets available for benefits divided by the LAPP's pension obligation shows that the LAPP is 119% (2018 – 108%) funded.

	December 31, 2019	December 31, 2018
LAPP net assets available for benefits	\$ 50,520,461	\$ 44,468,547
LAPP pension obligation	42,607,200	40,999,200
LAPP surplus	\$ 7,913,261	\$ 3,469,347

The LAPP net assets available for benefits are subject to significant market volatility due to the economic crisis stemming from the global COVID-19 pandemic. To the extent that the pension plans may not recover market-losses during the remainder of 2020, the funded status of the plans would experience a correlated decline. The financial market impact of the outbreak has been rapidly evolving, which precludes a reasonable estimate of the impact.

The 2020 and 2019 LAPP contribution rates are as follows:

Calendar 2020		Calendar 2019	
Employer	Employees	Employer	Employees
9.39% of pensionable earnings up to the YMPE and 13.84% of the excess	8.39% of pensionable earnings up to the YMPE and 12.84% of the excess	9.39% of pensionable earnings up to the YMPE and 13.84% of the excess	8.39% of pensionable earnings up to the YMPE and 12.84% of the excess

(c) Pension Expense

	2020	2019
Local Authorities Pension Plan	\$ 547,168	\$ 556,609
Defined contribution pension plans and group RRSPs	45,029	48,408
Supplemental Pension Plan	2,095	2,265
Management Employees Pension Plan	378	378
	\$ 594,670	\$ 607,660

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Notes to the Consolidated Financial Statements

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Note 14 Unexpended Deferred Operating Revenue

(a) Changes in the unexpended deferred operating revenue balance are as follows:

	2020				2019
	AH	Other Government ⁽ⁱ⁾	Donors and Non-Government	Total	Total
Balance, beginning of year	\$ 155,302	\$ 29,579	\$ 268,338	\$ 453,219	\$ 420,245
Received or receivable during the year, net of repayments	1,262,036	59,538	171,893	1,493,467	1,417,630
Unexpended deferred operating revenue returned	(13,061)	(60)	(1,013)	(14,134)	(2,496)
Restricted investment income	1,179	1,718	10,231	13,128	9,465
Transferred from unexpended deferred capital revenue ⁽ⁱⁱ⁾	2,265	28,227	1,761	32,253	57,237
Recognized as revenue	(1,290,302)	(92,152)	(157,289)	(1,539,743)	(1,434,092)
Miscellaneous other revenue recognized	(1,333)	(2,384)	(9,848)	(13,565)	(22,463)
	116,086	24,466	284,073	424,625	445,526
Changes in unrealized net gains attributable to portfolio investments related to endowments and unexpended deferred operating revenue	(1,025)	553	(18,202)	(18,674)	7,693
Balance, end of year	\$ 115,061	\$ 25,019	\$ 265,871	\$ 405,951	\$ 453,219

⁽ⁱ⁾ The balance at March 31, 2020 for other government includes \$1,007 of unexpended deferred operating revenue received from government entities outside the GOA (March 31, 2019 – \$506). The remaining balance in other government all relates to the GOA, see Note 21.

⁽ⁱⁱ⁾ The transfer is mainly comprised of restricted capital funding that was used for approved expenditures that did not meet the definition of a tangible capital asset

(b) The unexpended deferred operating revenue balance at the end of the year is externally restricted for the following purposes:

	2020				2019
	AH	Other Government	Donors and Non-Government	Total	Total
Research and education	\$ 8,811	\$ 2,505	\$ 165,575	\$ 176,891	\$ 182,763
Physician revenue and alternate relationship plans	36,375	309	-	36,684	37,070
Primary Care Networks	24,778	-	-	24,778	24,354
Long term care partnerships	-	15,093	-	15,093	18,329
Promotion, prevention and community	2,405	1,179	345	3,929	15,777
Addiction and mental health	32,951	226	1,950	35,127	31,668
Support services	945	481	61,196	62,622	58,265
Others less than \$10,000	8,705	1,688	16,757	27,150	42,642
Unrealized net gain attributable to portfolio investments related to endowments and unexpended deferred operating revenue (Note 10)	91	3,538	20,048	23,677	42,351
	\$ 115,061	\$ 25,019	\$ 265,871	\$ 405,951	\$ 453,219

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Notes to the Consolidated Financial Statements

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Note 15 Unexpended Deferred Capital Revenue

(a) Changes in the unexpended deferred capital revenue balance are as follows:

	2020				2019 (Note 26)
	AH	Other Government ⁽ⁱ⁾	Donors and Non- Government	Total	Total
Balance, beginning of year	\$ 53,862	\$ 6,173	\$ 68,359	\$ 128,394	\$ 124,289
Received or receivable during the year	170,756	118,764	43,266	332,786	349,293
Unexpended deferred capital revenue returned	(4,343)	-	(55)	(4,398)	(250)
Transfer to expended deferred capital revenue	(180,340)	(92,073)	(43,293)	(315,706)	(287,701)
Transferred to unexpended deferred operating revenue ⁽ⁱⁱ⁾	(2,265)	(28,227)	(1,761)	(32,253)	(57,237)
Balance, end of year	\$ 37,670	\$ 4,637	\$ 66,516	\$ 108,823	\$ 128,394

⁽ⁱ⁾ The balance at March 31, 2020 for other government all relates to the GOA, see Note 21.⁽ⁱⁱ⁾ The transfer is mainly comprised of restricted capital funding that was used for approved expenditures that did not meet the definition of a tangible capital asset

(b) The unexpended deferred capital revenue balance at the end of the year is externally restricted for the following purposes:

	2020	2019
AH		
Information systems	\$ 11,118	\$ 16,712
Medical Equipment Replacement Upgrade Program	1,457	2
Diagnostic equipment	18,189	19,585
Other equipment	6,906	17,563
Total AH	37,670	53,862
Other government		
Facilities and improvements	4,637	6,173
Total other government	4,637	6,173
Donors and non-government		
Equipment	59,809	59,849
Facilities and improvements	6,707	8,510
Total donors and non-government	66,516	68,359
	\$ 108,823	\$ 128,394

Alberta Health Services

Notes to the Consolidated Financial Statements

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Note 16 Expended Deferred Capital Revenue

Changes in the expended deferred capital revenue balance are as follows:

	2020				2019
	AH	Other Government ⁽ⁱ⁾	Donors and Non-Government	Total	Total
Balance, beginning of year	\$ 340,429	\$ 6,392,303	\$ 192,386	\$ 6,925,118	\$ 6,735,454
Transferred from unexpended deferred capital revenue	180,340	92,073	43,293	315,706	287,701
Contributed tangible capital assets	-	523,196	-	523,196	285,368
Less: amounts recognized as revenue	(61,354)	(308,581)	(34,470)	(404,405)	(383,405)
Balance, end of year	\$ 459,415	\$ 6,698,991	\$ 201,209	\$ 7,359,615	\$ 6,925,118

⁽ⁱ⁾ The balance at March 31, 2020 for other government includes \$20 of expended deferred capital revenue received from government entities outside the GOA (March 31, 2019 – \$36). The remaining balance in other government all relates to the GOA, see Note 21.

Note 17 Debt

	2020	2019
Debentures payable ^(a) :		
Parkade loan #1	\$ 23,447	\$ 26,500
Parkade loan #2	22,984	25,522
Parkade loan #3	31,338	34,048
Parkade loan #4	124,680	132,577
Parkade loan #5	30,388	32,200
Parkade loan #6	21,576	22,557
Parkade loan #7	47,479	49,516
Parkade loan #8	157,000	-
Energy savings initiative loan	22,336	24,089
Other	323	633
	\$ 481,551	\$ 347,642

(a) AHS issued debentures to Alberta Capital Financing Authority (ACFA), a related party, to finance the construction of parkades. AHS has pledged revenue derived directly or indirectly from the operations of all parking facilities being built, renovated, owned, and operated by AHS as security for these debentures.

AHS issued a debenture to ACFA relating to an energy savings initiative. AHS has pledged the mortgage on the Royal Alexandra hospital Lands and Alberta Hospital Lands as security for this debenture.

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Note 17 Debt (continued)

AHS is in compliance with all performance requirements of its debenture loans as at March 31, 2020.

The maturity dates and interest rates for the debentures are as follows:

	Maturity Date	Fixed Interest Rate
Parkade loan #1	September 2026	4.4025%
Parkade loan #2	September 2027	4.3870%
Parkade loan #3	March 2029	4.9150%
Parkade loan #4	September 2031	4.9250%
Parkade loan #5	June 2032	4.2280%
Parkade loan #6	December 2035	3.6090%
Parkade loan #7	March 2038	2.6400%
Parkade loan #8	December 2059	3.6010%
Energy savings initiative loan	December 2030	2.4160%
Other	March 2021	4.6000%

- (b) As at March 31, 2020, AHS holds a \$220,000 (March 31, 2019 - \$220,000) revolving demand facility with a Canadian chartered bank which may be used for operating purposes. Draws on the facility bear interest at the bank's prime rate less 0.50% per annum. As at March 31, 2020, AHS has \$nil (March 31, 2019 - \$nil) draws against this facility.

AHS also has access to a \$33,000 (March 31, 2019 - \$33,000) revolving demand letter of credit facility which may be used to secure AHS' obligations to third parties. At March 31, 2020, AHS has \$4,687 (March 31, 2019 - \$4,419) in a letter of credit outstanding against this facility. AHS is in compliance with the terms of the agreement relating to the letter of credit, therefore no liability has been recorded.

- (c) AHS is committed to making payments as follows:

Year Ended March 31	Principal Payments	Interest Payments	Total
2021	\$ 25,893	\$ 18,974	\$ 44,867
2022	26,666	17,866	44,532
2023	27,811	16,721	44,532
2024	29,008	15,524	44,532
2025	30,258	14,274	44,532
Thereafter	341,915	144,874	486,789
	\$ 481,551	\$ 228,233	\$ 709,784

During the year, the total interest related to debt was \$15,864 (2019 - \$15,199).

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Note 18 Tangible Capital Assets

Cost	2019	Additions ^(a)	Transfers	Disposals/write-downs ^(b)	2020
Facilities and improvements	\$ 9,401,390	\$ 1,867	\$ 244,882	\$ (2,839)	\$ 9,645,300
Work in progress	1,625,941	909,759	(763,719)	(27,293)	1,744,688
Equipment ^(c)	2,561,156	106,152	3,591	(47,283)	2,623,616
Information systems	1,474,803	17,073	389,281	(53,358)	1,827,799
Building service equipment	729,544	-	110,662	(84)	840,122
Land ^(d)	116,823	133	-	(30)	116,926
Leased facilities and improvements	229,874	21,695	3,830	(6)	255,393
Land improvements	94,188	-	11,473	(80)	105,581
	\$ 16,233,719	\$ 1,056,679	\$ -	\$ (130,973)	\$ 17,159,425

Accumulated Amortization	2019	Amortization Expense	Effect of Transfers	Disposals/write-downs ^(b)	2020
Facilities and improvements	\$ 3,915,175	\$ 263,455	\$ -	\$ (2,729)	\$ 4,175,901
Work in progress	-	-	-	-	-
Equipment ^(c)	1,933,533	147,466	-	(46,288)	2,034,711
Information systems	1,320,894	77,772	-	(41,877)	1,356,789
Building service equipment	434,531	42,533	-	(84)	476,980
Land ^(d)	-	-	-	-	-
Leased facilities and improvements	179,731	7,308	-	(6)	187,033
Land improvements	68,851	3,280	-	(80)	72,051
	\$ 7,852,715	\$ 541,814	\$ -	\$ (91,064)	\$ 8,303,465

	Net Book Value	
	2020	2019
Facilities and improvements	\$ 5,469,399	\$ 5,486,215
Work in progress	1,744,688	1,625,941
Equipment ^(c)	588,905	627,623
Information systems	471,010	153,909
Building service equipment	363,142	295,013
Land ^(d)	116,926	116,823
Leased facilities and improvements	68,360	50,143
Land improvements	33,530	25,337
	\$ 8,855,960	\$ 8,381,004

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Note 18 Tangible Capital Assets (continued)

Cost	2018	Additions ^(a)	Transfers	Disposals/write-downs ^(b)	2019
Facilities and improvements	\$ 9,300,463	\$ -	\$ 108,258	\$ (7,331)	\$ 9,401,390
Work in progress	1,179,069	692,879	(246,007)	-	1,625,941
Equipment ^(c)	2,512,888	164,704	(923)	(115,513)	2,561,156
Information systems	1,438,547	21,742	46,064	(31,550)	1,474,803
Building service equipment	648,352	-	81,747	(555)	729,544
Land ^(d)	116,875	-	-	(52)	116,823
Leased facilities and improvements	229,065	-	809	-	229,874
Land improvements	84,197	-	10,052	(61)	94,188
	\$ 15,509,456	\$ 879,325	\$ -	\$ (155,062)	\$ 16,233,719

Accumulated Amortization	2018	Amortization Expense	Effect of Transfers	Disposals/write-downs	2019
Facilities and improvements	\$ 3,666,479	\$ 255,001	\$ -	\$ (6,305)	\$ 3,915,175
Work in progress	-	-	-	-	-
Equipment ^(c)	1,898,695	148,701	-	(113,863)	1,933,533
Information systems	1,277,120	75,320	-	(31,546)	1,320,894
Building service equipment	398,200	36,884	-	(553)	434,531
Land ^(d)	-	-	-	-	-
Leased facilities and improvements	172,370	7,361	-	-	179,731
Land improvements	65,285	3,627	-	(61)	68,851
	\$ 7,478,149	\$ 526,894	\$ -	\$ (152,328)	\$ 7,852,715

	Net Book Value	
	2019	2018
Facilities and improvements	\$ 5,486,215	\$ 5,633,984
Work in progress	1,625,941	1,179,069
Equipment ^(c)	627,623	614,193
Information systems	153,909	161,427
Building service equipment	295,013	250,152
Land ^(d)	116,823	116,875
Leased facilities and improvements	50,143	56,695
Land improvements	25,337	18,912
	\$ 8,381,004	\$ 8,031,307

(a) Contributed Tangible Capital Assets

Additions include total contributed tangible capital assets of \$523,196 (2019 – \$285,368) consisting of \$523,196 from AI (2019 – \$285,322) and \$nil from other sources (2019 – \$46).

(b) Write-Downs

Write-downs include work in progress of \$22,615 (2019 - \$ nil) related to the cancelled Edmonton Lab Hub project.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 18 Tangible Capital Assets (continued)

(c) Leased Equipment

Equipment includes tangible capital assets acquired through capital leases at a cost of \$22,884 (2019 – \$17,240) with accumulated amortization of \$12,152 (March 31, 2019 – \$12,119). For the year ended March 31, 2020, leased equipment included a net increase of \$6,707 related to vehicles under capital leases (2019 – net increase of \$4,363).

(d) Leased Land

Land at the following sites has been leased to AHS at nominal values:

Site	Leased from	Lease Expiry
Evansburg Community Health Centre	Yellowhead County	April 2031
Jasper Healthcare Centre	Parks Canada	January 2034
Bethany Care Centre	Red Deer College	April 2034
Myrnam Land	Eagle Hill Foundation	May 2038
Two Hills Helipad	Stella Stefiuk	August 2041
McConnell Place North	City of Edmonton	September 2044
Northeast Community Health Centre	City of Edmonton	February 2047
Foothills Medical Centre Parkade	University of Calgary	July 2054
Alberta Children's Hospital	University of Calgary	December 2103
Kaye Edmonton Clinic (Parcel H)	The University of Alberta	February 2109

Alberta Health Services

Notes to the Consolidated Financial Statements

For the year ended March 31, 2020

(thousands of dollars)

Note 19 Accumulated Surplus

Accumulated surplus is comprised of the following:

	2020						2019
	Unrestricted Surplus	Invested in Tangible Capital Assets ^(a)	Endowments ^(b)	Internally Restricted Surplus for Future Purposes ^(c)	Internally Restricted Surplus for Insurance Equity Requirements and	Total	Total
Balance, beginning of year	\$ 42,108	\$ 940,283	\$ 75,157	\$ 152,842	\$ 68,034	\$ 1,278,424	\$ 1,317,055
Annual operating deficit	(145,823)	-	-	-	-	(145,823)	(38,631)
Tangible capital assets acquired with internal funds	(127,589)	151,391	-	(23,802)	-	-	-
Amortization of internally funded tangible capital assets	177,318	(177,318)	-	-	-	-	-
Principal payments on debt	(23,091)	23,091	-	-	-	-	-
Payments on obligations under capital leases	(3,204)	3,204	-	-	-	-	-
Net receipt of life lease deposits	281	(281)	-	-	-	-	-
Transfer of internally restricted surplus	129,040	-	-	(129,040)	-	-	-
Transfer of insurance equity requirements and foundations surpluses	(14,342)	-	-	-	14,342	-	-
Transfer of endowment contributions	(281)	-	281	-	-	-	-
Balance, end of year	\$ 34,417	\$ 940,370	\$ 75,438	\$ -	\$ 82,376	\$ 1,132,601	\$ 1,278,424

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 19 Accumulated Surplus (continued)

(a) Invested in Tangible Capital Assets

The accumulated surplus invested in tangible capital assets represents the net book value of tangible capital assets that have previously been purchased with AHS' unrestricted surplus.

Reconciliation of invested in tangible capital assets:

	2020	2019
Tangible Capital Assets (Note 18)	\$ 8,855,960	\$ 8,381,004
Less funded by:		
Expended Deferred Capital Revenue (Note 16)	(7,359,615)	(6,925,118)
Debt (Note 17)	(481,551)	(347,642)
Unexpended Debt	42,568	6,813
Other funding ⁽ⁱ⁾	-	(65,000)
Obligations under capital leases (Note 12b)	(103,990)	(97,053)
Life lease deposits (Note 12)	(13,002)	(12,721)
	\$ 940,370	\$ 940,283

- (i) Other funding comprises in substance, tangible capital assets financing provided to AHS by the Ministry of Infrastructure as accounts payable of \$109,150 (2019 - \$65,000) (Note 12), offset by unexpended debt proceeds of \$109,150 (2019 - \$nil) earmarked to settle the liability.

(b) Endowments

Endowments represent the portion of accumulated surplus that is restricted and must be maintained in perpetuity. Transfers of endowment contributions from unrestricted surplus include \$281 (2019 - \$463) of contributions received in the year (Note 5).

(c) Internally Restricted Surplus for Future Purposes

The restriction of accumulated surplus for AHS' future operating and capital purposes is as follows:

- (i) Ancillary services surplus of \$nil (2019 - \$112,508) comprising ancillary operation surpluses from parking, retail food services, and controlled entities. Transfers in the year include \$23,802 to invested in tangible capital assets representing tangible capital assets acquired with the funds and \$88,706 to unrestricted surplus to close the account.
- (ii) Other surplus of \$nil (2019 - \$40,334) comprising surpluses set aside to address funding of expenses for certain initiatives spanning multiple fiscal years. Transfers in the year include \$40,334 to unrestricted surplus to close the account.

(d) Internally Restricted Surplus for Insurance Equity Requirements and Foundations

Insurance equity requirements comprise surpluses of \$28,237 (2019 - \$15,725) related to equity of the LPIP mainly relating to legislative requirements per the Insurance Act. Foundations comprise surpluses amounting to \$54,139 (2019 - \$52,309) related to donations received by AHS' Controlled Foundations without external restrictions attached.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 20 Contractual Obligations and Contingent Liabilities

(a) Contractual Obligations

Contractual obligations are AHS' obligations to others that will become liabilities in the future when the terms of those contracts or agreements are met.

The estimated aggregate amount payable for the unexpired terms of these contractual obligations are as follows:

Year ended March 31	Services ⁽ⁱ⁾	Other ⁽ⁱⁱ⁾	Operating Lease Payments	Capital Projects	Total ⁽ⁱⁱⁱ⁾
2021	\$ 2,698,884	\$ 505,868	\$ 58,184	\$ 181,085	\$ 3,444,021
2022	1,449,315	273,261	54,595	30,407	1,807,578
2023	1,085,695	161,159	47,375	-	1,294,229
2024	927,396	91,475	38,270	-	1,057,141
2025	704,878	50,599	28,935	-	784,412
Thereafter	7,296,428	85,456	70,499	-	7,452,383
	\$ 14,162,596	\$ 1,167,818	\$ 297,858	\$ 211,492	\$ 15,839,764

- (i) Service obligations mainly relate to contracts entered into with third parties for the provision of long-term care and home care services.
- (ii) Other obligations mainly relate to contracts entered into with third parties for maintenance, information technology services, software, equipment, acquisitions, and procurement of medical supplies and food.
- (iii) In 2019-20, AHS reassessed for disclosure, contractual obligations related to services, other obligations and capital projects. These amounts were not previously reported.

(b) Contingent Liabilities

AHS is subject to legal claims during its normal course of business. AHS records a liability when the assessment of a claim indicates that a future event is likely to confirm that an asset had been impaired or a liability incurred at the date of the financial statements and the amount of the contingent loss can be reasonably estimated.

Accruals have been made in specific instances where it is likely that losses will be incurred based on a reasonable estimate. As at March 31, 2020, accruals have been recorded as part of the provision for unpaid claims and other liabilities (Note 12). Included in this accrual are claims in which AHS has been jointly named with the Minister. The accrual provided for these claims under the provision for unpaid claims represents AHS' portion of the liability.

AHS has been named in 262 legal claims (2019 – 225 claims) related to conditions in existence at March 31, 2020 where the likelihood of the occurrence of a future event confirming a contingent loss is not reasonably determinable. Of these, 222 claims have \$498,678 in specified amounts and 40 have no specified amounts (2019 – 205 claims with \$415,883 of specified claims and 20 claims with no specified amounts). The resolution of indeterminable claims may result in a liability, if any, that is different than the claimed amount.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 21 Related Parties

Transactions with related parties are included within these consolidated financial statements, unless otherwise stated.

The Minister appoints all members of the AHS Board. The viability of AHS' operations depends on transfers from AH. Transactions between AHS and AH are reported and disclosed in the Consolidated Statement of Operations, the Consolidated Statement of Financial Position, and the Notes to the Consolidated Financial Statements, and are therefore excluded from the tables below.

Related parties also include key management personnel of AHS. AHS has defined key management personnel to include those disclosed in Sub-Schedule 2A & 2B of these consolidated financial statements. Related party transactions with key management personnel primarily consist of compensation related payments and are considered to be undertaken on similar terms and conditions to those adopted if the entities were dealing at arm's length.

AHS is considered to be a related party with those entities consolidated or included on a modified equity basis in the GOA consolidated financial statements. Entities consolidated or included on a modified equity basis have been grouped with their respective ministry and transactions between AHS and the other ministries are recorded at their exchange amount as follows:

	Revenues ^(a)		Expenses	
	2020	2019	2020	2019
Ministry of Advanced Education ^(b)	\$ 56,092	\$ 57,266	\$ 191,053	\$ 184,812
Ministry of Infrastructure ^(c)	343,065	340,892	222	1
Other ministries ^(d)	58,326	55,399	29,794	31,064
Total for the year	\$ 457,483	\$ 453,557	\$ 221,069	\$ 215,877

	Receivable from		Payable to	
	2020	2019	2020	2019
Ministry of Advanced Education ^(b)	\$ 6,545	\$ 7,692	\$ 33,967	\$ 35,618
Ministry of Infrastructure ^(c)	25,477	50,566	109,150	65,000
Other ministries ^(d)	12,128	8,483	488,080	349,886
Balance, end of year	\$ 44,150	\$ 66,741	\$ 631,197	\$ 450,504

(a) Revenues with GOA ministries include other government transfers of \$425,845 (2019 – \$420,622), (Note 4), other income of \$28,077 (2019 – \$30,847), (Note 6), and fees and charges of \$3,561 (2019 – \$2,088).

(b) Most of AHS transactions with the Ministry of Advanced Education relate to initiatives with the University of Alberta and the University of Calgary. These initiatives include teaching, research, and program delivery. A number of physicians are employed by either AHS or the universities but perform services for both. Due to proximity of locations, some initiatives result in sharing physical space and support services. The revenue and expense transactions are a result of funding provided from one to the other and recoveries of shared costs.

(c) The transactions with the Ministry of Infrastructure (AI) relate to the construction and funding of tangible capital assets. These transactions include operating transfers of \$35,271 (2019 – \$58,957) and recognition of expended deferred capital revenue of \$307,794 (2019 – \$281,935) relating to tangible capital assets with stipulations or external restrictions to utilize over their remaining useful lives. Not included in the table above but included in total amounts disclosed in Note 18(a) is the transfer of land and other tangible capital assets from AI of \$523,196 (2019 – \$285,322).

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 21 Related Parties (continued)

(d) The payable transactions with other ministries include the debt payable to ACFA (Note 17(a)).

At March 31, 2020, AHS has recorded deferred revenue from other ministries within the GOA, excluding AH, of \$24,012 (March 31, 2019 – \$29,073) related to unexpended deferred operating revenue (Note 14), \$4,637 (March 31, 2019 – \$6,173) related to unexpended deferred capital revenue (Note 15) and \$6,698,971 (March 31, 2019 – \$6,392,267) related to expended deferred capital revenue (Note 16).

Contingent liabilities in which AHS has been jointly named with other government entities within the GOA are disclosed in Note 20.

Note 22 Government Partnerships

AHS has proportionately consolidated 50% of the results of the PCNs and NACTRC and 33.33% of the results in iRSM. The following is 100% of the financial position and results of operations for AHS' government partnerships.

	2020	2019
Financial assets (portfolio investments, accounts receivable, other assets)	\$ 74,273	\$ 72,548
Liabilities (trade accounts payable, unexpended deferred operating revenue)	74,273	72,548
Accumulated surplus	\$ -	\$ -
Total revenues	\$ 260,975	\$ 248,538
Total expenses	260,975	248,538
Annual surplus	\$ -	\$ -

Note 23 Trusts under Administration

(a) Health Benefit Trust of Alberta (HBTA)

AHS is one of more than 30 participants in the HBTA and has a majority of representation on the HBTA governance board. The HBTA is a formal health and welfare trust established under a Trust Agreement effective January 1, 2000. The HBTA provides health and other related employee benefits pursuant to the authorizing Trust Agreement.

HBTA's balances as at March 31 are as follows:

	2020	2019
Financial assets	\$ 128,181	\$ 155,705
Liabilities	17,486	16,401
Net financial assets	\$ 110,695	\$ 139,304
Non-financial assets	4	3
Net assets	\$ 110,699	\$ 139,307

AHS has included in prepaid expenses \$74,828 (2019 – \$93,784) representing in substance a prepayment of future premiums to HBTA. For the fiscal year ended March 31, 2020, AHS paid premiums of \$407,512 (2019 – \$391,734) which is approximately 98% (2019 – 98%) of the total premiums received by HBTA.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 23 Trusts under Administration (continued)

(b) Other Trust Funds

AHS receives funds in trust for research and development, education, and other programs. These amounts are held and administered on behalf of others in accordance with the terms and conditions embodied in the relevant agreements with no unilateral power to appropriate the funds or to change the conditions set out in the trust indenture (or agreement) and therefore are not reported in these consolidated financial statements. As at March 31, 2020, the balance of funds held in trust by AHS for research and development is \$100 (March 31, 2019 – \$100).

AHS receives funds in trust from continuing care residents for personal expenses. As at March 31, 2020, the balance of these funds is \$1,390 (March 31, 2019 – \$1,452). These amounts are not included in the consolidated financial statements.

AHS and a third party trustee administer the SERP in accordance with a retirement compensation arrangement trust agreement. As at March 31, 2020, there are \$29,181 in plan assets (March 31, 2019 - \$32,674). These amounts are not included in the consolidated financial statements

Note 24 Segment Disclosure

The Consolidated Schedule of Segment Disclosures – Schedule 3 is intended to enable users to better understand the reporting entity and identify the resources allocated to the major activities of AHS.

AHS' revenues, as reported on the Consolidated Statement of Operations, are most informatively presented by source and are not reasonably assignable to the reportable segments. For each reported segment, the expenses are directly or reasonably attributable to the segment.

The segments have been selected based on the presentation that is adopted for the financial reporting, planning and budget processes, and represent the major distinguishable activities of AHS.

Segments include:

(a) Continuing care

Continuing care is comprised of long-term care including chronic and psychiatric care in facilities operated by AHS and contracted providers.

(b) Community care

Community care includes supportive living, palliative and hospice care, and community programs including Primary Care Networks, Family Care Clinics, urgent care centres, and community mental health. This segment excludes community-based dialysis, oncology, and surgical services.

(c) Home care

Home care is comprised of home nursing and support.

(d) Acute care

Acute care is comprised predominantly of patient care units such as medical, surgical, intensive care, obstetrics, pediatrics, mental health, emergency, day/night care, clinics, day surgery, and contracted surgical services. This segment also includes operating and recovery rooms.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 24 Segment Disclosure (continued)

(e) Ambulance services

Ambulance services is comprised of ground ambulance, air ambulance, patient transport, and Emergency Medical Services (EMS) central dispatch. AHS also supports community paramedic programs, as well as other programs that support the learning, development, quality and safety of EMS professionals.

(f) Diagnostic and therapeutic services

Diagnostic and therapeutic services support and provide care for patients through clinical lab (both in the community and acute settings), diagnostic imaging, pharmacy, acute and therapeutic services such as physiotherapy, occupational therapy, respiratory therapy, and speech language pathology.

(g) Population and public health

Population and public health is comprised primarily of health promotion, disease and injury prevention, and health protection.

(h) Research and education

Research and education is comprised primarily of costs pertaining to formally organized health research and graduate medical education, primarily funded by donations, and third party contributions.

(i) Information technology

Information technology is comprised of costs pertaining to the provision of services to design, develop, implement, and maintain effective and efficient management support systems in the areas of data processing, systems engineering, technical support, and systems research and development.

(j) Support services

Support services is comprised of building maintenance operations (including utilities), materials management (including purchasing, central warehousing, distribution and sterilization), housekeeping, patient registration, health records, food services, and emergency preparedness.

(k) Administration

Administration is comprised of human resources, finance, communications and general administration, as well as a share of administration of certain contracted health service providers. General administration includes senior executives and many functions such as planning and development, infection control, quality assurance, patient safety, insurance, privacy, risk management, internal audit, and legal.

Alberta Health Services

Notes to the Consolidated Financial Statements

(thousands of dollars)

Note 25 Impact of COVID-19

The World Health Organization assessed the novel strain of coronavirus, COVID-19, a global pandemic and recommended containment measures worldwide. On March 17, 2020, a state of public health emergency was declared in Alberta.

In response to the global pandemic, AH provided AHS with \$23,317 in funding up to March 31, 2020. These funds were used for incremental expenditures arising from testing, surveillance and treatment of patients. Assessment and treatment centers were also set up to assist with the anticipated demand of COVID-19 related healthcare services. Expenditures were also incurred for hospital supplies and equipment, including personal protective equipment.

Subsequent to March 31, 2020, AHS has received additional funding from AH to assist in the continued response to COVID-19. To date, the following impacts of the pandemic have been identified:

- Additional measures relating to the provision of personal protective equipment for the overall safety of Albertans. This has entailed the entering into of significant purchase commitments, including payment of deposits to secure essential supplies and equipment for both AHS and third party health service providers within Alberta. At the direction of the Minister, AHS also acquired personal protective equipment to provide to other provinces.
- Delays or deferrals of certain health care related services
- Increases in funding provided to third party service providers, especially long term care providers which have been significantly impacted by COVID-19
- Operation of assessment and treatment centers as well as increased laboratory testing relating to COVID-19
- Redeployment of parts of the AHS workforce as the organization responds to measures such as contact tracing and testing, remote solutions such as virtual health and health link support.
- Delays in the implementation of certain information systems initiatives
- Delays in negotiations and settlement of open contracts relating to the AHS labor workforce
- Potential declines in the value of marketable securities held by AHS as described in Note 9
- Increased uncertainty relating to recoverability of outstanding accounts receivable balances
- Temporary suspension of parking fees
- Receipt of donated ventilators, supplies and personal protective equipment as Albertans come together to assist in the response to COVID-19

As Alberta progresses through the first wave of COVID-19, AHS continues to closely monitor the COVID-19 developments. Overall, as the response is ongoing and an end to the pandemic is indeterminable, the related financial and operational impacts of the pandemic cannot be reliably estimated at this time.

Note 26 Corresponding Amounts

In the current year, AHS determined that certain immaterial donor contributions related to the purchase of equipment had been recorded before the contributions were authorized by the donor. As a result, AHS has retrospectively adjusted certain consolidated financial statement accounts as at and for the year ended March 31, 2019 including accounts receivable and unexpended deferred capital revenue which have been reduced by \$78,485 to more appropriately reflect the timing of the donor authorizations.

Certain other corresponding amounts have been reclassified to conform to the 2020 consolidated financial statement presentation.

Note 27 Approval of Consolidated Financial Statements

The consolidated financial statements were approved by the AHS Board on June 24, 2020.

Alberta Health Services

Schedule 1 – Consolidated Schedule of Expenses by Object

For the year ended March 31, 2020

(thousands of dollars)

	2020		2019
	Budget (Note 3)	Actual	Actual
Salaries and benefits (Schedule 2)	\$ 8,327,000	\$ 8,530,683	\$ 8,321,637
Contracts with health service providers	2,781,000	2,823,741	2,749,686
Contracts under the Health Care Protection Act	20,000	20,041	17,186
Drugs and gases	501,000	560,661	506,662
Medical supplies	588,000	581,490	592,517
Other contracted services	1,378,000	1,319,640	1,322,806
Other ^(a)	1,216,000	1,196,379	1,273,213
Amortization and loss on disposals/write-downs of tangible capital assets (Note 18)	554,000	581,723	529,628
	\$ 15,365,000	\$ 15,614,358	\$ 15,313,335
(a) Significant amounts included in Other are:			
Equipment expense	\$ 203,000	\$ 223,174	\$ 212,312
Building rent	121,000	126,387	112,921
Utilities	109,000	115,577	118,372
Housekeeping, laundry and linen, plant maintenance and biomedical engineering supplies	88,000	86,877	90,608
Building and ground expenses	126,000	85,683	118,206
Food and dietary supplies	81,000	80,855	80,827
Office supplies	53,000	60,311	63,279
Fundraising and grants awarded	54,000	52,298	51,336
Minor equipment purchases	56,000	51,922	59,246
Insurance and liability claims	46,000	42,439	90,867
Telecommunications	39,000	39,983	38,917
Travel	54,000	39,809	45,423
Licenses, fees and memberships	18,000	21,650	17,211
Education	18,000	11,893	12,428
Other	150,000	157,521	161,260
	\$ 1,216,000	\$ 1,196,379	\$ 1,273,213

Alberta Health Services

Schedule 2 – Consolidated Schedule of Salaries and Benefits

For the year ended March 31, 2020

(thousands of dollars)

	2020								2019	
	FTE ^(a)	Base Salary ^(b)	Other Cash Benefits ^(c)	Other Non-Cash Benefits ^(d)	Subtotal	Severance ^(e)		Total	FTE ^(a)	Total
						Number of Individuals	Amount			
Total Board (Sub-Schedule 2A)	10.87	\$ -	\$ 336	\$ -	\$ 336	-	\$ -	\$ 336	10.82	\$ 334
Total Executive (Sub-Schedule 2B)	13.95	5,234	58	843	6,135	1	557	6,692	14.00	6,028
Management Reporting to CEO Direct Reports	56.46	13,452	345	2,420	16,217	-	-	16,217	56.86	15,797
Other Management	3,120.38	372,487	5,207	81,279	458,973	30	3,504	462,477	3,031.59	452,716
Medical Doctors not included above ^(f)	115.97	36,690	472	3,382	40,544	-	-	40,544	141.84	49,934
Regulated nurses not included above:										
RNs, Reg. Psych. Nurses, Grad Nurses	19,930.49	1,906,924	272,159	415,656	2,594,739	7	542	2,595,281	19,566.39	2,544,809
LPNs	5,432.11	358,937	46,374	79,747	485,058	2	86	485,144	5,233.39	464,883
Other health technical and professional	16,557.82	1,530,576	89,423	342,227	1,962,226	11	413	1,962,639	16,948.48	1,963,001
Unregulated health service providers	10,142.83	524,456	61,394	126,612	712,462	5	59	712,521	9,310.80	638,452
Other staff	27,672.17	1,758,794	104,964	382,326	2,246,084	70	3,187	2,249,271	27,251.74	2,186,104
Sub-total	83,053.05	6,507,550	580,732	1,434,492	8,522,774	126	8,348	8,531,122	81,565.91	8,322,058
Less amounts included in Other contracted services		(360)	(2)	(77)	(439)	-	-	(439)		(421)
Total		\$ 6,507,190	\$ 580,730	\$ 1,434,415	\$ 8,522,335	126	\$ 8,348	\$ 8,530,683		\$ 8,321,637

This schedule does not include \$29,799 in capitalized salaries and benefits (2019 - \$27,393).

The accompanying footnotes and sub-schedules are part of this schedule.

Alberta Health Services

Sub-Schedule 2A – Board Remuneration

For the year ended March 31, 2020

(thousands of dollars)

	Term	2020 Committees	2020 Remuneration	2019 Remuneration
Board Chairs^(g)				
Linda Hughes	Nov 27, 2015 to Aug 19, 2019	ARC, CEC, FC, GC, HRC, QSC	\$ 26	\$ 69
David Weyant	Since Aug 20, 2019	ARC, CEC, FC, GC, HRC, QSC	43	-
Board Members				
Dr. Brenda Hemmelgarn (Vice Chair)	Since Nov 27, 2015	CEC (Chair), HRC, QSC	49	49
David Carpenter	Since Nov 27, 2015	ARC (Chair), FC (Chair), HRC	34	35
Heather Crowshoe	Nov 3, 2016 to Nov 2, 2019	CEC, GC	17	30
Richard Dicerni	Since Nov 27, 2015	CEC, FC, HRC (Chair)	28	27
Robb Foote	Apr 12, 2018 to Feb 1, 2019	-	-	24
Linda Hughes	Aug 20, 2019 to Sep 30, 2019	-	1	-
Stephen Mandel	Since Sep 25, 2019	CEC, FC, QSC	16	-
Heidi Overguard	Since Sep 25, 2019	CEC, FC, GC, HRC	17	-
Hugh Sommerville	Since Nov 27, 2015	ARC, GC (Chair)	32	31
Marliss Taylor	Nov 27, 2015 to Oct 24, 2019	GC, HRC (Chair), QSC	18	34
Brian Vaasjo	Since Aug 20, 2019	ARC, FC, GC	19	-
Glenda Yeates	Since Nov 27, 2015	ARC, FC, QSC (Chair)	32	31
Board Committee Participants^(h)				
Dr. Brian Postl	Since Jan 1, 2018	QSC	2	2
Gord Winkel	Since Nov 27, 2015	QSC	2	2
Total Board			\$ 336	\$ 334

Board members were remunerated with monthly honoraria. In addition, they received remuneration for attendance at Board and committee meetings.

Board committees were established by the Board to assist in governing AHS and overseeing the management of AHS' business and affairs. Board committee participants are eligible to receive remuneration for meetings attended, and in addition Board committee chairs also receive a monthly honorarium.

Committee legend: ARC = Audit and Risk Committee, CEC = Community Engagement Committee, FC = Finance Committee, GC = Governance Committee, HRC = Human Resources Committee, QSC = Quality and Safety Committee

Alberta Health Services

Sub-Schedule 2B – Executive Salaries and Benefits

For the year ended March 31, 2020

(thousands of dollars)

For the Current Fiscal Year	2020						
	FTE ^(a)	Base Salary ^(b,i)	Other Cash Benefits ^(c)	Other Non-Cash Benefits ^(d)	Subtotal	Severance ^(e)	Total
Board Direct Reports							
Andrea Beckwith-Ferraton – Chief Ethics and Compliance Officer ^(q)	1.00	\$ 212	\$ 3	\$ 37	\$ 252	\$ -	\$ 252
Ronda White – Chief Audit Executive ^(q)	1.00	278	1	56	335	-	335
Dr. Verna Yiu – President and Chief Executive Officer ^(i,r)	1.00	576	-	78	654	-	654
CEO Direct Reports							
Dr. Francois Belanger – VP, Quality and Chief Medical Officer ^(q)	1.00	465	-	88	553	-	553
Dr. Ted Braun – VP and Medical Director, Clinical Operations ^(q)	1.00	399	-	61	460	-	460
Mauro Chies – VP, CancerControl Alberta and Clinical Support Services ^(q)	1.00	331	-	55	386	-	386
Sean Chilton – VP, Health Professions and Practice and Information Technology ^(k,q)	1.00	331	-	64	395	-	395
Todd Gilchrist – VP, People ^(l,q)	1.00	452	1	72	525	-	525
Deb Gordon – VP and Chief Operations Officer, Clinical Operations ^(q)	1.00	372	-	65	437	-	437
Robert Hawes – Interim VP, Corporate Services and Chief Financial Officer ^(m,s)	0.08	34	3	2	39	-	39
Brenda Huband – VP and Chief Health Operations Officer, Central and Southern Alberta ⁽ⁿ⁾	0.95	353	-	57	410	-	410
Dr. Mark Joffe – VP and Medical Director, CancerControl Alberta, Clinical Support Services and Provincial Clinical Excellence ^(o,t)	1.00	451	34	42	527	-	527
Deborah Rhodes – VP, Corporate Services and Chief Financial Officer ^(p,u)	0.92	357	1	51	409	557	966
Dr. Kathryn Todd – VP, Provincial Clinical Excellence ^(o,t)	1.00	291	15	45	351	-	351
Colleen Turner – VP, Community Engagement and Communications ^(q)	1.00	332	-	70	402	-	402
Total Executive	13.95	\$ 5,234	\$ 58	\$ 843	\$ 6,135	\$ 557	\$ 6,692

Alberta Health Services

Sub-Schedule 2B – Executive Salaries and Benefits

For the year ended March 31, 2020 (continued)

(thousands of dollars)

For the Prior Fiscal Year	2019						
	FTE ^(a)	Base Salary ^(b,i)	Other Cash Benefits ^(c)	Other Non-Cash Benefits ^(d)	Subtotal	Severance ^(e)	Total
Board Direct Reports							
Andrea Beckwith-Ferraton – Chief Ethics and Compliance Officer	1.00	\$ 211	\$ 3	\$ 34	\$ 248	\$ -	\$ 248
Ronda White – Chief Audit Executive	1.00	276	2	43	321	-	321
Dr. Verna Yiu – President and Chief Executive Officer	1.00	572	-	84	656	-	656
CEO Direct Reports							
Dr. Francois Belanger – VP, Quality and Chief Medical Officer	1.00	462	-	62	524	-	524
Dr. Ted Braun – VP and Medical Director, Central and Southern Alberta	1.00	395	-	71	466	-	466
Mauro Chies – VP, CancerControl Alberta and Clinical Support Services	0.52	171	-	28	199	-	199
Sean Chilton – VP, Health Professions and Practice	1.00	329	-	55	384	-	384
Todd Gilchrist – VP, People, Legal and Privacy	1.00	448	1	62	511	-	511
Deb Gordon – VP and Chief Health Operations Officer, Northern Alberta	1.00	369	-	83	452	-	452
Karen Horon – Interim VP, Clinical Support Services	0.48	110	-	19	129	-	129
Brenda Huband – VP and Chief Health Operations Officer, Central and Southern Alberta	1.00	369	-	62	431	-	431
Dr. Mark Joffe – VP and Medical Director, Northern Alberta	1.00	447	35	41	523	-	523
Deborah Rhodes – VP, Corporate Services and Chief Financial Officer	1.00	388	1	54	443	-	443
Dr. Kathryn Todd – VP, System Innovations and Programs	1.00	289	15	43	347	-	347
Colleen Turner – VP, Community Engagement and Communications	1.00	329	-	65	394	-	394
Total Executive	14.00	\$ 5,165	\$ 57	\$ 806	\$ 6,028	\$ -	\$ 6,028

Alberta Health Services

Sub-Schedule 2C – Executive Supplemental Pension Plan and Supplemental Executive Retirement Plan

(thousands of dollars)

Certain employees will receive retirement benefits that supplement the benefits limited under the registered plans for service. The Supplemental Pension Plan (SPP) is a defined contribution plan and the Supplemental Executive Retirement Plan (SERP) is a defined benefit plan. The SERP is disclosed in Note 2(h)(i). The amounts in this table represent the total SPP and SERP benefits earned by the individual during the fiscal year. The current period benefit costs for SPP and the other costs for SERP included in other non-cash benefits disclosed in Sub-Schedule 2B are prorated for the period of time the individual was in their position directly reporting to the Board or directly reporting to the President and Chief Executive Officer. Only individuals holding a position directly reporting to the Board or President and Chief Executive Officer during the current fiscal year are disclosed.

	2020			2019			
	SPP	SERP					
	Current Period Benefit Costs ⁽¹⁾	Other Costs ⁽²⁾	Total	Total	Account Balance ⁽³⁾ or Accrued Benefit Obligation March 31, 2019	Change During the Year ⁽⁴⁾	Account Balance ⁽³⁾ or Accrued Benefit Obligation March 31, 2020
Andrea Beckwith-Ferraton - Chief Ethics and Compliance Officer	\$ 6	\$ -	\$ 6	\$ 6	\$ 17	\$ 8	\$ 25
Dr. Francois Belanger - VP, Quality and Chief Medical Officer	36	-	36	36	239	87	326
Dr. Ted Braun - VP and Medical Director, Clinical Operations							
SERP	-	-	-	5	220	(14)	206
SPP	28	-	28	28	140	49	189
Mauro Chies - VP, CancerControl Alberta and Clinical Support Services	20	-	20	18	102	36	138
Sean Chilton - VP, Health Professions and Practice and Information Technology	20	-	20	20	156	42	198
Todd Gilchrist - VP, People	34	-	34	34	132	59	191
Deb Gordon - VP and Chief Operations Officer, Clinical Operations							
SERP	-	(1)	(1)	15	683	(41)	642
SPP	25	-	25	25	165	54	219
Robert Hawes - Interim VP, Corporate Services and Chief Financial Officer	-	-	-	-	-	-	-
Brenda Huband - VP and Chief Health Operations Officer, Central and Southern Alberta							
SERP	-	(1)	(1)	9	400	(33)	367
SPP	25	-	25	25	174	52	226
Dr. Mark Joffe - VP and Medical Director, CancerControl Alberta, Clinical Support Services and Provincial Clinical Excellence ⁽⁶⁾	-	-	-	-	-	-	-
Deborah Rhodes - VP, Corporate Services and Chief Financial Officer ⁽⁴⁾	25	-	25	27	229	(229)	-
Dr. Kathryn Todd - VP, Provincial Clinical Excellence ⁽⁶⁾	-	-	-	-	-	-	-
Colleen Turner - VP, Community Engagement and Communications	20	-	20	20	108	42	150
Ronda White - Chief Audit Executive	13	-	13	14	90	28	118
Dr. Verna Yiu - President and Chief Executive Officer	49	-	49	49	137	66	203

(1) The SPP current period benefit costs are AHS contributions earned in the period.

(2) Other SERP costs include retirement benefits, interest expense on the obligations, and amortization of actuarial gains and losses, offset by the expected return on the plans' assets. AHS uses the straight line method to amortize actuarial gains and losses over the expected average remaining service life of the plan members.

(3) The account balance represents the total cumulative earned contributions to the SPP as well as cumulative investment gains or losses on the contributions.

(4) Changes in the accrued benefit obligation include current period benefit cost, interest accruing on the obligations and the amortization of any actuarial gains or losses in the period. Changes in the account balance include the current benefit costs and investment gains or losses related to the account.

Alberta Health Services

Footnotes to the consolidated Schedule of Salaries and Benefits

For the year ended March 31, 2020

Definitions

- a. For this schedule, full time equivalents (FTE) are determined by actual hours earned divided by 2,030.50 annual base hours. FTE for the Board and Board committee members are prorated using the number of days in the fiscal year between either the date of appointment and the end of the year, the date of appointment and the termination date, or the beginning of the year and the termination date.
- b. Base salary is regular salary and includes all payments earned related to actual hours earned other than those reported as other cash benefits.

Vacation accruals are included in base salary except for direct reports of the Board or President and Chief Executive Officer whose vacation accruals are included in other non-cash benefits.
- c. Other cash benefits include, as applicable, honoraria, overtime, acting pay, membership fees, travel and automobile allowances, lump sum payments and an allowance for professional development. Relocation expenses are excluded from compensation disclosure as they are considered to be recruiting costs to AHS and not part of compensation unless related to severance. Expense reimbursements are also excluded from compensation disclosure except where the expenses meet the definition of the other cash benefits listed above.
- d. Other non-cash benefits include:
 - Employer's current period benefit costs and other costs of supplemental pension plan and supplemental executive retirement plans as defined in Sub-Schedule 2C
 - Employer's share of employee benefit contributions and payments made on behalf of employees including pension, health care, dental and vision coverage, out-of-country medical benefits, group life insurance, accidental disability and dismemberment insurance, and long and short-term disability plans
 - Vacation accruals for direct reports of the Board or President and Chief Executive Officer, and
 - Employer's share of the cost of additional benefits including sabbaticals or other special leave with pay.
- e. Severance includes direct or indirect payments to individuals upon termination which are not included in other cash benefits or other non-cash benefits.
- f. Compensation provided by AHS for medical doctors included in salaries and benefits expense includes medical doctors paid through AHS payroll. The compensation provided by AHS for the remaining medical doctors is included in other contracted services.

Board and Board Committee Participants

- g. The Board Chair is an Ex-Officio member on all committees.
- h. These individuals were participants of Board committees, but are not Board members or AHS employees.

Executive

- i. Base salary reported for executives are the actual payments earned during the year, and is therefore contingent on the number of AHS' work days in the year. For the year ended March 31, 2020, the number of work days at AHS is 262 (2019 – 260 work days).
- j. The incumbent is engaged in an employment agreement with AHS while on leave of absence from the University of Alberta. The contract term ends June 2, 2021.
- k. The incumbent received a vacation payout of \$16 during the year for unused accrued vacation earned in prior periods; accrued vacation has been recorded in their compensation as a non-cash benefit in the period it was earned.
- l. The incumbent received a vacation payout of \$9 during the year for unused accrued vacation earned in prior periods; accrued vacation has been recorded in their compensation as a non-cash benefit in the period it was earned.
- m. The incumbent was appointed to the temporary position effective March 2, 2020. The term ends July 8, 2020.
- n. As a result of restructuring, the incumbent ceased to be a direct report to the President and Chief Executive Officer effective March 16, 2020.
- o. The incumbent is on secondment from the University of Alberta. The incumbent's total remuneration is comprised of salary amounts from both AHS and the University of Alberta, and AHS reimburses the University for the incumbent's base salary and benefits. In lieu of enrollment into the AHS SPP, the incumbent will receive an annual lump sum supplemental payment equivalent to the amount the incumbent would have received as a member of the SPP and payable from AHS. The lump sum has been included in Other Cash Benefits.

Alberta Health Services

Footnotes to the consolidated Schedule of Salaries and Benefits (Continued)

For the year ended March 31, 2020

- p. The incumbent held the position until February 28, 2020 at which time the incumbent left AHS. The incumbent received salary and other accrued entitlements to the date of departure. The reported severance included 65 weeks base salary at the rate in effect at the date of departure, and 15% of the severance in lieu of benefits. AHS will also make payments for the incumbent to attend an outplacement program for a maximum of five months. In addition, the incumbent received a vacation payout of \$46 for unused accrued vacation at the time of departure; accrued vacation has been recorded in their compensation as a non-cash benefit in the period it was earned.

Termination Obligations

- q. The incumbent's termination benefits have not been predetermined.
- r. In the case of termination without just cause by AHS, the incumbent shall receive severance pay equal to one month base salary for each completed month of service during the first year of the term or, after completion of one year of service of the term, 12 months base salary.
- s. There is no severance associated with the temporary position.
- t. There is no severance associated with the secondment agreement. Upon termination of the secondment agreement, the incumbent would return to the incumbent's regular position at the University of Alberta.
- u. Based on the provision of the applicable SPP, the following outlines the benefits received by individuals who terminated employment with AHS within the 2019-20 fiscal period. As a result of retirement or termination, the incumbents are entitled to the benefits accrued to them up to the date of retirement or termination. For participants of SPP, the benefit includes the account balances as at March 31, 2019 and the current period benefit costs and investment gains or losses related to the account that were incurred during the current year. The AHS obligations are paid through either a lump sum payment or regular instalments:

Position	Supplemental Plan Commencement Date	Benefit (not in thousands)	Frequency	Payment Terms
VP, Corporate Services and Chief Financial Officer	June 1, 2010	\$288,935	Once	June 2020

Alberta Health Services

Schedule 3 – Consolidated Schedule of Segment Disclosures

For the year ended March 31, 2020

(thousands of dollars)

	2020								
	Salaries and benefits	Contracts with health service providers	Contracts under the Health Care Protection Act	Drugs and gases	Medical supplies	Other contracted services	Other	Amortization and loss on disposals/write-downs of tangible capital assets	Total
Continuing care	\$ 321,315	\$ 806,123	\$ -	\$ 7,881	\$ 4,533	\$ 5,802	\$ 28,489	\$ 2,324	\$ 1,176,467
Community care	698,728	698,918	-	4,445	3,451	52,003	67,693	551	1,525,789
Home care	338,742	257,699	-	187	9,308	88,298	22,244	84	716,562
Acute care	3,022,568	388,383	20,041	511,365	356,465	562,803	148,107	56,075	5,065,807
Ambulance services	305,282	173,494	-	2,018	3,945	1,472	27,322	17,129	530,662
Diagnostic and therapeutic services	1,595,207	289,472	-	24,473	188,168	286,529	106,626	49,091	2,539,566
Population and public health	305,676	14,620	-	7,704	4,809	10,225	13,825	258	357,117
Research and education	189,530	3,495	-	103	1,364	124,776	25,217	149	344,634
Information technology	298,101	735	-	-	29	40,667	166,852	90,621	597,005
Support services	1,102,401	152,542	-	2,481	9,167	115,228	541,106	364,280	2,287,205
Administration	353,133	38,260	-	4	251	31,837	48,898	1,161	473,544
Total	\$ 8,530,683	\$ 2,823,741	\$ 20,041	\$ 560,661	\$ 581,490	\$ 1,319,640	\$ 1,196,379	\$ 581,723	\$ 15,614,358

Alberta Health Services

Schedule 3 – Consolidated Schedule of Segment Disclosures (Continued)

For the year ended March 31, 2020

(thousands of dollars)

	2019								
	Salaries and benefits	Contracts with health service providers	Contracts under the Health Care Protection Act	Drugs and gases	Medical supplies	Other contracted services	Other	Amortization and loss on disposals/write-downs of tangible capital assets	Total
Continuing care	\$ 317,473	\$ 775,491	\$ -	\$ 7,600	\$ 4,664	\$ 5,278	\$ 24,705	\$ 2,152	\$ 1,137,363
Community care	682,532	657,092		3,562	4,556	31,951	65,218	372	1,445,283
Home care	327,974	246,052	-	179	7,910	83,441	22,591	148	688,295
Acute care	2,994,658	394,435	17,186	461,210	366,996	569,378	149,794	64,810	5,018,467
Ambulance services	299,635	174,932	-	1,953	4,094	1,572	28,690	17,169	528,045
Diagnostic and therapeutic services	1,563,230	293,708	-	22,738	187,983	291,808	101,463	44,688	2,505,618
Population and public health	301,511	9,860	-	6,840	4,403	14,836	11,904	315	349,669
Research and education	186,172	2,911	-	15	1,171	129,589	27,216	165	347,239
Information technology	240,922	514	-	-	32	37,156	152,109	76,593	507,326
Support services	1,070,420	154,714	-	2,560	10,237	112,198	587,269	323,023	2,260,421
Administration	337,110	39,977	-	5	471	45,599	102,254	193	525,609
Total	\$ 8,321,637	\$ 2,749,686	\$ 17,186	\$ 506,662	\$ 592,517	\$ 1,322,806	\$ 1,273,213	\$ 529,628	\$ 15,313,335

Health Quality Council of Alberta

Financial Statements

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Management's Responsibility for Financial Reporting

The accompanying financial statements are the responsibility of management and have been reviewed and approved by Senior Management. The financial statements were prepared in accordance with Canadian Public Sector Accounting Standards, and of necessity, include some amounts that are based on estimates and judgement.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains a system of internal accounting controls comprising written policies, standards and procedures, a formal authorization structure, and satisfactory processes for reviewing internal controls. This system provides management with reasonable assurance that transactions are in accordance with governing legislation and are properly authorized, reliable financial records are maintained, and assets are adequately safeguarded.

The Health Quality Council of Alberta's Board of Directors carries out their responsibility for the financial statements through the Audit and Finance Committee. The Committee meets with management and the Auditor General of Alberta to review financial matters, and recommends the financial statements to the Health Quality Council of Alberta Board of Directors for approval upon finalization of the audit. The Auditor General of Alberta has open and complete access to the Audit and Finance Committee.

The Auditor General of Alberta provides an independent audit of the financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures, which allow him to report on the fairness of the financial statements prepared by management.

On behalf of the Health Quality Council of Alberta.

[Original signed by Andrew Neuner]

Chief Executive Officer
Andrew Neuner
May 28, 2020

[Original signed by Jessica Wing]

Director, Financial Services
Jessica Wing
May 28, 2020

Independent Auditor's Report

To the Board of Directors of the Health Quality Council of Alberta



**Auditor
General**
OF ALBERTA

Report on the Financial Statements

Opinion

I have audited the financial statements of the Health Quality Council of Alberta, which comprise the statement of financial position as at March 31, 2020, and the statements of operations, change in net financial assets, and cash flows for the year then ended, and notes to the financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying financial statements present fairly, in all material respects, the financial position of the Health Quality Council of Alberta as at March 31, 2020, and the results of its operations, its changes in net financial assets, and its cash flows for the year then ended in accordance with Canadian public sector accounting standards.

Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Financial Statements* section of my report. I am independent of the Health Quality Council of Alberta in accordance with the ethical requirements that are relevant to my audit of the financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

Other information

Management is responsible for the other information. The other information comprises the information included in the *Annual Report*, but does not include the financial statements and my auditor's report thereon. The *Annual Report* is expected to be made available to me after the date of this auditor's report.

My opinion on the financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

Responsibilities of management and those charged with governance for the financial statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is responsible for assessing the Health Quality Council of Alberta's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the Health Quality Council of Alberta's financial reporting process.

Auditor's responsibilities for the audit of the financial statements

My objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgment and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Health Quality Council of Alberta's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Health Quality Council of Alberta's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit evidence obtained up to the date of my auditor's report. However, future events or conditions may cause the Health Quality Council of Alberta to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Original signed by W. Doug Wylie FCPA, FCMA, ICD.D]
Auditor General

May 28, 2020
Edmonton, Alberta

Health Quality Council of Alberta

Statement of Operations

Year ended March 31, 2020

(thousands of dollars)

	2020		2019
	Budget	Actual	Actual
	(Note 4)		
Revenues			
Government transfers			
Alberta Health - operating grant	\$ 6,585	\$ 6,560	\$ 7,222
Investment income	20	27	22
Other revenue	-	25	70
	6,605	6,612	7,314
Expenses			
Administration	2,072	2,111	1,893
Health system analytics	2,563	2,596	2,348
Health system improvement	1,419	1,425	1,520
Communication	902	946	993
Ministerial assessment/study	220	79	719
	7,176	7,157	7,473
Annual operating (deficit)	(571)	(545)	(159)
Accumulated operating surplus, beginning of year	1,767	1,767	1,926
Accumulated operating surplus, end of year	\$ 1,196	\$ 1,222	\$ 1,767

The accompanying notes and schedules are part of these financial statements.

Health Quality Council of Alberta

Statement of Financial Position

As at March 31, 2020

(thousands of dollars)

	2020	2019
Financial Assets		
Cash	\$ 1,017	\$ 1,462
Accounts receivable	30	60
	<u>1,047</u>	<u>1,522</u>
Liabilities		
Accounts payable and accrued liabilities (Note 6)	686	757
Employee future benefits (Note 7)	134	121
Deferred revenue (Note 8)	-	-
Deferred lease inducements (Note 9)	111	147
	<u>931</u>	<u>1,025</u>
Net Financial Assets	<u>116</u>	<u>497</u>
Non-Financial Assets		
Tangible capital assets (Note 10)	936	1,188
Prepaid expenses	170	82
	<u>1,106</u>	<u>1,270</u>
Net Assets	<u>1,222</u>	<u>1,767</u>
Net Assets		
Accumulated operating surplus (Note 12)	<u>\$ 1,222</u>	<u>\$ 1,767</u>

Contractual obligations (Note 11)

The accompanying notes and schedules are part of these financial statements.

Health Quality Council of Alberta

Statement of Change in Net Financial Assets

Year ended March 31, 2020

(thousands of dollars)

	2020		2019
	Budget	Actual	Actual
Annual operating (deficit)	\$ (571)	\$ (545)	\$ (159)
Acquisition of tangible capital assets (Note 10)	(74)	(44)	(374)
Amortization and write down of tangible capital assets (Note 10)	293	296	253
(Increase) in prepaid expenses	-	(88)	(15)
(Decrease) in net financial assets in the year	(352)	(381)	(295)
Net financial assets, beginning of year	497	497	792
Net financial assets, end of year	\$ 145	\$ 116	\$ 497

The accompanying notes and schedules are part of these financial statements.

Health Quality Council of Alberta

Statement of Cash Flows

Year ended March 31, 2020

(thousands of dollars)

	2020	2019
Operating Transactions		
Annual operating (deficit)	\$ (545)	\$ (159)
Non-cash items:		
Amortization and write down of tangible capital assets (Note 10)	296	253
Amortization of deferred lease inducements (Note 9)	(36)	(37)
Increase in employee future benefits (Note 7)	13	27
	<u>(272)</u>	<u>84</u>
Decrease (Increase) in accounts receivable	30	(11)
(Increase) in prepaid expenses	(88)	(15)
(Decrease) Increase in accounts payable and accrued liabilities (Note 6)	(71)	130
(Decrease) in deferred revenue	-	(6)
Increase in deferred lease inducements (Note 9)	-	86
Cash (used) provided by operating transactions	<u>(401)</u>	<u>268</u>
Capital Transactions		
Acquisition of tangible capital assets (Note 10)	(44)	(374)
Cash (applied to) capital transactions	<u>(44)</u>	<u>(374)</u>
(Decrease) in cash	(445)	(106)
Cash at beginning of year	1,462	1,568
Cash at end of year	\$ 1,017	\$ 1,462

The accompanying notes and schedules are part of these financial statements.

Health Quality Council of Alberta

Notes to the Financial Statements

Year ended March 31, 2020

(thousands of dollars)

Note 1 AUTHORITY

The Health Quality Council of Alberta (HQCA) is a government not-for-profit organization formed under the *Health Quality Council of Alberta Act*.

Pursuant to the Act, the HQCA has a mandate to promote and improve patient safety and health service quality on a province-wide basis.

The HQCA is exempt from income taxes under the *Income Tax Act*.

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES

These financial statements are prepared in accordance with Canadian public sector accounting standards (PSAS).

(a) Reporting Entity

The financial statements reflect the assets, liabilities, revenues and expenses of the HQCA.

(b) Basis of Financial Reporting

Revenues

All revenues are reported on the accrual basis of accounting. Cash received for which services have not been provided by year end is recognized as deferred revenue.

Government transfers

Transfers from all governments are referred to as government transfers.

Government transfers and the associated externally restricted investment income are recognized as deferred revenue if the eligibility criteria for use of the transfer, or the stipulations together with the HQCA's actions and communications as to the use of the transfer, create a liability. These transfers are recognized as revenue as the stipulations are met and, when applicable, the HQCA complies with its communicated use of these transfers.

All other government transfers, without stipulations for the use of the transfer, are recognized as revenue when the transfer is authorized and the HQCA meets the eligibility criteria (if any).

Expenses

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed.

Grants and transfers are recognized as expenses when the transfer is authorized and eligibility criteria have been met by the recipient.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

(b) Basis of Financial Reporting (Cont'd) Valuation of Financial Assets and Liabilities

The HQCA's financial assets and liabilities are generally measured as follows:

<u>Financial Statement Component</u>	<u>Measurement</u>
Cash	Cost
Accounts receivable	Lower of cost or net recoverable value
Accounts payable and accrued liabilities	Cost

The HQCA does not hold equities traded in an active market, nor engage in derivative contracts or foreign currency transactions. The HQCA is not exposed to remeasurement gains or losses and, consequently, a statement of remeasurement gains and losses is not presented.

Financial Assets

Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations.

Financial assets are the HQCA's financial claims on external organizations and individuals at the year end.

Cash

Cash comprises cash on hand and demand deposits.

Accounts Receivable

Accounts receivable are recognized at the lower of cost or net recoverable value. A valuation allowance is recognized when recovery is uncertain.

Liabilities

Liabilities represent present obligations of the HQCA to external organizations and individuals arising from past transactions or events occurring before the year end, the settlement of which is expected to result in the future sacrifice of economic benefits. They are recognized when there is an appropriate basis of measurement and management can reasonably estimate the amounts.

Deferred Lease Inducements

Deferred lease inducements represent amounts received for leasehold improvements and the value of a rent-free period. Lease inducements are deferred and amortized on a straight-line basis over the term of the related lease and the amortization is recognized as a reduction of rent expense for the year.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

(b) Basis of Financial Reporting (Cont'd)

Employee Future Benefits

The HQCA Board has approved a defined contribution Supplementary Executive Retirement Plan (SERP) for certain members of its executive staff. The SERP supplements the benefit under the HQCA registered plan that is limited by the *Income Tax Act* (Canada). The HQCA contributes a certain percentage of an eligible employee's pensionable earnings in excess of the limits of the *Income Tax Act* (Canada). This plan provides participants with an account balance at retirement based on the contributions made to the plan and investment income earned on the contributions based on investment decisions made by the participants.

Non-Financial Assets

Non-financial assets are acquired, constructed, or developed assets that do not normally provide resources to discharge existing liabilities, but instead:

- (a) are normally employed to deliver government services;
- (b) may be consumed in the normal course of operations; and
- (c) are not for sale in the normal course of operations.

Non-financial assets are limited to tangible capital assets and prepaid expenses.

Tangible Capital Assets

Tangible capital assets are recognized at cost less amortization, which includes amounts that are directly related to the acquisition, design, construction, development, improvement or betterment of the assets. Cost includes overhead directly attributable to construction and development, as well as interest costs that are directly attributable to the acquisition or construction of the asset.

The cost, less residual value, of the tangible capital assets, excluding work-in-progress, is amortized on a straight-line basis over their estimated useful lives as follows:

Computer hardware and software	5 years
Office equipment	10 years
Leasehold improvements	Over term of lease

Tangible capital assets are written down when conditions indicate that they no longer contribute to the HQCA's ability to provide services, or when the value of future economic benefits associated with the tangible capital assets are less than their book value. The net write-downs are accounted for as expenses in the Statement of Operations.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

(b) Basis of Financial Reporting (Cont'd)

Prepaid Expenses

Prepaid expenses are recognized at cost and amortized based on the terms of the agreement.

Funds and Reserves

Certain amounts, as approved by the Board of Directors, are set aside in accumulated operating surplus for future operating and capital purposes. Transfers to/from funds and reserves are an adjustment to the respective fund when approved.

Measurement Uncertainty

Measurement uncertainty exists when there is a variance between the recognized or disclosed amount and another reasonably possible amount. The amounts recognized for amortization of tangible capital assets are based on estimates of the useful life of the related assets. Actual results could differ from estimates.

Note 3 FUTURE ACCOUNTING CHANGES

The Public Sector Accounting Board has approved the following accounting standards:

- **PS 3280 Asset Retirement Obligations (effective April 1, 2021)**

Effective April 1, 2021, this standard provides guidance on how to account for and report liabilities for retirement of tangible capital assets.

- **PS 3400 Revenue (effective April 1, 2022)**

This standard provides guidance on how to account for and report on revenue, and specifically, it addresses revenue arising from exchange transactions and unilateral transactions.

Management is currently assessing the impact of these standards on the financial statements.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 4 BUDGET

The HQCA's 2019-2020 operating budget with a budgeted deficit of (\$571) was approved by the Board of Directors on February 7, 2019 and submitted to the Ministry of Health.

Note 5 FINANCIAL RISK MANAGEMENT

The HQCA has the following financial instruments: accounts receivable, accounts payable and accrued liabilities.

The HQCA has exposure to the following risks from its use of financial instruments: interest rate risk, liquidity risk, other price risk and credit risk.

(a) Interest rate risk

Interest rate risk is the risk that the rate of return and future cash flows on the HQCA's short-term investments will fluctuate because of changes in market interest rates. As the HQCA invests in short term deposits of 90 days or less and accounts payable are non-interest bearing, the HQCA is not exposed to significant interest rate risk relating to its financial instruments.

(b) Liquidity risk

Liquidity risk is the risk that the HQCA will encounter difficulty in meeting obligations associated with financial liabilities. The HQCA enters into transactions to purchase goods and services on credit. Liquidity risk is measured by reviewing the HQCA's future net cash flows for the possibility of negative net cash flow. The HQCA manages the liquidity risk resulting from its accounts payable obligations by maintaining cash resources and investing in short-term deposits of 90 days or less.

(c) Other price risk

Other price risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate because of changes in market prices (other than those arising from interest rate risk or foreign currency risk), whether those changes are caused by factors specific to the individual financial instrument or its issuer, or factors affecting all similar financial instruments traded in the market. Price risk is managed by holding short-term deposits for 90 days or less.

(d) Credit risk

The HQCA is exposed to credit risk from potential non-payment of accounts receivable. During the fiscal year most of the HQCA's receivables are from provincial agencies; therefore the credit risk is minimized.

Note 6 ACCOUNTS PAYABLE AND ACCRUED LIABILITIES

Included in accounts payable and accrued liabilities is \$160 (2019 - \$0) of funds held on behalf of the partners of PROactive: Partners in Professionalism initiative to cover expenses which the HQCA will incur on their behalf.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 7 EMPLOYEE FUTURE BENEFITS

The HQCA participates in the Local Authorities Pension Plan (LAPP), a multi-employer defined benefit pension plan.

The HQCA accounts for this multi-employer pension plan on a defined contribution basis. The HQCA is not responsible for future funding of the plan deficit other than through contribution increases. Pension expense recorded in the financial statements is equivalent to HQCA's annual contributions of \$388 for the year ended March 31, 2020 (2019 - \$405).

At December 31, 2019, the Local Authorities Pension Plan reported a surplus of \$7,913,261 (2018 – surplus of \$3,469,347).

The fair value of the pension plan is subject to significant market volatility due to the economic crisis stemming from the global pandemic COVID-19 virus. To the extent that the pension plan may not recover market-losses during the remainder of 2020, the funded status of the plan would experience a correlated decline. The financial market impact of the outbreak has been rapidly evolving, which precludes a reasonable estimate of the impact.

The Supplementary Executive Retirement Plan (SERP) payable at year ended March 31, 2020 is \$134 (2019 - \$121). The current year contribution related to this plan is \$13 (2019 - \$27).

Note 8 DEFERRED REVENUE

Deferred revenue represents unspent externally restricted resources. Changes in the balance are as follows:

	2020	2019
Balance, beginning of the year	\$ -	\$ 6
Amount received during the year	-	-
Less: Amount recognized as revenue	-	(6)
Balance, end of the year	\$ -	\$ -

Note 9 DEFERRED LEASE INDUCEMENTS

The HQCA received a lease inducement in the form of free rent relating to a lease renewal of the premises effective 2018. This amount will be amortized on a straight-line basis over the term of the related lease and the amortization is recognized as a reduction of rent expense.

	2020	2019
Lease inducements - rent free periods	\$ 209	\$ 209
Less accumulated amortization	(98)	(62)
	\$ 111	\$ 147

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 10 TANGIBLE CAPITAL ASSETS

	2020				2019
	Office Equipment	Computer Hardware & Software	Leasehold improvements	Total	Total
Estimated useful life	10 years	5 years	5-10 years		
Historical Cost					
Beginning of year	\$ 401	\$ 813	\$ 1,013	\$ 2,227	\$ 1,880
Additions	-	44	-	44	374
Disposals, including write-downs	-	(38)	-	(38)	(27)
	401	819	1,013	2,233	2,227
Accumulated Amortization					
Beginning of year	217	300	522	1,039	813
Amortization expense	32	139	123	294	253
Effect of disposals, including write-downs	-	(36)	-	(36)	(27)
	249	403	645	1,297	1,039
Net book value at March 31, 2020	\$ 152	\$ 416	\$ 368	\$ 936	
Net book value at March 31, 2019	\$ 184	\$ 513	\$ 491		\$ 1,188

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 11 CONTRACTUAL OBLIGATIONS

Contractual obligations are obligations of the HQCA to others that will become liabilities in the future when the terms of those contracts or agreements are met.

Estimated payment requirements for each of the next three years and thereafter are as follows:

Year ended March 31	Total lease payments
2020 - 21	\$ 475
2021 - 22	479
2022 - 23	479
Thereafter	-
	<u>\$ 1,433</u>

Note 12 ACCUMULATED OPERATING SURPLUS

Accumulated operating surplus is comprised of the following:

	2020			2019	
	Investment in Tangible Capital Assets ^(a)	Internally Restricted Surplus ^(b)	Unrestricted Surplus (Deficit)	Total	Total
Balance, April 1, 2019	\$ 1,186	\$ 581	\$ -	\$ 1,767	\$ 1,926
Annual operating (deficit)	-	-	(545)	(545)	(159)
Net investments in capital assets	(250)	-	250	-	-
Transfers, prior year restricted	-	(581)	581	-	-
Transfers, current year restricted	-	286	(286)	-	-
Balance, March 31, 2020	<u>\$ 936</u>	<u>\$ 286</u>	<u>\$ -</u>	<u>\$ 1,222</u>	<u>\$ 1,767</u>

- (a) Net assets equal to net book value of internally funded tangible capital assets are restricted as these net assets are not available for any other purpose.

Health Quality Council of Alberta

Notes to the Financial Statements

(thousands of dollars)

Note 12 ACCUMULATED OPERATING SURPLUS (CONT'D)

- (b) The internally restricted surplus represents amounts set aside by the Board for future purposes. Those amounts are not available for other purposes without the approval of the Board. Internally restricted surplus based on the business plan is summarized as follows:

	2020	2019
Experience surveys	\$ 218	\$ -
FOCUS on Healthcare	50	-
Ministerial assessments and studies	18	-
Measure to improve	-	240
Monitor the health system	-	37
Engage with the public	-	304
	<u>\$ 286</u>	<u>\$ 581</u>

Note 13 COMPARATIVE FIGURES

Certain 2019 figures have been reclassified to conform to the 2020 presentation.

Note 14 APPROVAL OF THE FINANCIAL STATEMENTS

The financial statements were approved by the HQCA Board of Directors on May 28, 2020.

Health Quality Council of Alberta

Schedule 1 – Expenses – Detailed By Object

Year ended March 31

(thousands of dollars)

	2020		2019
	Budget	Actual	Actual
Salaries and benefits	\$ 3,819	\$ 3,979	\$ 4,207
Supplies, services and other	3,064	2,884	3,013
Amortization of tangible capital assets (Note 10)	293	294	253
	\$ 7,176	\$ 7,157	\$ 7,473

Health Quality Council of Alberta

Schedule 2 – Salary and Benefits Disclosures

Year ended March 31

(thousands of dollars)

	2020			2019	
	Base Salary ⁽¹⁾	Other Cash Benefits ⁽²⁾	Other Non- Cash Benefits ⁽³⁾	Total	Total
Board of Directors-Chair	\$ -	\$ 19	\$ -	\$ 19	\$ 13
Board of Directors-Members	-	54	-	54	50
Chief Executive Officer	249	6	43	298	453
Executive Director	184	-	34	218	219
	\$ 433	\$ 79	\$ 77	\$ 589	\$ 735

(1) Base salary includes pensionable base pay.

(2) Other cash benefits include honoraria for board members and vehicle allowance, flexible spending allowance and vacation payouts to employees. There were no bonuses paid in 2019/2020.

(3) Other non-cash benefits include: employer's portion of all employee benefits and contributions or payments made on behalf of employees, including pension, Supplementary Executive Retirement Plan, health care benefits, dental coverage, vision coverage, out of country medical benefits, group life insurance, accidental disability and dismemberment insurance, employee assistance program, Canadian Pension Plan, Employment Insurance and fair market value parking.

Health Quality Council of Alberta

Schedule 3 – Related Party Transactions

Year ended March 31
(thousands of dollars)

Related parties are those entities consolidated or accounted for on a modified equity basis in the Government of Alberta's Consolidated Financial Statements. Related parties also include key management personnel and close family members of those individuals in the HQCA. The HQCA and its employees paid or collected certain taxes and fees set by regulation for premiums, licenses and other charges. These amounts were incurred in the normal course of business, reflect charges applicable to all users, and have been excluded from this schedule.

The HQCA had the following transactions with related parties recorded in the Statement of Operations and the Statement of Financial Position at the amount of consideration agreed upon between the related parties.

	2020	2019
Revenues		
Grants	\$ 6,560	\$ 7,222
Other	6	33
	<u>\$ 6,566</u>	<u>\$ 7,255</u>
Expenses		
Other services	<u>\$ 114</u>	<u>\$ 301</u>
Receivable from related parties	<u>\$ 1</u>	<u>\$ -</u>
Payable to related parties	<u>\$ 29</u>	<u>\$ 19</u>

Other Financial Information

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Department of Health

Statement of Credit or Recovery (unaudited)

Year ended March 31, 2020
(in thousands)

	2020				
	Authorized	Actual Revenue Recognized	Unearned Revenue	Total Revenue Received / Receivable	(Shortfall) / Excess
Support Programs					
Other Support Programs ^(a)	\$ 1,000	\$ -	\$ -	\$ -	\$ (1,000)
	<u>\$ 1,000</u>	<u>\$ -</u>	<u>\$ -</u>	<u>\$ -</u>	<u>\$ (1,000)</u>

(a) The Department receives funding from Health Canada to enhance existing health services to persons with chronic Hepatitis C virus infection.

Department of Health

Lapse/Encumbrance (unaudited)

Year ended March 31, 2020
(in thousands)

Expense Vote by Program	Voted Estimate ⁽¹⁾	Supplementary Estimate	Adjustments ⁽²⁾	Adjusted Voted Estimate	Voted Actuals ⁽³⁾	Unexpended / (Over Expended)
Operating Expense						
1 Ministry Support Services						
1.1 Minister's Office	\$ 1,195	\$ -	\$ -	\$ 1,195	\$ 1,184	\$ 11
1.2 Associate Minister's Office	395	-	-	395	380	15
1.3 Deputy Minister's Office	1,553	-	-	1,553	1,370	183
1.4 Strategic Corporate Support	40,068	-	-	40,068	43,028	(2,960)
1.5 Policy Development and Strategic Support	16,667	-	-	16,667	13,678	2,989
1.6 Health Advocates' Office	1,675	-	-	1,675	1,162	513
	61,553	-	-	61,553	60,802	751
2 Alberta Health Services						
2.1 Continuing Care	1,123,000	-	-	1,123,000	1,123,000	-
2.2 Community Care	1,254,000	-	-	1,254,000	1,254,000	-
2.3 Home Care	688,000	-	-	688,000	688,000	-
2.4 Acute Care	3,680,000	-	-	3,680,000	3,680,000	-
2.5 Ambulance Services	465,000	-	-	465,000	465,000	-
2.6 Diagnostic and Therapeutic Services	2,373,000	-	-	2,373,000	2,373,000	-
2.7 Population and Public Health	330,000	-	-	330,000	330,000	-
2.8 Health Workforce Education and Research	81,000	-	-	81,000	81,000	-
2.9 Information Technology	484,000	-	-	484,000	484,000	-
2.10 Support Services	1,634,000	-	-	1,634,000	1,634,000	-
2.11 Administration	488,000	-	-	488,000	488,000	-
	12,600,000	-	-	12,600,000	12,600,000	-
3 Physician Compensation and Development						
3.1 Program Support	7,529	-	-	7,529	8,303	(774)
3.2 Physician Remuneration	4,536,470	-	-	4,536,470	4,560,681	(24,211)
3.3 Physician Development	174,851	-	-	174,851	174,824	27
3.4 Physician Benefits	291,132	-	-	291,132	292,521	(1,389)
	5,009,982	-	-	5,009,982	5,036,329	(26,347)

Department of Health

Lapse/Encumbrance (unaudited) (continued)

Year ended March 31, 2020

(in thousands)

Expense Vote by Program	Voted Estimate ⁽¹⁾	Supplementary Estimate	Adjustments (2)	Adjusted Voted Estimate	Voted Actuals ⁽³⁾	Unexpended / (Over Expended)
Operating Expense						
4 Drugs and Supplemental Health Benefits						
4.1 Program Support	\$ 47,964	\$ -	\$ -	\$ 47,964	\$ 47,987	\$ (23)
4.2 Outpatient Cancer Therapy Drugs	226,700	-	-	226,700	226,700	-
4.3 Outpatient Specialized High Cost Drugs	119,715	-	-	119,715	119,715	-
4.4 Seniors Drug Benefits	572,362	-	-	572,362	608,233	(35,871)
4.5 Seniors Dental, Optical and Supplemental Health Benefits	138,746	-	-	138,746	134,748	3,998
4.6 Non-Group Drug Benefits	218,417	-	-	218,417	213,964	4,453
4.7 Non-Group Supplemental Health Benefits	900	-	-	900	834	66
4.8 Assured Income for the Severely Handicapped Health Benefit	241,780	-	-	241,780	231,347	10,433
4.9 Child Health Benefit	31,352	-	-	31,352	30,545	807
4.10 Adult Health Benefit	225,033	-	-	225,033	231,582	(6,549)
4.11 Alberta Aids to Daily Living	165,279	-	-	165,279	170,166	(4,887)
4.12 Pharmaceutical Innovation and Management	103,089	-	-	103,089	104,450	(1,361)
	<u>2,091,337</u>	<u>-</u>	<u>-</u>	<u>2,091,337</u>	<u>2,120,271</u>	<u>(28,934)</u>
5 Addiction and Mental Health						
5.1 Program Support	3,293	-	-	3,293	3,240	53
5.2 Addiction and Mental Health	93,228	-	-	93,228	70,925	22,303
	<u>96,521</u>	<u>-</u>	<u>-</u>	<u>96,521</u>	<u>74,165</u>	<u>22,356</u>
6 Primary Health Care						
6.1 Program Support	3,096	-	-	3,096	2,919	177
6.2 Primary Health Care	238,044	-	-	238,044	241,753	(3,709)
	<u>241,140</u>	<u>-</u>	<u>-</u>	<u>241,140</u>	<u>244,672</u>	<u>(3,532)</u>
7 Population and Public Health						
7.1 Program Support	13,658	-	-	13,658	11,448	2,210
7.2 Immunization Support	2,620	-	-	2,620	1,521	1,099
7.3 Community-Based Health Services	60,334	-	-	60,334	57,149	3,185
7.4 Research and Support Programs	15,362	-	-	15,362	13,007	2,355
7.5 Palliative Care	5,000	-	-	5,000	-	5,000
	<u>96,974</u>	<u>-</u>	<u>-</u>	<u>96,974</u>	<u>83,125</u>	<u>13,849</u>
8 Allied Health Services	<u>116,627</u>	<u>-</u>	<u>-</u>	<u>116,627</u>	<u>123,233</u>	<u>(6,606)</u>
9 Human Tissue and Blood Services	<u>175,000</u>	<u>-</u>	<u>-</u>	<u>175,000</u>	<u>158,719</u>	<u>16,281</u>

Department of Health

Lapse/Encumbrance (unaudited) (continued)

Year ended March 31, 2020

(in thousands)

Expense Vote by Program	Voted Estimate ⁽¹⁾	Supplementary Estimate	Adjustments (2)	Adjusted Voted Estimate	Voted Actuals ⁽³⁾	Unexpended / (Over Expended)
Operating Expense						
10 Support Programs						
10.1 Program Support	\$ 8,913	\$ -	\$ -	\$ 8,913	\$ 8,168	\$ 745
10.2 Health Quality Council of Alberta	7,559	-	-	7,559	6,560	999
10.3 Protection for Persons in Care	1,900	-	-	1,900	2,026	(126)
10.4 Monitoring, Investigations and Licensing	7,720	-	-	7,720	7,227	493
10.5 Health System Projects	4,000	-	-	4,000	2,160	1,840
	30,092	-	-	30,092	26,141	3,951
11 Out-of-Province Health Care Services						
11.1 Program Support	6,872	-	-	6,872	6,521	351
11.2 Out-of-Province Health Care Services	144,304	-	-	144,304	148,092	(3,788)
	151,176	-	-	151,176	154,613	(3,437)
12 Information Technology						
12.1 Program Support	6,943	-	-	6,943	6,437	506
12.2 Development and Operations	71,130	-	-	71,130	59,526	11,604
	78,073	-	-	78,073	65,963	12,110
13 Cancer Research and Prevention Investment	25,000	-	-	25,000	14,799	10,201
Capital Grants						
14 Infrastructure Support						
14.1 Continuing Care Beds	28,326	-	-	28,326	5,313	23,013
	28,326	-	-	28,326	5,313	23,013
Capital Payments to Related Parties						
9 Human Tissue and Blood Services	-	-	-	-	1,000	(1,000)
14 Infrastructure Support						
14.1 Continuing Care Beds	4,700	-	-	4,700	4,414	286
14.2 External Information Systems Development	5,748	-	-	5,748	3,005	2,743
14.3 Equipment for Cancer Corridor Projects	3,469	-	-	3,469	-	3,469
14.4 Medical Equipment Replacement and Upgrade Program	30,000	-	-	30,000	30,000	-
14.5 Clinical Information System	136,751	-	-	136,751	136,751	-
	180,668	-	-	180,668	175,170	5,498
Credit or Recovery (Shortfall)	-	-	(1,000)	(1,000)	-	(1,000)
Total	\$ 20,982,469	\$ -	\$ (1,000)	\$ 20,981,469	\$ 20,943,315	\$ 38,154
Lapse						\$ 38,154

Department of Health

Lapse/Encumbrance (unaudited) (continued)

Year ended March 31, 2020
(in thousands)

	Voted Estimate ⁽¹⁾	Supplementary Estimate	Adjustments (2)	Adjusted Voted Estimate	Voted Actuals ⁽³⁾	Unexpended / (Over Expended)
Capital Investment Vote by Program						
Department Capital Acquisitions						
7 Population and Public Health						
7.2 Immunization Support	\$ -	\$ -	\$ -	\$ -	\$ 24	\$ (24)
12 Information Technology						
12.2 Development and Operations	22,230	-	-	22,230	20,606	1,624
Total	<u>\$ 22,230</u>	<u>\$ -</u>	<u>\$ -</u>	<u>\$ 22,230</u>	<u>\$ 20,630</u>	<u>\$ 1,600</u>
Lapse						<u>\$ 1,600</u>
Financial Transactions Vote by Program						
Inventory Acquisition						
4.3 Outpatient Specialized High Cost Drugs	\$ 9,000	\$ -	\$ -	\$ 9,000	\$ 5,892	\$ 3,108
7 Population and Public Health						
7.2 Immunization Support	58,819	-	-	58,819	54,013	4,806
Total	<u>\$ 67,819</u>	<u>\$ -</u>	<u>\$ -</u>	<u>\$ 67,819</u>	<u>\$ 59,905</u>	<u>\$ 7,914</u>
Lapse						<u>\$ 7,914</u>
Contingency and Disaster and Emergency Assistance						
15 COVID-19 Pandemic Response	\$ -	\$ -	\$ 33,594	\$ 33,594	\$ 27,490	\$ 6,104
Total	<u>\$ -</u>	<u>\$ -</u>	<u>\$ 33,594</u>	<u>\$ 33,594</u>	<u>\$ 27,490</u>	<u>\$ 6,104</u>
Lapse						<u>\$ 6,104</u>

⁽¹⁾ As per "Expense Vote by Program", "Capital Investment Vote by Program" and "Financial Transactions Vote by Program" page 118 to 120 of 2019-2020 Government Estimates.

⁽²⁾ Adjustments include encumbrances, capital carry over amounts, transfers between votes and credit or recovery increases approved by Treasury Board Committee and credit or recovery shortfalls. An encumbrance is incurred when, on a vote by vote basis, the total of actual disbursements in the prior year exceed the total adjusted estimate.

Adjustments also include supply vote transfers for "Contingency and Disaster and Emergency Assistance" as approved by the Lieutenant Governor in Council under the direction of the Minister of Finance. The Contingency and Disaster and Emergency Assistance supply vote consists of a provisional funding authority transferable to any ministry. Upon approval by the Lieutenant Governor in Council, the President of Treasury Board and Minister of Finance may either spend or transfer all or a portion of this supply vote to another minister for public emergencies, disasters or unanticipated costs. As per Order in Council 094/2020 approved on March 25, 2020, \$33,594 was transferred from the Department of Treasury Board and Finance to the Department of Health towards pandemic response.

⁽³⁾ Actuals exclude non-voted amounts such as amortization, inventory consumption, and valuation adjustments.

Department of Health

Statement of Remissions, Compromises and Write-offs (unaudited)

Year ended March 31, 2020

(in thousands)

	<u>2020</u>	<u>2019</u>
Write-Offs		
Medical Claim Recoveries	\$ 2,914	\$ 2,621
Pharmaceuticals and Health Benefits	916	85
Other Receivables	456	458
Total Write-offs ⁽¹⁾	<u>\$ 4,286</u>	<u>\$ 3,164</u>

⁽¹⁾ There were no remissions or compromises during the year.

The above statement has been prepared pursuant to Section 23 of the Financial Administration Act. The statement includes all remissions, compromises and write-offs made or approved during the fiscal year.

Ministry of Health

Year ended March 31, 2020
(in thousands)

Payments Based on Agreements (unaudited)

The Ministry entered into agreements, under the Alberta Health Care Insurance Plan, with other Provincial and Territorial Governments to provide health services on their behalf. The Ministry pays service providers for services rendered under the agreements and recovers the amount paid from the program sponsors. Service providers include Alberta Health Services and physicians. Costs based on these agreements are incurred by the Ministry under authority in section 25 of the *Financial Administration Act*.

Amounts paid based on agreements with program sponsors are as follows:

	2020	2019
Other Provincial and Territorial Governments	\$ 323,290	\$ 310,183

Accounts receivable includes \$33,551 (2019 - \$46,723).

Trust Funds under Administration (unaudited)

Trust funds under administration are funds consisting of public money over which the Legislature has no power of appropriation. Because the Ministry has no equity in the funds and administers them for the purposes of various trusts, they are not included in the consolidated financial statements.

At March 31, 2020, the Health Benefit Trust of Alberta reported fund balance of \$110,699 (2019 - \$139,307). At March 31, 2020, balance of trust funds held for others is \$1,490 (2019 - \$1,552).

The Ministry and a third party trustee administer the SERP in accordance with a retirement compensation arrangement trust agreement. At March 31, 2020, there are \$29,181 in plan assets (2019 - \$32,674).

Annual Report Extracts and Other Statutory Reports

Public Interest Disclosure (Whistleblower Protection) Act

Section 32 of the *Public Interest Disclosure (Whistleblower Protection) Act* reads:

- 32(1) Every chief officer must prepare a report annually on all disclosures that have been made to the designated officer of the department, public entity or office of the Legislature for which the chief officer is responsible.
- (2) The report under subsection (1) must include the following information:
- (a) the number of disclosures received by the designated officer, the number of disclosures acted on and the number of disclosures not acted on by the designated officer;
 - (b) the number of investigations commenced by the designated officer as a result of disclosures;
 - (c) in the case of an investigation that results in a finding of wrongdoing, a description of the wrongdoing and any recommendations made or corrective measures taken in relation to the wrongdoing or the reasons why no corrective measure was taken.
- (3) The report under subsection (1) must be included in the annual report of the department, public entity or office of the Legislature if the annual report is made publicly available.

Public Interest Disclosure Report

The following report pertains to the **Department of Health** (April 1, 2019 to March 31, 2020):

1 – Disclosure:

- 1 disclosure acted on
- 0 disclosures not acted on

0 – Investigations

0 – Investigations resulting in a finding of wrongdoing

Note: Alberta Health Services and the Health Quality Council of Alberta are considered separate entities for the purposes of the Act, and therefore have individual reporting obligations.